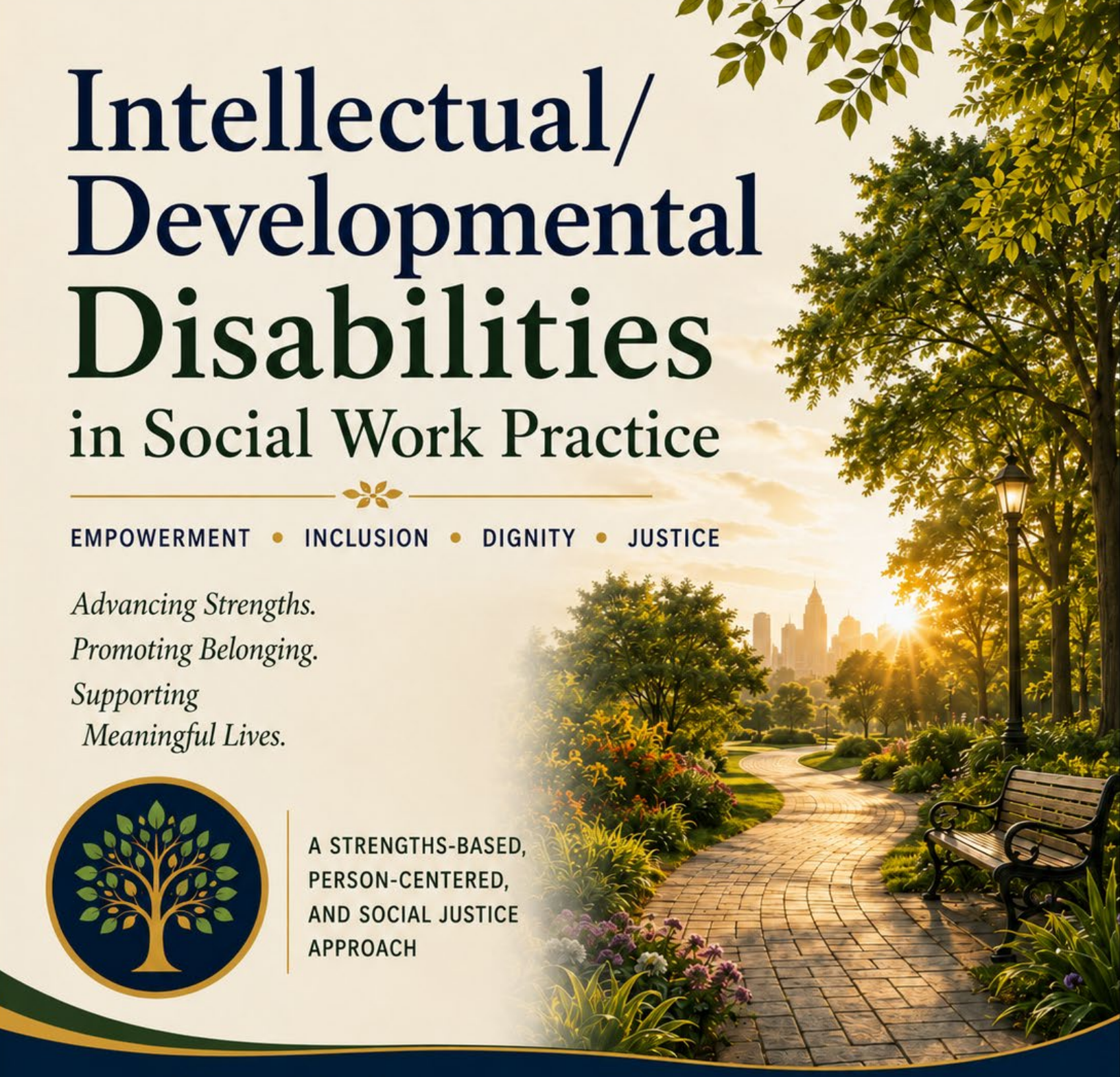
Intellectual/ Developmental Disabilities in Social Work Practice

EMPOWERMENT • INCLUSION • DIGNITY • JUSTICE

Advancing Strengths.
Promoting Belonging.
Supporting Meaningful Lives.



A STRENGTHS-BASED, PERSON-CENTERED, AND SOCIAL JUSTICE APPROACH

Sabretta Alford, PhD, MPhil, LMSW • **Esther Son,** PhD, MSW

VOLUME 1 • FIRST EDITION

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Intellectual Developmental Disabilities in Social Work Practice

Sabretta Alford & Esther Son

Volume 1. First Edition

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Disability Laws

The Americans with Disabilities Act (ADA)

The **Americans with Disabilities Act (ADA)** is a civil rights law enacted in 1990 that prohibits discrimination against individuals with disabilities in all areas of public life, including employment, education, transportation, public accommodations, communications, and access to state and local government programs and services. The ADA's purpose is to ensure that people with disabilities have the same rights and opportunities as everyone else, promoting their full participation in society.

The ADA is divided into five titles (sections) that relate to different aspects of public life. The first section, **Title I: Employment,** prohibits discrimination against qualified individuals with disabilities in all aspects of employment, including hiring, firing, promotions, training, and job assignments. Employers with 15 or more employees must provide reasonable accommodation to employees with disabilities, as long as it does not cause undue hardship to the business. The next section is **Title II: Public Services (State and Local Government)**, which ensures that individuals with disabilities have equal access to all public services, programs, and activities provided by state and local governments. This includes public transportation, public education, and access to government buildings and services. The third section, **Title III: Public Accommodations and Commercial Facilities,** prohibits discrimination in public accommodations, including restaurants, hotels, theaters, shopping centers, and recreational facilities. It requires these facilities to be accessible to individuals with disabilities and mandates removing barriers to access where feasible. The fourth section, **Title IV: Telecommunications,** requires telephone and internet companies to provide telecommunications services that allow individuals with hearing or speech disabilities to communicate by phone, including relay services, and the fifth section, **Title V: Miscellaneous Provisions**, contains various provisions relating to the ADA's implementation, including regulations, enforcement, and the relationship between the ADA and other laws.

Various federal agencies enforce the ADA, depending on the specific title. For example, the Equal Employment Opportunity Commission (EEOC) enforces Title I, while the Department of Justice (DOJ) enforces Titles II and III, and has been instrumental in improving accessibility and ensuring that individuals with disabilities are treated with dignity and respect in all aspects of public life. It has also led to the development of physical infrastructure, technology, and policies that accommodate people with diverse disabilities.

Key Terms and Definitions

Term	Definition
Americans with Disabilities Act (ADA)	A comprehensive civil rights law enacted in 1990 that prohibits discrimination against individuals with disabilities in all areas of public life, including employment, education, transportation, public accommodations, communications, and access to state and local government programs and services.
└ Title I: Employment	Prohibits discrimination against qualified individuals with disabilities in all aspects of employment, including hiring, firing, promotions, training, and job assignments. Employers with 15 or more employees must provide reasonable accommodations, provided doing so does not cause undue hardship to the business.
└ Title II: Public Services	Ensures that individuals with disabilities have equal access to all programs, services, and activities provided by state and local governments, including public transportation, public education, and access to government buildings and services.
└ Title III: Public Accommodations and Commercial Facilities	Prohibits discrimination in public accommodations such as restaurants, hotels, theaters, shopping centers, and recreational facilities, requiring these settings to be accessible to individuals with disabilities and mandating the removal of barriers where feasible.
└ Title IV: Telecommunications	Requires telephone and internet companies to provide relay services and other telecommunications accommodations that allow individuals with hearing or speech disabilities to communicate by phone on an equal basis with others.
└ Title V: Miscellaneous Provisions	Contains additional provisions related to the ADA's implementation, including regulations, enforcement mechanisms, and the relationship between the ADA and other applicable federal and state laws.

Individuals with Disabilities Education Act (IDEA)

The **Individuals with Disabilities Education Act (IDEA)** is a federal law enacted in 1975, originally known as the Education for All Handicapped Children Act. It mandates that children with disabilities have the right to a free and appropriate public education (FAPE) in the least restrictive environment (LRE) that meets their individual needs. IDEA governs how states and public agencies provide early intervention, special education, and related services to children with disabilities from birth to age 21.

IDEA is structured around six main principles. The first principle, **Free Appropriate Public Education (FAPE),** guarantees that all children with disabilities have the right to receive a public education tailored to their needs at no cost to the parents. The education provided must meet the child's unique needs and prepare them for further education, employment, and independent living. The next is **the Least Restrictive Environment (LRE),** which states that Children with disabilities should be educated in the least restrictive environment, meaning they should be placed in general education classrooms with their non-disabled peers to the maximum extent appropriate. Special classes or separate schooling should occur only if the nature or severity of the disability prevents the child from making satisfactory progress in a general education setting, even with supplementary aids and services.

The **Individualized Education Program (IEP) section** requires that an IEP be developed for each child with a disability. The IEP is a written document that outlines the child's current levels of academic performance, specific educational goals, the services and supports the child will receive, and how progress will be measured. A team that includes the child's parents, teachers, and other relevant professionals develops the IEP collaboratively. The **Procedural Safeguards** section includes various procedural safeguards to protect the rights of children with disabilities and their parents. These safeguards ensure that parents have the right to be involved in their child's education, give informed consent for evaluations and placements, access educational records, and challenge school decisions through mediation or due process hearings. The **Parent and Student Participation** emphasizes the importance of parent and student involvement in the educational process. Parents have the right to participate in all meetings related to their child's identification, evaluation, and educational placement. As children with disabilities grow older, they are encouraged to participate in their IEP meetings and actively plan their education and transition to adulthood. The **Appropriate Evaluation** section requires that children suspected of having a disability receive a comprehensive and non-discriminatory evaluation to determine their eligibility for special education services. A team of qualified professionals must conduct the evaluation and must use a variety of assessment tools and strategies to gather relevant information about the child's educational needs.

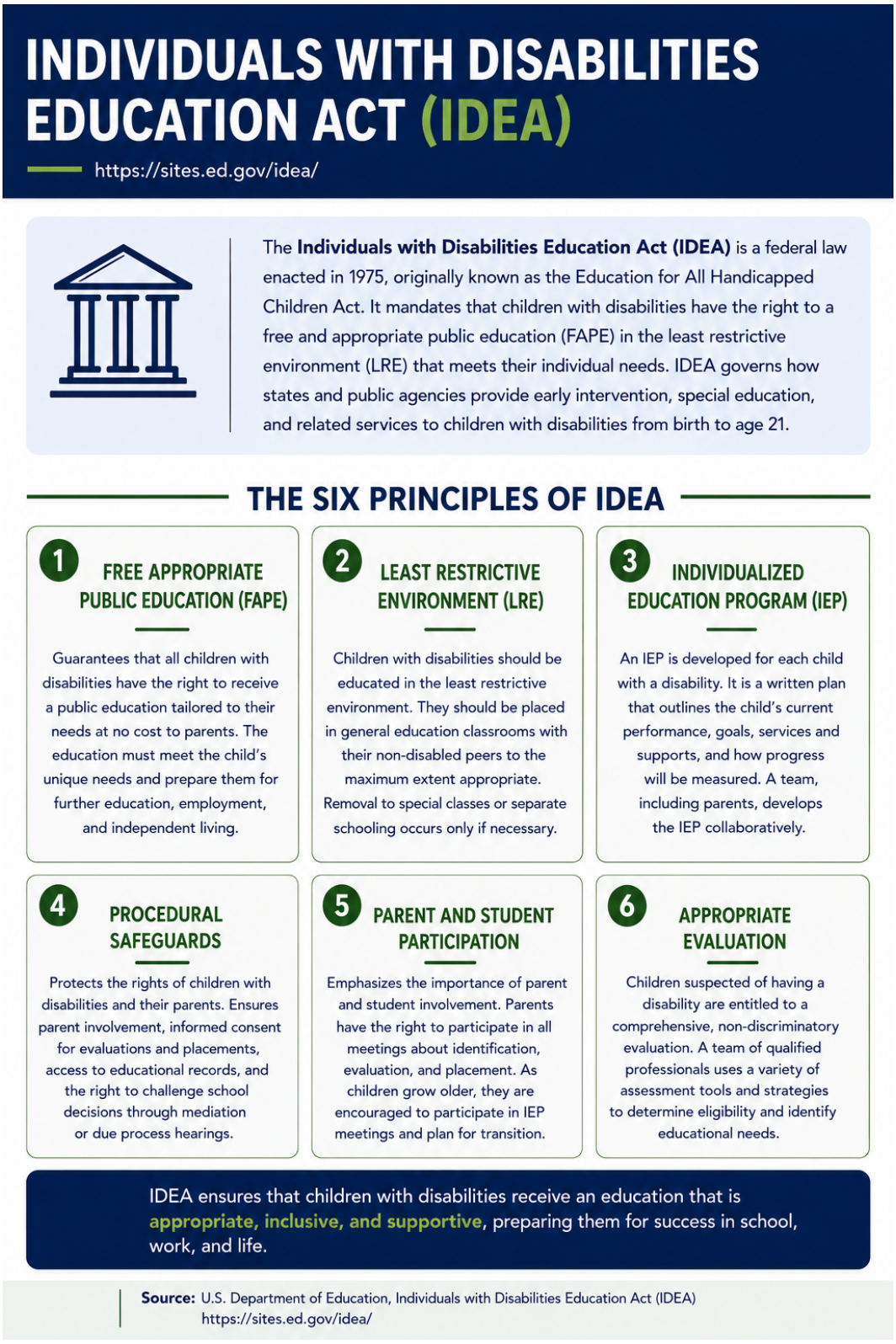


Figure A. *Individuals with Disabilities Education Act (IDEA): The Six Principles of IDEA* (Alford & Son, 2026)

Key Terms and Definitions

Term	Definition
Individuals with Disabilities Education Act (IDEA)	A federal law originally enacted in 1975 that mandates children with disabilities have the right to a free and appropriate public education (FAPE) in the least restrictive environment. IDEA governs how states and public agencies provide early intervention, special education, and related services to children with disabilities from birth to age 21.
└ Free Appropriate Public Education (FAPE)	A core IDEA principle guaranteeing that all children with disabilities have the right to receive a public education tailored to their individual needs at no cost to parents, designed to prepare them for further education, employment, and independent living.
└ Least Restrictive Environment (LRE)	An IDEA principle requiring that children with disabilities be educated alongside their non-disabled peers in general education settings to the maximum extent appropriate, with separate schooling occurring only when the nature or severity of the disability prevents satisfactory progress even with supplementary aids and services.
└ Individualized Education Program (IEP)	A written document required by IDEA for each child with a disability that outlines the child's current levels of academic performance, specific educational goals, services and supports to be received, and how progress will be measured, developed collaboratively by a team that includes parents, teachers, and relevant professionals.
└ Procedural Safeguards	Legal protections under IDEA that ensure parents have the right to be involved in their child's education, provide informed consent for evaluations and placements, access educational records, and challenge school decisions through mediation or due process hearings.
└ Parent and Student Participation	An IDEA principle emphasizing the importance of family and student involvement in the educational process, including the right of parents to participate in all meetings related to identification, evaluation, and placement, and encouraging older students to actively participate in their own IEP meetings and transition planning.
└ Appropriate Evaluation	An IDEA requirement that children suspected of having a disability receive a comprehensive, non-discriminatory evaluation conducted by qualified professionals using a variety of assessment tools and strategies to determine

	eligibility for special education services and identify the child's educational needs.

The Rehabilitation Act of 1973

The Rehabilitation Act of 1973 (*Rehabilitation Act of 1973*, n.d.) is a landmark civil rights law in the United States that prohibits discrimination based on disability in programs conducted by federal agencies, programs receiving federal financial assistance, federal employment, and the employment practices of federal contractors. It was the first significant piece of legislation to address the rights of individuals with disabilities, laying the groundwork for subsequent laws such as the Americans with Disabilities Act (ADA).

The Rehabilitation Act is divided into several sections, each addressing different aspects of disability rights and protections. The first section, **Section 501: Federal Employment,** requires federal agencies to provide equal employment opportunities for individuals with disabilities. It mandates affirmative action and nondiscrimination in the hiring, placement, and advancement of people with disabilities within federal government jobs. Federal agencies are required to develop affirmative action plans to recruit, hire, and retain employees with disabilities. **Section 503: Federal Contractors** prohibits federal contractors and subcontractors with contracts exceeding $10,000 from discriminating against individuals with disabilities in employment. It also requires these contractors to take affirmative action to employ and advance in employment qualified individuals with disabilities. This section is enforced by the Office of Federal Contract Compliance Programs (OFCCP). **Section 504: Nondiscrimination Under Federal Grants and Programs** is one of the most significant provisions of the Rehabilitation Act. It prohibits discrimination based on disability in any program or activity that receives federal financial assistance. This includes a wide range of entities such as schools, colleges, hospitals, and government agencies. Under Section 504, qualified individuals with disabilities cannot be excluded from, denied the benefits of, or subjected to discrimination under any program or activity receiving federal funds. Section 504 also requires that public schools and colleges provide students with disabilities appropriate accommodations and modifications to ensure equal access to education. This might include providing assistive technology, modifying curriculum, or ensuring physical accessibility to school facilities.

Section 505: Remedies and Attorneys' Fees provides the legal remedies and attorneys' fees available to individuals who prevail in discrimination claims brought under the Rehabilitation Act. This section ensures that individuals with disabilities who face discrimination in violation of the Act can seek enforcement in court. **Section 508: Electronic and Information Technology Accessibility** requires that all federal agencies' electronic and information technology (such as websites, software, and hardware) be accessible to people with disabilities. The goal of Section 508 is to eliminate barriers in information technology, ensure that new technologies are accessible, and provide individuals with disabilities opportunities to participate fully in the digital age. This section has broad implications for federal agencies, contractors, and vendors who provide electronic and information technology products and services to the federal government. Various federal agencies monitor and enforce compliance with Section 508, including the General Services Administration (GSA) and the U.S. Access Board.

The **Rehabilitation Act of 1973** is considered a foundational civil rights law for individuals with disabilities, setting a precedent for future legislation aimed at ensuring equal opportunities and protections. It was the first time that federal law recognized the rights of

individuals with disabilities as a protected class, similar to other groups protected by civil rights laws based on race, color, sex, and national origin. The Act's provisions, particularly Section 504, were a critical step toward inclusion and accessibility, requiring institutions and organizations that receive federal funding to make reasonable accommodations and eliminate discriminatory practices. The implementation of the Rehabilitation Act has led to increased access to education, employment, and public services for people with disabilities, and it remains a cornerstone of disability rights law in the United States.

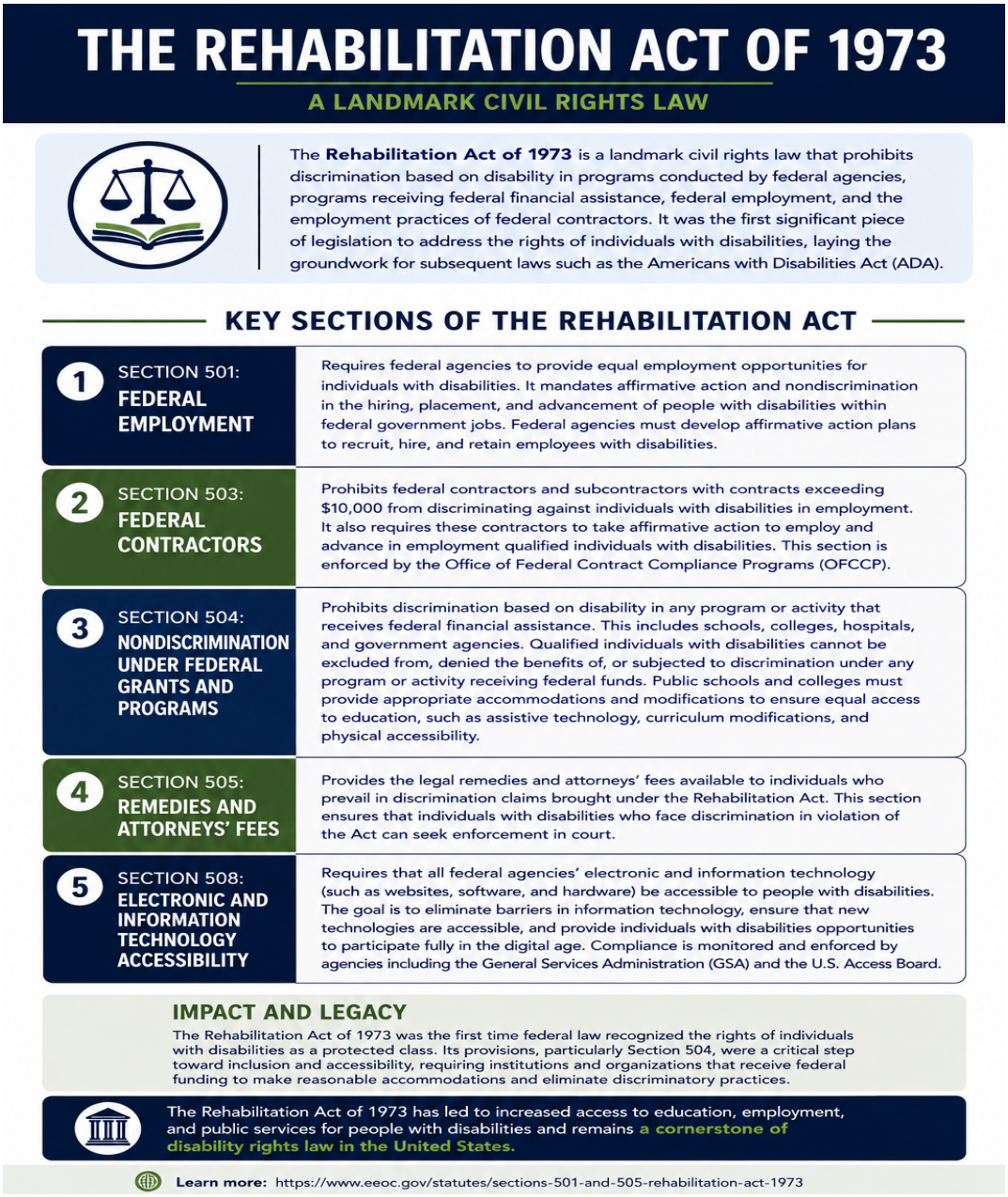


Figure B. *The Rehabilitation Act of 1973: Key Sections, Impact, and Legacy* (Alford & Son, 2026)

Key Terms and Definitions

Term	Definition
Rehabilitation Act of 1973	A landmark federal civil rights law that prohibits discrimination based on disability in programs conducted by federal agencies, programs receiving federal financial assistance, federal employment, and the employment practices of federal contractors. It was the first significant legislation to address the rights of individuals with disabilities, laying the groundwork for subsequent laws such as the ADA.
└ Section 501: Federal Employment	Requires federal agencies to provide equal employment opportunities for individuals with disabilities, mandating affirmative action and nondiscrimination in the hiring, placement, and advancement of people with disabilities within federal government positions.
└ Section 503: Federal Contractors	Prohibits federal contractors and subcontractors with contracts exceeding $10,000 from discriminating against individuals with disabilities in employment, and requires affirmative action to employ and advance qualified individuals with disabilities. Enforced by the Office of Federal Contract Compliance Programs (OFCCP).
└ Section 504: Nondiscrimination Under Federal Grants and Programs	Prohibits discrimination based on disability in any program or activity receiving federal financial assistance, including schools, colleges, hospitals, and government agencies. Requires public schools and colleges to provide appropriate accommodations and modifications, such as assistive technology, curriculum modifications, and physical accessibility, to ensure equal access to education.
└ Section 505: Remedies and Attorneys' Fees	Provides the legal remedies and attorneys' fees available to individuals who prevail in discrimination claims brought under the Rehabilitation Act, ensuring that individuals with disabilities who experience discrimination can seek judicial enforcement of their rights.
└ Section 508: Electronic and Information Technology Accessibility	Requires all federal agencies' electronic and information technology, including websites, software, and hardware, to be accessible to people with disabilities. The goal is to eliminate barriers in information technology and ensure individuals with disabilities can fully participate in the digital age. Monitored by the General Services Administration (GSA) and the U.S. Access Board.

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Rehabilitation Act (1973—H.R. 8070). (n.d.). *GovTrack.Us*. Retrieved April 18, 2023, from https://www.govtrack.us/congress/bills/93/hr8070

The Social Model of Disability

The **Social Model of Disability** is a framework for understanding disability that emphasizes the role of societal structures, attitudes, and environmental barriers in disabling individuals rather than focusing solely on the individual's physical or mental impairments (Oliver, 1990, 2013). This model shifts the focus from the individual's condition to the broader context in which they live, arguing that it is society's failure to accommodate diversity that results in disability rather than the individual's impairment itself (Kattari et al., 2017).

The Social Model of Disability includes several key concepts. One distinction is between impairment and disability. **Impairment** refers to an individual's actual physical, mental, or sensory condition, such as blindness, deafness, mobility limitations, or cognitive differences. **Disability** is not inherent in the individual but results from social and environmental barriers that prevent full participation in society. For example, a person with a mobility impairment is not "disabled" by their inability to walk but by the lack of accessible public transportation or buildings.

Another distinction is Social and Environmental Barriers**,** which the social model identifies as several types of barriers that disable people with impairment. One barrier is **attitudinal,** which includes negative attitudes, stereotypes, and prejudices that result in discrimination, social exclusion, and a lack of opportunities for people with disabilities. Examples include assumptions about what people with disabilities can or cannot do. **Environmental barriers** are physical barriers in the built environment that prevent access, such as stairs without ramps, lack of elevators, inaccessible public transportation, and inadequate signage for people with visual impairments. **Institutional Barriers** are systemic issues within policies, laws, and practices that disadvantage people with disabilities, such as inflexible work practices, inaccessible education systems, and discriminatory laws or regulations. **Communication barriers** affect the exchange of information, such as a lack of sign language interpreters, inaccessible websites, or the absence of materials in alternative formats, such as Braille.

Rather than locating disability in individuals, this framework centers on **empowerment and agency,** supporting the idea that individuals with disabilities should be active participants in society, with the same rights, responsibilities, and opportunities as others. This model promotes the idea that society should adapt to meet the needs of all its members, rather than expecting individuals with disabilities to conform to existing structures.

Grounded in the principles of **social justice,** equality, and the full inclusion of people with disabilities in all aspects of society. The focus on social justice and inclusion **c**hallenges discriminatory practices and calls for changes in laws, policies, and cultural norms to remove barriers and create a more inclusive society. From this approach, disability is a matter of human rights, and individuals with disabilities should have equal access to education, employment, healthcare, and community life.

In contrast, the **Medical Model of Disability** views disability primarily as a medical issue that resides within the individual. The medical model focuses on diagnosis, treatment, and rehabilitation, often aiming to "fix" or "cure" the individual's impairment (Brosco, 2010; Kattari et al., 2017). However, the social model critiques this approach for **pathologizing disability** and

for ignoring the social, environmental, and attitudinal factors that contribute to disabling experiences. Critique of the medical model argues that the medical model reinforces negative stereotypes and fails to recognize the potential of people with disabilities to live fulfilling lives when supported by an inclusive and accommodating society (Brosco, 2010).

Applications for the Social Model of Disability

Across global contexts, this framework has influenced **disability rights movement**s and the development of disability-related legislation worldwide. **Policy and legislation** have played a key role in shaping laws such as the **Americans with Disabilities Act (ADA)** in the United States, the Equality Act in the United Kingdom, and the **United Nations Convention on the Rights of Persons with Disabilitie**s (**UNCRPD**). These laws reflect the social model's emphasis on removing barriers and ensuring equal rights and opportunities for people with disabilities.

Influence is also evident in the design of public spaces, buildings, transportation, and technology to make them more accessible to everyone, regardless of ability. Guided by inclusive and **universal design** principles, **inclusive design** and accessibility aim to create environments that are usable by all people, to the greatest extent possible, without the need for adaptation or specialized design. Accessible design practices influenced by the social model include wheelchair ramps, Braille signage, video captioning, and website accessibility features.

This approach also supports **inclusive education** systems where students with disabilities are educated alongside their non-disabled peers. Emphasis is placed on classroom adaptations and accommodations, such as assistive technology, individualized learning plans, and teacher training to support diverse learning needs. From this perspective, curriculum and assessment practices should be flexible and inclusive, allowing all students to demonstrate their knowledge and skills in ways that work best for them.

Employment practices have also been reshaped through this lens, with a focus on removing barriers to work for people with disabilities. **Workplace inclusion** provides **reasonable accommodation**, such as flexible work schedules, adaptive equipment, and accessible workplaces, and helps combat discrimination in hiring, promotion, and retention. It also encourages policies that support work-life balance, job training, and career development opportunities for employees with disabilities.

Finally**, disability advocacy and activism** have been a driving force behind disability advocacy and activism, empowering people with disabilities to challenge societal norms and demand equal treatment. Disability rights organizations and movements often use the social model as a foundation for their work, advocating for systemic change and greater inclusion.

Limitations and Critiques of the Social Model

While the social model has been highly influential and transformative in theory, policy, and practice, it has also faced criticism. Some critics argue that the social model **oversimplifies** the experience of disability by focusing almost exclusively on societal barriers and downplaying the role of individual impairments. They suggest that a more nuanced understanding of disability

should recognize the interplay between social factors and the physical or mental aspects of impairment.

Questions have also been raised about the neglect of personal experiences, which are best addressed through lived experience. For some individuals, disability involves chronic pain, fatigue, and other functional limitations that cannot be entirely attributed to social barriers alone. When these lived realities are not considered, the framework appears disconnected from the everyday challenges faced by individuals with specific disabilities.

Some scholars and activists advocate a more integrated approach that combines elements of the social and medical models with other perspectives, such as the **biopsychosocial model**, which considers the biological, psychological, and social factors influencing health and disability.

These critiques do not diminish the value of the social model but instead reveal the importance of applying it with nuance and flexibility. It is essential to recognize structural barriers and life experiences using a **holistic approach** to understanding disability. The social model remains a critical tool in advancing equity, inclusion, and person-centered practice when integrated thoroughly, and will address the complex and diverse realities faced by individuals with disabilities.

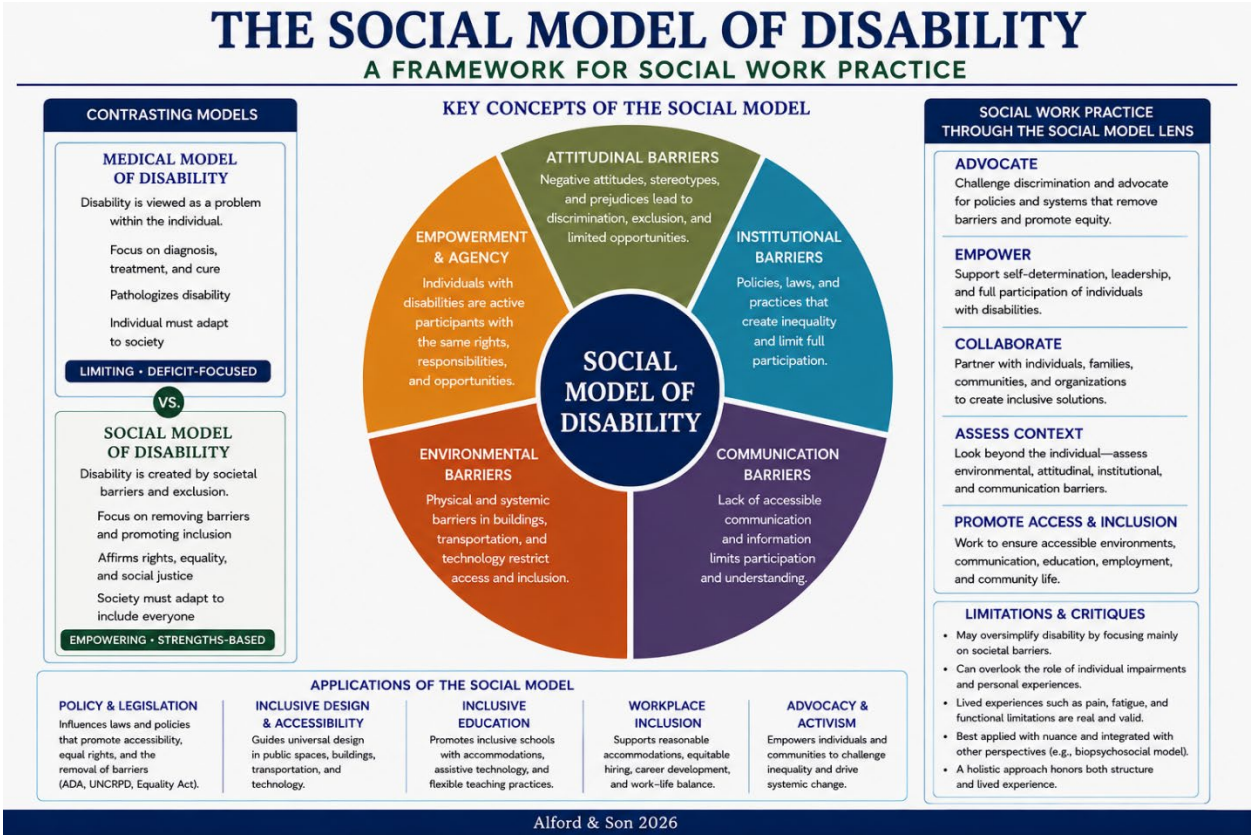


Figure C. *The Social Model of Disability: A Framework for Social Work Practice* (Alford & Son, 2026)

Key Terms and Definitions

Term	Definition
Stigma Process Model	A cyclical framework that explains how stigma is produced and sustained through four interconnected mechanisms: ignorance, prejudice, discrimination, and demoralization. Applied to individuals with I/DD, the model illustrates how societal misunderstanding and negative attitudes translate into harmful behaviors and internalized harm.
└ **Ignorance**	A lack of accurate knowledge or understanding about intellectual and developmental disabilities that contributes to stereotypes, fear, and misinformation within communities and institutions.
└ **Prejudice**	Negative attitudes or beliefs about individuals with I/DD arising from misinformation, bias, or social conditioning that position disability as a deficiency or deviation from the norm.
└ **Discrimination**	Behavioral expressions of stigma involving unequal treatment, exclusion, or denial of rights and opportunities at interpersonal, institutional, or systemic levels.
└ **Demoralization**	The internal psychological impact of stigma on individuals with I/DD, including reduced self-worth, confidence, agency, and social participation resulting from chronic exposure to stigmatizing attitudes and behaviors.
Multi-Level Model of Stigmatization	A framework developed by Werner and Scior identifying stigma as occurring across personal, interpersonal, community, and societal levels, emphasizing its systemic and structural nature and the need for multilevel intervention.
└ **Personal Level**	Internalized stigma experienced by individuals with I/DD, including shame, low self-esteem, and reduced self-determination resulting from absorbing negative societal beliefs about disability.
└ **Interpersonal Level**	Stigma experienced in direct interactions with others, including discriminatory behaviors, social exclusion, and negative assumptions made by peers, professionals, and community members.
└ **Community Level**	Stigma embedded in organizational and community practices, including exclusionary policies, inaccessible environments, and limited opportunities for participation in community life.
└ **Societal Level**	Structural stigma embedded in laws, policies, media representations, and cultural norms that systematically

	disadvantage individuals with I/DD across institutions and social systems.
Affiliate Stigma	The internalization of societal stigma by family members of individuals with I/DD, resulting in shame, guilt, self-blame, or social withdrawal due to their association with a stigmatized person.
Ableism	A system of beliefs and social practices that positions non-disabled people as the norm and devalues, excludes, or discriminates against individuals with disabilities, often manifesting in institutional structures, cultural attitudes, and interpersonal interactions.
Courtesy Stigma	Stigma experienced by family members, caregivers, or close associates of individuals with I/DD due to their relationship or association with a stigmatized person, leading to social exclusion or negative treatment.
Dehumanization	The process of denying the full humanity, worth, or agency of individuals with I/DD, often used historically to justify exclusion, institutionalization, neglect, or abuse.
Disability Stigma	Negative beliefs, attitudes, and practices directed toward individuals with disabilities that result in social exclusion, lowered expectations, and structural inequities across life domains.
Eugenics	A historical and ideological movement promoting beliefs of biological superiority that justified forced sterilization, segregation, and dehumanization of people with disabilities, leaving lasting impacts on disability policy and public attitudes.
Internalized Stigma	The process by which individuals with I/DD absorb and accept negative societal beliefs about disability, negatively impacting self-esteem, identity formation, and mental health outcomes.
Structural Vulnerability	Heightened exposure to harm resulting from social, economic, and political systems that limit access to resources, protections, and opportunities for individuals with I/DD.

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Applying the Social Model of Disability in Social Work Practice

The Social Model of Disability (refer to Figure D) is critically important for social work practice because it aligns with social work's core values and principles, including social justice, empowerment, and advocacy. From this framework, social workers can understand and address systemic issues affecting individuals with disabilities by emphasizing the role of societal and environmental barriers in creating disability.

The social model is essential to social work practice for several key reasons. First, it focuses on social justice and advocacy, promoting equality and highlighting the importance of creating an equitable society where people with disabilities have the same rights and opportunities as others. Social workers committed to promoting social justice can use this model to advocate for policies, practices, and services that remove barriers and ensure equal access for individuals with disabilities. When viewing disability as a result of societal barriers rather than individual deficits, the social model empowers social workers to challenge **discriminatory practices** and attitudes across settings such as education, employment, healthcare, and public services. This advocacy helps to dismantle systemic discrimination and promote inclusivity.

The next is the **empowerment of individuals with disabilities,** in which the social model encourages social workers to focus on the strengths, capabilities, and aspirations of individuals with disabilities, rather than viewing them through the lens of their impairments. This **client-centered approach** aligns with the social work principle of client empowerment, enabling individuals with disabilities to take control of their lives, make their own decisions, and pursue their goals.

Social workers can use the social model to support individuals with disabilities' **self-determination**. This approach ensures that their voices are heard and respected in decision-making processes that affect their lives. This includes advocating for their right to choose where they live, work, and access services, and supporting their participation in community life.

From a **holistic understanding of disability** perspective, the social model encourages social workers to consider practice in the broader social, economic, and cultural context in which disability occurs. This holistic understanding helps social workers to address the environmental and societal factors that contribute to disability, such as inaccessible infrastructure, discriminatory policies, and a lack of support services. From this perspective, social workers explore how disability intersects with other factors, such as race, gender, socioeconomic status, and age. This **intersectional approach** helps social workers to develop more nuanced and tailored interventions that address the unique challenges faced by individuals with multiple marginalized identities

Guiding policy and program development encourages social workers to use the social model to advocate for **inclusive policies** that promote accessibility, inclusion, and participation for people with disabilities. This includes pushing for legislation that mandates accessible public spaces, inclusive education, and equitable healthcare services.

Informed by the social model, social workers can design and implement programs that remove barriers and create enabling environments. For example, program design can help social workers develop community-based programs that provide support and resources to help individuals with disabilities live independently and participate fully in society.

The social model challenges the traditional medical model of disability, which views disability as an individual problem that needs to be fixed or cured. Social workers can use the social model to shift the focus from "fixing" individuals to addressing the societal barriers that restrict their opportunities and participation.

Adopting the social model can help social workers collaborate in practice with other professionals (such as educators, urban planners, and healthcare providers) to create more inclusive and supportive environments. This interdisciplinary approach ensures that interventions are comprehensive and address the multiple dimensions of disability.

Holistic Family Support recognizes that the well-being of individuals with disabilities is closely linked to the support they receive from their families and communities. Social workers can use this model to provide holistic support to families and caregivers, helping them navigate barriers, access resources, and advocate for their loved ones. Focusing on societal barriers rather than individual deficits, the social model helps to reduce the stigma associated with disability. Social workers can educate families, caregivers, and communities about the social model, encouraging a more positive and inclusive understanding of disability.

The social model aligns with the **Ethics of Care**, a framework that emphasizes the importance of relationships, interdependence, and the moral responsibility to care for one another (Banks, 2020; Botes, 2000). From this perspective, social workers can advocate for care practices that respect the dignity, autonomy, and rights of individuals with disabilities.

Using a **person-centered ethics of care approach**, where the individual with a disability is at the center of all decisions and interventions (Alford & Son, 2025). Social workers can use this model to ensure that services and supports are tailored to each person's unique needs, preferences, and goals, rather than applying a one-size-fits-all solution.



Figure D. *Applying the Social Model of Disability in Social Work Practice: Integrating Social Justice, Empowerment, and Person-Centered Ethics of Care* (Alford & Son, 2026)

Case Scenario: Applying the Person-Centered Ethics of Care

Case Scenario: Maria

Maria is a 28-year-old woman with a moderate intellectual disability who lives with her aging parents, Rosa and Eduardo, in a close-knit immigrant community in New York City. Maria's parents are her primary caregivers and have managed her daily routines, medical appointments, and community activities for her entire life. Rosa and Eduardo, both in their late sixties, are beginning to experience their own health challenges — Rosa was recently diagnosed with early-stage Parkinson's disease, and Eduardo has been managing Type 2 diabetes with increasing complications.

Maria attends a day habilitation program five days a week, where she has developed strong friendships and participates in art and cooking classes she deeply enjoys. She communicates primarily through a combination of verbal speech and a picture-based communication system. While Maria has clear preferences and opinions about her daily life, she dislikes loud environments, loves cooking, and wants to work in a café someday, she has rarely been included in formal planning conversations, as her family and providers have historically made decisions on her behalf.

A social worker named Diana is assigned to Maria's case through OPWDD following a referral triggered by Rosa's health decline. During her initial assessment, Diana observes that Maria becomes visibly anxious when her parents discuss the future, and that Rosa and Eduardo have not yet engaged in any future planning regarding housing, guardianship, or care coordination. The family has no formal supports beyond the day habilitation program, and Rosa has expressed reluctance to seek outside help, citing cultural values around family responsibility and a distrust of government systems rooted in their immigration experience.

Framework Connection: PC-EoC in Social Work Practice

This case illustrates the core principles of the Person-Centered Ethics of Care Model (Alford & Son, 2025). Maria's preferences and goals are central to the planning process, yet systemic barriers, including limited family planning, cultural factors, and historical exclusion from decision-making, prevent her from fully exercising her autonomy and self-determination. Diana's role requires balancing relational care, cultural responsiveness, advocacy, and interdisciplinary coordination across multiple systems.

Discussion Questions

Using the Person-Centered Ethics of Care framework, consider the following:

1. How should Diana approach the tension between Rosa and Eduardo's cultural values around family caregiving and Maria's right to autonomy and self-determination? What culturally responsive strategies might she employ?

2. Maria has rarely been included in planning conversations. How would you use person-centered planning principles to ensure her voice, preferences, and goals guide the development of her care plan?
3. What systemic and environmental barriers might Maria face in pursuing her goal of working in a café, and how could Diana advocate across micro, mezzo, and macro levels to address them?
4. How does the PC-EoC Model help Diana understand the interdependence among Maria, her parents, the day habilitation program, and the broader service system? Where might gaps or breakdowns in coordination occur?
5. As Rosa's health declines, what proactive planning conversations should Diana initiate with the family, and how can she support both Maria's future stability and the well-being of her aging caregivers?

Key Terms and Definitions

Term	Definition
Empowerment of Individuals with Disabilities	A client-centered approach that focuses on the strengths, capabilities, and aspirations of individuals with disabilities, enabling them to take control of their lives, make their own decisions, and pursue their goals.
Ethics of Care	A values framework that emphasizes relationships, interdependence, and the moral responsibility to care for one another, guiding social workers to advocate for care practices that respect the dignity, autonomy, and rights of individuals with disabilities (Banks, 2020; Botes, 2000).
Guiding Policy and Program Development	The use of the social model to advocate for inclusive legislation and design community-based programs that remove barriers, promote accessibility, and support independent living and full community participation.
Holistic Family Support	A social work approach that recognizes the well-being of individuals with disabilities as closely linked to family and community support, helping families navigate barriers, access resources, and advocate for their loved ones.
Holistic Understanding of Disability	A perspective that considers disability within its broader social, economic, and cultural context, addressing environmental and societal factors while recognizing how disability intersects with race, gender, socioeconomic status, age, and other identities.
Interdisciplinary Collaboration	A practice approach in which social workers partner with educators, urban planners, healthcare providers, and other professionals to create inclusive and supportive environments that address the multiple dimensions of disability.
Person-Centered Ethics of Care Approach	An individualized framework in which the person with a disability is at the center of all decisions and interventions, ensuring that services and supports are tailored to each person's unique needs, preferences, and goals rather than applying a standardized approach (Alford & Son, 2025).
Self-Determination	The right of individuals with disabilities to have their voices heard and respected in decision-making processes, including choices about where they live, work, and access services, and how they participate in community life.

Social Justice and Advocacy	A foundational social work commitment to promoting equality and removing systemic barriers that prevent individuals with disabilities from accessing the same rights, opportunities, and services as others.

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Alford, S., & Son, E. (2025). Teaching social work students using a person-centered ethics of care model approach to working with individuals with intellectual/developmental disabilities and their caregivers. *Social Work Education*, 1–16. https://doi.org/10.1080/02615479.2025.2560657

Person-Centered Ethics of Care for Individuals with Intellectual Disabilities

The **Person-Centered Ethics of Care** is a framework that focuses on the holistic well-being of individuals, particularly those with intellectual disabilities (ID). It emphasizes the importance of empathy, relationships, and individualized support, ensuring that care is tailored to each person's unique needs, preferences, and circumstances. This approach is deeply rooted in the ethics of care, a moral theory that prioritizes the interdependence of people and the responsibility to care for one another (Banks, 2020; Botes, 2000).

For individuals with intellectual disabilities, the Person-Centered Ethics of Care promotes dignity, respect, and autonomy while recognizing the crucial roles of caregivers, families, and communities in supporting their well-being. (Alford & Son, 2025). This framework challenges traditional models of care that often focus solely on medical or institutional approaches, advocating instead for a more inclusive, compassionate, and responsive system of support.

Key Principles of Person-Centered Ethics of Care

Central to the Person-Centered Ethics of Care is the recognition of the inherent dignity of every individual, regardless of their intellectual abilities. **Respect for Individual Dignity** in care practices must affirm the value and worth of individuals with intellectual disabilities, ensuring they are treated with respect and consideration.

Even when individuals have significant intellectual disabilities, the ethics of care framework emphasizes their right to **autonomy and self-determination**. Autonomy means supporting individuals in making choices about their lives, to the greatest extent possible, and involving them in decisions that affect their care and well-being.

The Person-Centered Ethics of Care acknowledges that human beings are inherently relational, and that the quality of care is deeply influenced by the relationships between caregivers, individuals with disabilities, and their families. **Relational Care** practices should foster positive, supportive, and trusting relationships that enhance the individual’s sense of belonging and security.

Rather than viewing individuals with intellectual disabilities as isolated or dependent, this approach recognizes the interdependence of all people. **Interdependence** emphasizes the mutual responsibilities among individuals, caregivers, families, and communities, creating a network of support that upholds the well-being of individuals with disabilities.

At the heart of this approach is **person-centered planning**, in which care and support are tailored to the individual's unique needs, goals, and preferences. This involves understanding the whole person, considering their physical, emotional, social, and spiritual needs, and developing care plans that reflect their personal identity, aspirations, and circumstances.

The ethics of care framework advocates for addressing all aspects of an individual’s life, not just their medical or physical needs. This holistic approach includes fostering social

connections, supporting mental health, providing opportunities for meaningful engagement, and ensuring access to education, employment, and community activities.

The Person-Centered Ethics of Care aims to empower individuals with intellectual disabilities by building their confidence, skills, and opportunities for self-expression. **Empowerment and Advocacy** include supporting their involvement in decision-making processes, advocating for their rights, and providing them with the tools they need to live as independently as possible.

Social workers, caregivers, and families play a crucial role in advocating for the rights and needs of individuals with intellectual disabilities. **Advocacy** involves challenging societal barriers, promoting inclusive policies, and ensuring that individuals receive the support and resources they need to thrive.

Care practices should be responsive to the changing needs, circumstances, and preferences of individuals with intellectual disabilities. **Responsive and flexible care** requires a flexible approach that can adapt to new challenges, support transitions, and respond to crises in a timely and compassionate manner. The ethics of care emphasizes the importance of continuity in relationships and care practices to ensure consistency and reliability, and that long-term support helps build trust and stability, which are especially important for individuals with intellectual disabilities who may be vulnerable to disruptions in care.

Community Integration is a central component of the Person-Centered ethics of care, promoting inclusion of individuals with intellectual disabilities in all aspects of community life. This means creating opportunities for social participation, fostering connections with peers, and ensuring that individuals are not isolated or segregated from society. Supporting social connection and peer engagement is essential to well-being and quality of life for individuals with intellectual disabilities. Equally important is the commitment to **cultural sensitivity** within care practices. In this sense, care should be culturally sensitive, respecting the individual's background, traditions, and values. This includes understanding how cultural factors may influence the individual's experience of disability and ensuring care is inclusive and respectful of diversity, and include the following:

- **Ethical Deliberation:** The ethics of care approach requires ongoing ethical deliberation, where caregivers, families, and social workers carefully consider the moral implications of their decisions. This involves reflecting on how care practices affect the individual's dignity, autonomy, and well-being, and making decisions that align with the individual's values and best interests.

- **Collaborative Decision-Making:** Ethical decision-making should be a collaborative process that involves the individual with an intellectual disability, their family, and other key stakeholders. This ensures that decisions are made transparently and that the perspectives and preferences of all involved are considered.

Applications of Person-Centered Ethics of Care in Social Work Practice

Developing **Individualized Care Plans** is a key application of the Person-Centered Ethics of Care. Social workers are encouraged to create care plans tailored to the specific needs, strengths, and goals of individuals with intellectual disabilities. These plans should be developed in collaboration with the individual, their family, and other care providers, ensuring that they reflect the person's unique identity and aspirations, and that their lived experiences guide decision-making.

Equally critical is supporting **families and caregivers**, who often serve as the primary sources of care and advocacy. Families and caregivers play a crucial role in the lives of individuals with intellectual disabilities. Social workers can support them by providing resources, education, and counseling that help them navigate the challenges of caregiving. This framework explicitly recognizes the importance of supporting caregivers' well-being and their essential role in providing care.

Advocacy efforts grounded in this approach focus on advancing inclusive policies and services that uphold dignity, autonomy, and access. Social workers can advocate for policies and services that align with the Person-Centered Ethics of Care, ensuring that individuals with intellectual disabilities have access to the support they need to live fulfilling lives. This includes promoting inclusive education, employment opportunities, accessible healthcare, and community-based services that uphold the dignity and rights of individuals with disabilities.

Facilitating Community Integration represents another core application of the ethics of care approach, which encourages social workers to facilitate the integration of individuals with intellectual disabilities into their communities. This can involve creating opportunities for social participation, organizing inclusive events, and working with community organizations to ensure that their services are accessible and welcoming to people with disabilities.

Finally, **promoting ethical reflection and practice** remains an ongoing responsibility of person-centered practice. Social workers are encouraged to regularly examine the consequences of their professional decisions, using this framework to guide their ethical reflection and practice. This involves regularly assessing the impact of their choices and actions on the well-being of individuals with intellectual disabilities, seeking input from the individuals themselves, and engaging in continuous learning and professional development to enhance their care practices.

The IDD Person-Centered Ethics of Care (PC-EoC) model (Alford & Son, 2025), in Figure 1, illustrates the complex, dynamic, and interdependent needs of individuals with intellectual and developmental disabilities (I/DD) and their caregivers. Grounded in relational ethics, this model emphasizes attentiveness, responsibility, responsiveness, and respect for autonomy, recognizing that care does not occur in isolation but within layered systems of family, service providers, institutions, and policy environments.

The model simultaneously acknowledges the emotional, physical, and structural demands placed on caregivers, centering on the lived experiences, preferences, and evolving capacities of individuals with I/DD. Rather than viewing care as a one-way service, the model conceptualizes care as a reciprocal, adaptive process that requires flexibility across the lifespan. Integrating

person-centered planning, cultural humility, and ethical responsiveness, the PC-EoC model helps social workers use a **holistic** framework to support quality of life, dignity, and sustainable care relationships for individuals with I/DD and their support staff.

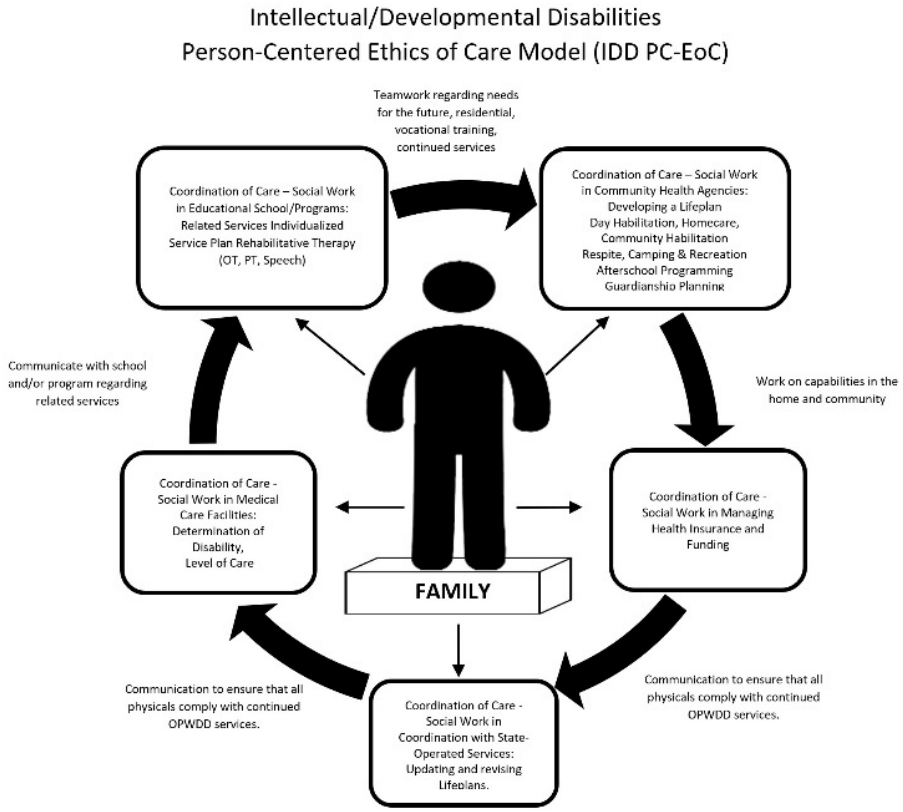


Figure 2. *IDD Person-Centered Ethics of Care (PC-EoC) Model (Alford & Son, 2025)*

Pause & Reflect

- Looking at the model, how do the different systems (education, medical, social services, and community agencies) work together to support the individual and family? Where might breakdowns in coordination occur?
- Why is the family placed at the center of this model? How does this reflect person-centered and ethical social work practice?
- How might a lack of communication between systems (e.g., school, healthcare, OPWDD services) impact outcomes for individuals with I/DD?
- In what ways does this model reflect a holistic approach rather than focusing on a single service or system?
- As a social worker, how would you ensure that services remain coordinated, responsive, and aligned with the individual's needs across the lifespan?

Challenges and Considerations

While the Person-Centered Ethics of Care provides a comprehensive framework for supporting individuals with intellectual disabilities, its implementation presents several practical and ethical

challenges. One of the most complex is **balancing autonomy and protection.** Social workers must navigate the delicate balance between respecting an individual's autonomy and ensuring their safety and well-being. This can be particularly challenging when individuals with intellectual disabilities are at risk of harm or exploitation.

Practice restraining also shapes the extent to which person-centered care can be implemented. Implementing person-centered care requires adequate resources, including time, funding, and trained personnel. In settings where resources are limited, social workers must advocate for these resources while also finding creative solutions to provide quality care within existing constraints. Finally, **cultural competence** is an additional ongoing responsibility within this framework. Social workers must be culturally competent and understand how cultural beliefs and values influence the experience of disability and care. This requires ongoing education and a commitment to inclusive and culturally sensitive practices.

Key Terms and Definitions

Term	Definition
Person-Centered Ethics of Care (PC-EoC) Model	A theoretical framework developed by Alford and Son (2025) that integrates person-centered planning with the ethics of care to guide holistic, rights-affirming social work practice with individuals with intellectual and developmental disabilities across the lifespan.
└ Educational Component	The PC-EoC Model's educational dimension focused on ensuring access to appropriate educational services, transition planning, vocational supports, and lifelong learning opportunities that promote independence and community participation.
└ Medical Component	The dimension of the PC-EoC Model focused on coordinating access to high-quality, accessible healthcare and specialty services that address the unique developmental and medical needs of individuals with I/DD.
└ Governmental and Financial Component	The dimension of the PC-EoC Model concerned with navigating state and federal funding mechanisms, public benefits, Medicaid systems, and OPWDD services to secure the resources necessary for long-term stability and independence.
└ Social Services Component	The dimension of the PC-EoC Model that highlights the social worker's role in connecting individuals with I/DD and their families to community resources, family care programs, respite services, and integrated support networks.
└ Community Engagement Component	The dimension of the PC-EoC Model focused on facilitating access to recreational programs, employment opportunities, social supports, housing resources, and civic participation to ensure active, meaningful community membership.
Advocacy	The active effort by social workers, caregivers, and families to promote the rights and needs of individuals with I/DD by challenging systemic barriers, influencing inclusive policies, and securing the resources necessary for full community participation and quality of life.
Autonomy and Self-Determination	The right of individuals with I/DD to make choices about their own lives and to be meaningfully involved in decisions affecting their care and well-being, to the greatest extent possible.

Care Coordination	The process of organizing, integrating, and monitoring services across multiple providers and systems to ensure comprehensive, person-centered, and continuous care for individuals with I/DD.
Collaborative Decision-Making	A transparent, inclusive process in which the individual with I/DD, their family, and key stakeholders jointly participate in decisions affecting care, ensuring that all perspectives are heard and respected.
Community Integration	The active inclusion of individuals with I/DD in all aspects of community life, including social participation, peer engagement, and access to civic and cultural opportunities.
Cultural Responsiveness	The ongoing commitment to delivering services that are respectful of and responsive to cultural differences, values, beliefs, language needs, and lived experiences of individuals with I/DD and their families.
Dignity and Respect	A foundational principle of the PC-EoC Model affirming the inherent worth and value of every individual with I/DD, requiring that all care practices treat persons with genuine consideration and respect regardless of ability.
Ethical Deliberation	An ongoing reflective process in which caregivers, families, and social workers carefully consider the moral implications of care decisions in relation to the individual's dignity, autonomy, and well-being.
Holistic Care	An approach that addresses all dimensions of an individual's life — physical, emotional, social, and spiritual — including access to education, employment, mental health support, and community engagement.
Individual Dignity	The inherent worth and value of every person with I/DD, which must be affirmed and protected in all care practices, regardless of the level or nature of the individual's disability.
Interdependence	The recognition that all people are mutually reliant on one another, emphasizing shared responsibilities among individuals, caregivers, families, and communities in supporting the well-being of individuals with I/DD.
Person-Centered Planning	A collaborative planning process in which care and support are tailored to an individual's unique needs, goals, preferences, and personal identity, developed in partnership with the individual, family, and care providers.
Relational Care	An approach to care that recognizes the centrality of relationships between caregivers, individuals with

	disabilities, and families, fostering trust, belonging, and security as foundations of quality support.
Responsive and Flexible Care	A care approach that adapts to the changing needs, circumstances, and preferences of individuals with I/DD, including timely and compassionate responses to transitions and crises.
Structural Awareness	The recognition that systemic inequities, social determinants of health, and structural barriers significantly shape the experiences and outcomes of individuals with I/DD, requiring advocacy at multiple levels of practice.

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Chapter 1: Introduction to Intellectual and Developmental Disabilities

Learning Objectives

- Define and distinguish intellectual and developmental disabilities (I/DD), including intellectual disability and autism spectrum disorder, based on cognitive functioning, adaptive behavior, developmental onset, and severity.
- Explain the purpose of formal diagnosis of I/DD in determining service eligibility, protecting rights, and ensuring access to supports across educational, healthcare, and social service systems.
- Analyze screening and assessment processes used to identify I/DD, including IQ testing, adaptive functioning measures, behavioral assessments, and factors that may contribute to diagnostic bias or inequity.
- Examine epidemiological trends and health disparities related to I/DD, including prevalence, demographic patterns, diagnostic overshadowing, and systemic barriers to healthcare across the lifespan.
- Apply person- and family-centered ethics of care to social work practice by identifying the roles of coordinated systems and demonstrating culturally responsive assessment, care planning, and advocacy.

Table 1. *Alignment of Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	Competency Alignment	Rationale
Define and distinguish intellectual and developmental disabilities (I/DD)	*Competency 1:* Demonstrate Ethical and Professional Behavior	Establishes foundational professional knowledge and appropriate use of terminology
Explain the purpose and significance of the formal diagnosis of I/DD	*Competency 5:* Engage in Policy Practice	Diagnosis directly affects eligibility, access, and rights within service and policy systems
Analyze diagnostic criteria, screening, and assessment processes	*Competency 7:* Assess Individuals, Families, Groups, Organizations, and Communities	Emphasizes assessment literacy, interpretation, and limitations
Examine epidemiological patterns and health disparities related to I/DD	*Competency 3*: Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Highlights inequities, systemic barriers, and social determinants of health
Apply person- and family-centered ethics of care principles in practice	*Competency 6:* Engage with Individuals, Families, Groups, Organizations, and Communities	
Competency 8: Intervene with Individuals, Families, Groups, Organizations, and Communities	Centers relational practice, care coordination, and culturally responsive intervention	

Understanding Intellectual and Developmental Disabilities (I/DD)

Intellectual and Developmental Disabilities (I/DD) encompass a range of conditions characterized by significant limitations in cognitive functioning and adaptive behavior, which emerge during the developmental period (Patel et al., 2018). These limitations affect three or more areas, such as cognition, self-direction, mobility, self-care, language use, independent living skills, and economic self-sustainability in adulthood. This chapter introduces key concepts in I/DD and emphasizes the importance of early recognition and intervention.

I/DD are conditions that manifest early in life, typically before the age of 18. They can result from genetic factors, environmental influences, or a combination of both. Students will explore how these factors contribute to developmental delays in various domains, including cognitive and adaptive functioning. While many types of disabilities fall under the I/DD umbrella, this chapter will focus on intellectual disability and autism, with the understanding that instructors may expand on other disabilities based on their expertise or student interest.

Establishing a diagnosis of I/DD is critical for determining eligibility for various services and supports. It ensures that individuals' rights are protected and that they receive appropriate interventions across their lifespan. However, a diagnosis of I/DD serves several purposes: **eligibility for services**, which is a formal diagnosis that opens doors to essential services such as early intervention programs, special education, Medicaid home and community-based services, and Social Security benefits. Another purpose is the **Protection of Rights,** ensuring that individuals with I/DD receive the accommodations and supports they need to thrive.

Establishing the Diagnosis of I/DD

Each I/DD has distinct diagnostic criteria that guide healthcare professionals in identifying and categorizing conditions that fall under the intellectual disability umbrella. For example, in intellectual disability, the diagnostic criteria may include examining the extent to which the deficits in intellectual functioning are confirmed through clinical assessment and standardized testing, which typically include an IQ test, using standardize tools to identifying levels of adaptive functioning deficiencies that failure to meet the standards for personal independence, and examining the extent of developmental onset of intellectual disability, which begins before the age of 18. Severity can range from mild to profound.

Purpose of Diagnosis

A formal diagnosis of intellectual and developmental disabilities (I/DD) serves multiple critical functions across service, policy, and rights-based systems. One purpose is to establish eligibility for essential supports and services, including early intervention, special education, Medicaid home- and community-based services, and Social Security benefits. Access to these resources is often contingent upon documented diagnostic criteria. Beyond service access, diagnosis also plays an important role in protecting individual rights. It helps ensure that people with I/DD receive appropriate accommodation, legal protections, and supports necessary for full participation and well-being across educational, healthcare, employment, and community settings.

Diagnostic Criteria for I/DD

The distinct diagnostic criteria guide clinicians and other qualified professionals in identifying and classifying intellectual and developmental disabilities. These criteria support consistency in assessment while informing service planning and intervention. The following overview highlights key diagnostic components for intellectual disability and autism spectrum disorder:

Intellectual Disability

Assessment of intellectual disability involves many domains of functioning. Deficits in intellectual functioning are identified through comprehensive clinical evaluation and standardized measures, most commonly including intelligence testing. In addition, limitations in adaptive functioning are assessed by examining an individual's ability to meet sociocultural standards for personal independence and social responsibility using validated assessment tools. Onset must occur during the developmental period, before age 18, and severity is classified as mild, moderate, severe, or profound based on adaptive functioning rather than IQ alone.

Autism Spectrum Disorder (ASD)

Diagnosis of autism spectrum disorder is characterized by persistent challenges in communication and social interaction, as well as repetitive behaviors, interests, or activities. Standardized diagnostic instruments are used to evaluate social development in relation to cognitive functioning. These may include structured observational measures and intelligence testing. A comprehensive developmental history is also essential to establishing the presence and pattern of symptoms over time. Levels of support are identified as Level 1 (requiring support), Level 2 (requiring substantial support), or Level 3 (requiring very substantial support), reflecting the intensity of assistance needed across settings.

Epidemiology and Demographics of I/DD

Patterns of prevalence and demographic distribution vary across intellectual and developmental disabilities, revealing the importance of understanding these trends for accurate diagnosis, effective intervention, and equitable service delivery. Epidemiological data provide critical insight into who is affected, how frequently these conditions occur, and how social factors shape identification and access to support.

Intellectual Disability

The rates of intellectual disability in the United States vary widely, reflecting differences in severity and measurement. Estimates indicate that mild to profound intellectual disability affects approximately 8.7 to 36.8 per 1,000 children, while more severe forms, defined by an IQ below 50, occur in roughly 2.5 to 5 per 1,000 children (Boat et al., 2015). Although intellectual disability is present across all racial, ethnic, and cultural groups, variations in prevalence are influenced by socioeconomic conditions, access to healthcare and education, and potential biases in assessment and testing practices (Emerson, 2007, 2012; Graham, 2005; Magaña & Vanegas, 2021).

Autism Spectrum Disorder

Autism spectrum disorder is among the most commonly diagnosed developmental disabilities in the United States (Baio, 2014; "Prevalence of Autism Spectrum Disorders—Autism and Developmental Disabilities Monitoring Network, 14 Sites, United States, 2008," 2012; Zablotsky et al., 2019). Current estimates suggest that approximately one in 44 children receives an autism diagnosis, with about 35 percent of these individuals also meeting criteria for intellectual disability. Gender disparities are notable, as boys are diagnosed at rates approximately four times higher than girls. In contrast, prevalence does not differ significantly across racial and ethnic groups, with comparable rates observed among Black, White, Hispanic, and Asian/Pacific Islander children—highlighting the role of improved screening and diagnostic practices across populations.

Screening and Assessment

Early screening plays a critical role in identifying intellectual and developmental disabilities. This will help inform timely, appropriate intervention planning. Comprehensive assessment processes draw on multiple sources of information to evaluate developmental progress, cognitive functioning, and adaptive skills across settings.

Screening Tools

Initial identification often begins with information provided by caretakers, whose observations offer valuable insight into early developmental delays and functional concerns. These reports are frequently supplemented by standardized screening instruments designed to flag potential areas of need. Commonly used tools include the Modified Checklist for Autism in Toddlers, Revised (MCHAT-R) (*M-CHAT™ - Autism Screening*, n.d.), which supports early autism screening, and the Vineland Adaptive Behavior Scales (VABS) (*Vineland Adaptive Behavior Scale - an Overview | ScienceDirect Topics*, n.d.), which assesses adaptive functioning across key domains (Cordeaux, 2019; Grigore et al., 2024; Wieckowski et al., 2023).

IQ Testing

Measures of intellectual functioning are used to assess general cognitive abilities such as reasoning, problem-solving, planning, and abstract thinking. This testing provides standardized data that contribute to diagnostic decision-making, with an IQ score of approximately 75 or below typically indicating intellectual disability when considered alongside adaptive functioning and developmental history.

Adaptive Functioning

Assessment of adaptive functioning examines how individuals manage everyday demands relative to sociocultural expectations. Core areas of evaluation include social skills, communication, personal and daily living skills, and motor functioning. While academic achievement may be delayed, individuals with mild intellectual and developmental disabilities may still acquire foundational literacy and mathematical skills, particularly when appropriate support is in place.

Behavioral Assessment

A comprehensive assessment process also considers behavioral and emotional functioning. Individuals with I/DD experience higher rates of co-occurring conditions, including externalizing disorders such as attention-deficit/hyperactivity disorder and impulse control difficulties, as well as internalizing conditions such as anxiety (Alford & Graham Holmes, 2025; Charlot & Beasley, 2013). A thorough behavioral assessment is needed to identify these concerns, as targeted interventions can significantly enhance overall functioning and quality of life.

Intervention and Treatment

Effective intervention and treatment strategies are vital for supporting individuals with I/DD, especially when delivered across the lifespan through care coordination of supports. Services are designed to reflect the developmental needs as they evolve from early childhood throughout adulthood, focusing on adaptive living skills (ADL), including well-being.

Support Services Across the Lifespan

Intervention often begins in early childhood (Ages <3), with services provided before age three **to address** developmental delays as soon as they are identified. **Early intervention** programs offer targeted support during a critical period of growth, helping to strengthen communication, motor, cognitive, and social skills.

From ages three through twenty-one, **special education** services play a central role in supporting children and adolescents with I/DD. Services are tailored to individual learning needs and delivered through individualized education programs (IEPs) that promote academic access, skill development, and participation in school environments, along with support within the classroom settings.

Starting at age 5 and continuing through adulthood (5+), individuals can receive services through Care Coordination, which can assist them with home care, in-home and overnight respite, community habilitation, recreation, family reimbursement, and additional services to promote quality of life and support families (Alford & Graham Holmes, 2025; Alford & Son, 2025a). Care coordination remains a critical component across all stages of intervention. State and regional agencies, such as the New York State Office for People with Developmental Disabilities, play a key role in organizing, funding, and overseeing services. Through coordinated planning and service delivery, these systems help ensure continuity of care and access to appropriate supports throughout the lifespan.

As individuals transition into adulthood (Ages 21+), the focus of services shifts toward independence, community participation, and long-term stability. **Transitional and adult services** may include vocational training, supported employment, day programs, residential options, and ongoing family support. These services are essential for promoting meaningful engagement, autonomy, and sustained well-being in adult life.

Health Disparities and Inequities

Despite advancements in healthcare, individuals with I/DD continue to face significant health disparities and systemic inequities. The challenges often stem from structural barriers, gaps in provider service delivery and training, and fragmented systems of care. All of these negatively influence health outcomes.



Figure 1. *Understanding Intellectual and Developmental Disabilities*

Pause & Reflect

Take a moment to think critically about what you've learned:

- How has your understanding of intellectual and developmental disabilities (I/DD) changed after reviewing this figure?
- Which area (definition, causes, characteristics, prevalence, diagnosis/early intervention, or social attitudes) do you feel most confident about—and which do you need to explore further?
- How might societal attitudes and stigma influence the experiences of individuals with I/DD and their families?
- As a future social worker, how can you promote inclusion, dignity, and advocacy in your everyday practice?

Diagnostic Overshadowing

A persistent barrier to effective healthcare is diagnostic overshadowing, a phenomenon in which physical or mental health symptoms are misattributed to an individual's intellectual or developmental disability. **Diagnostic Overshadowing** is a term used when specific health problems coexist with a more pressing healthcare need (Mizen, 2024). For example, if an individual with I/DD has a behavioral outburst, the clinician may not focus on his or her other health concerns. As the saying goes, "the squeaky wheel gets the grease." As a result, some problems may not be clinically identified or included in a person's diagnosis. In essence, these problems may be overshadowed by the primary diagnosis (Molloy et al., 2021; Orim et al., 2022). There can be significant challenges in identifying coexisting illnesses (Bouras, 1999), such as trying to distinguish a physical illness from a mental disorder. Therefore, clinicians need to be resourceful in determining which screening measures are most appropriate for an individual with I/DD (Robinson et al., 2012).

Access to Healthcare Providers

Access to consistent and qualified healthcare remains a significant challenge for many individuals with I/DD. Adults in particular often rely on Medicaid as their primary source of insurance, yet many healthcare providers do not accept it (Havercamp et al., 2004; Institute of Medicine (US) Committee on Understanding and Eliminating Racial and Ethnic Disparities in Health Care, 2003; Reichard et al., 2004). Limited provider participation, combined with long wait times and geographic disparities, restricts access to timely care. As a result, continuity of care is frequently disrupted, contributing to fragmented treatment and unmet health needs.

Increased Case Complexity

Healthcare for individuals with I/DD is further complicated by the presence of multiple, interconnected needs. Co-occurring physical and mental health conditions, communication barriers, and inconsistent coordination among providers can lead to misdiagnosis or incomplete treatment planning (Beadle-Brown et al., 2016; Wark & Kingstone, 2019). These complexities are sometimes met with negative attitudes or discomfort among medical personnel, making it difficult for individuals and families to identify providers who can deliver comprehensive, respectful care. Addressing these challenges requires coordinated, interdisciplinary approaches that prioritize communication, training, and person-centered practice.

Person/Family-Centered Planning

Person-centered planning is a foundational approach in supporting individuals with intellectual and developmental disabilities (I/DD), emphasizing collaboration, dignity, and responsiveness to individual needs across systems of care. Within the Person-Centered Ethics of Care model (Alford & Son, 2025). Five interconnected systems shape planning and service delivery. *Educational systems* play an essential role by ensuring access to appropriate educational services and supports that promote learning, inclusion, and skill development. *Medical systems* are equally essential, requiring healthcare providers to understand the unique physical, behavioral, and communication needs of individuals with I/DD to deliver effective and equitable healthcare. *Governmental*

systems influence access through programs and legal protections that determine eligibility, funding*, and available services*. *Financial* systems support long-term stability through benefit planning, income support, and future care considerations. Lastly, social *service* systems, often led by social workers and other professionals, provide direct support services, advocacy, and care coordination to support individuals with I/DD and their families across the lifespan. The systems show the importance of coordinated planning that centers the individual while recognizing the essential role of families and caregivers.

Implications for Social Work Practice

Social workers play a central role in promoting well-being, access, and inclusion for individuals with I/DD (John & Schrandt, 2019). Their responsibilities extend across assessment, care coordination, advocacy, and culturally responsive practice.

Social Work Responsibilities

Comprehensive assessment is a fundamental part of social work with individuals with I/DD. Social workers assess cognitive, adaptive, social, and environmental aspects to create personalized care plans that highlight each client's strengths, needs, and goals. Additionally, social workers coordinate referrals to appropriate services and provide ongoing follow-up to track progress, resolve obstacles, and modify supports as needs change.

Cultural Competency

Culturally responsive practice is vital when working with individuals with I/DD and their families. Social workers must stay aware of potential biases in diagnostic testing, language accessibility, cultural beliefs and values, socioeconomic conditions, health status, and family and community dynamics. Recognizing these factors helps ensure ethical, respectful, and effective engagement while fostering equity in assessment and service delivery.

Resources for Individuals with Intellectual/Developmental Disabilities

Access to accurate and up-to-date resources is critical for individuals with I/DD, their families, and practitioners. The following resources support navigation of services within New York State. Instructors and practitioners should regularly review and update these materials to ensure they remain relevant to their local region.

The *Resource Guide for Families of Children with Disabilities or Developmental Delays* (NYC Department of Health, 2019) provides a comprehensive overview of available services and supports for families navigating early intervention and special education systems. Families and practitioners seeking broader guidance on available support may also consult the *Guide to Understanding Supportive Services*, produced by Parent to Parent of New York State, which outlines a range of community-based and state-funded options. The New York State Office for People With Developmental Disabilities (OPWDD) serves as the primary state agency coordinating services for individuals with developmental disabilities and maintains an extensive

online directory of programs, eligibility information, and provider resources at opwdd.ny.gov. Local resource guide directories, which vary by county and service area, may identify additional regional and provider-specific resources.

Chapter Discussion Questions:

1. How do genetic and environmental risk factors contribute to the development of Intellectual and Developmental Disabilities (I/DD)?
2. Why is it important to establish a diagnosis of I/DD early in life? What are the benefits and challenges associated with early diagnosis?
3. How do the diagnostic criteria for Intellectual Disability differ from those for Autism Spectrum Disorder? Why is it important to understand these differences?
4. What are the implications of the epidemiology and demographics of I/DD for social work practice?
5. How can social workers effectively support families during the screening and assessment process for I/DD?

Case Scenario: Early Identification and Diagnosis of I/DD

Case Scenario: Sarah

Sarah is a 3-year-old girl who has been attending a local daycare since she was six months old. Her parents, Jane and Michael, have started noticing that Sarah seems to be developing more slowly than her peers. While other children in her age group are beginning to speak in short sentences and engage in imaginative play, Sarah struggles with forming words and often prefers solitary activities like stacking blocks. She has difficulty following simple instructions and often becomes frustrated when trying to communicate her needs.

Jane and Michael have also observed that Sarah makes little eye contact and rarely responds when her name is called. She seems fascinated by spinning objects and can spend long periods watching the wheels of her toy car go round. Concerned, they bring up these observations during Sarah's routine check-up with her pediatrician.

The pediatrician performs a developmental screening and notes delays in language, social interaction, and fine motor skills. Given these concerns, the pediatrician refers Sarah to a developmental specialist for a more comprehensive evaluation.

Assessment and Diagnosis

The developmental specialist conducts a series of assessments, including a speech and language evaluation, a psychological assessment, and observations of Sarah's behavior in a structured setting. The results indicate that Sarah has significant delays in communication and social interaction, as well as repetitive behaviors and restricted interests. Based on these findings, the specialist diagnoses Sarah with Autism Spectrum Disorder (ASD).

In addition to the ASD diagnosis, the specialist also identifies Sarah as having an intellectual disability, as her cognitive testing scores fall significantly below the average for her age group. The diagnosis is shared with Jane and Michael, along with a thorough explanation of what it means for Sarah's development and future.

Intervention and Support

The specialist recommends that Sarah begin early intervention services immediately to address her developmental delays. An Individualized Family Service Plan (IFSP) is developed to outline the services Sarah will receive, including speech therapy, occupational therapy, and behavioral therapy. These services are provided in ways that actively involve Jane and Michael, equipping them with strategies to support Sarah's development at home.

The specialist also connects the family with a local support group for parents of children with autism and intellectual disabilities. Jane and Michael are encouraged to be active participants in planning and implementing Sarah's interventions, ensuring that services are tailored to her unique needs, preferences, and strengths.

Case Study Questions

1. What are the benefits of identifying Sarah's developmental delays and diagnosing her with ASD and intellectual disability at an early age?
2. How do Sarah's behaviors align with the diagnostic criteria for Autism Spectrum Disorder and intellectual disability? Why is it important to differentiate between the two?
3. How can early intervention services, like those outlined in Sarah's IFSP, improve her developmental outcomes?
4. What role do Jane and Michael play in supporting Sarah's development, and how can social workers assist them in navigating the early intervention process?
5. What challenges might Sarah's family face as they adjust to her diagnosis, and what additional support could be beneficial for them?

Key Terms & Definitions

Term	Definition
Adaptive Behavior	The collection of conceptual, social, and practical skills individuals use to function in everyday life, including communication, self-care, social participation, and independent living.
Adaptive Functioning	An individual's ability to meet age- and culturally appropriate expectations for personal independence and social responsibility is typically assessed alongside intellectual functioning.
Autism Spectrum Disorder (ASD)	A developmental disability characterized by persistent challenges in social communication and interaction, along with restricted or repetitive patterns of behavior, interests, or activities, with varying levels of support needs.
Behavioral Assessment	An evaluation process used to identify behavioral, emotional, or mental health concerns, including co-occurring conditions such as anxiety, ADHD, or mood disorders.
Care Coordination	A service approach that assists individuals with I/DD and their families in navigating, accessing, and managing multiple supports and services across systems over time.
Co-Occurring Conditions	Physical, mental health, or behavioral conditions that occur alongside an intellectual or developmental disability.
Cultural Competency (Culturally Responsive Practice)	The ability of professionals to deliver services that are respectful of and responsive to cultural differences, values, beliefs, language needs, and lived experiences.
Developmental Onset	The requirement that symptoms of intellectual and developmental disabilities emerge during the developmental period, typically before age 18.
Diagnostic Criteria	Established standards used by qualified professionals to determine whether an individual meets the requirements for a specific diagnosis.
Diagnostic Overshadowing	A phenomenon in which physical or mental health symptoms are incorrectly attributed to an individual's intellectual or developmental disability, resulting in missed or delayed diagnoses.
Early Intervention	Services provided to infants and toddlers (birth to age three) designed to address developmental delays and support early skill development.

Epidemiology	The study of the distribution, prevalence, and patterns of health conditions, including intellectual and developmental disabilities, across populations.
Health Disparities	Differences in health outcomes and access to healthcare that are closely linked to social, economic, environmental, and systemic inequalities.
Intellectual Disability (ID)	A developmental disability characterized by significant limitations in intellectual functioning and adaptive behavior, with onset before age 18, and classified by levels of support needs rather than IQ alone.
IQ (Intelligence Quotient) Testing	Standardized testing used to assess general cognitive abilities such as reasoning, problem-solving, and abstract thinking, typically interpreted alongside adaptive functioning.
Person- and Family-Centered Planning	An approach to service planning that prioritizes the preferences, strengths, goals, and needs of individuals with I/DD while actively involving families and caregivers.
Person-Centered Ethics of Care	A values-based framework emphasizing dignity, relationships, responsiveness, cultural humility, and coordinated support across systems of care.
Prevalence	The proportion of a population that has a particular condition at a given point in time.
Screening	A preliminary process used to identify potential developmental concerns and determine whether further assessment is needed.
Severity Levels	Classifications describing the level of support an individual with I/DD requires, ranging from mild to profound, based primarily on adaptive functioning.
Social Determinants of Health	The social, economic, and environmental conditions that influence health outcomes, access to services, and overall quality of life.
Special Education Services	Educational supports provided to students with disabilities, typically delivered through an Individualized Education Program (IEP), to promote access, learning, and participation.
Systems of Care	Interconnected service systems — including educational, medical, governmental, financial, and social service systems — that collectively support individuals with I/DD and their families.

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Chapter 2: Intellectual/Developmental Disability and Stigmatization

Learning Objectives

- Explain the historical and social foundations of disability stigma**,** including the role of language, eugenics, and dehumanizing ideologies in shaping contemporary attitudes toward individuals with I/DD.
- Analyze social psychological and theoretical models of stigmatization**,** including the stigma process model and disability frameworks (social, medical, and neurodiversity), to understand how stigma is produced and sustained.
- Examine the individual and family-level impacts of I/DD stigma, including mental health effects, social exclusion, courtesy stigma, and structural vulnerability.
- Identify the multi-level contexts in which I/DD stigma occurs**,** including personal, interpersonal, community, and societal systems, using multi-level models of stigmatization.
- Apply social work and neurodiversity-informed strategies to challenge stigma through micro, mezzo, and macro-level interventions, advocacy, policy engagement, and inclusive practice.

Table 2.1. *Alignment of Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	Competency	Alignment Rationale
Explain the historical and social foundations of disability stigma	***Competency 3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Addresses historical oppression, dehumanization, and structural inequality affecting people with I/DD
Analyze social, psychological, and theoretical models of stigmatization	***Competency 4:*** Engage in Practice-Informed Research and Research-Informed Practice	Requires critical analysis of theory to inform understanding of stigma and intervention
Examine the individual and family-level impacts of I/DD stigma	***Competency 6*****:** Engage with Individuals, Families, Groups, Organizations, and Communities	Centers lived experience, relational impacts, and family systems
Identify multi-level contexts in which I/DD stigma occurs	***Competency 5:*** Engage in Policy Practice	Highlights institutional, community, and societal structures that shape stigma
Apply social work and neurodiversity-informed strategies to challenge stigma	***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities	Emphasizes action-oriented intervention, advocacy, and inclusive practice across systems

Understanding the Stigma of Disability

Individuals with Intellectual and Developmental Disabilities (I/DD) are among the most socially isolated and marginalized groups in society. The stigma surrounding disability contributes to significant disparities in health, employment, and housing, impacting nearly every aspect of life for people with I/DD. This chapter explores the historical context, theoretical foundations, and ongoing effects of stigmatization on individuals with I/DD and their families.

The History of Developmental Disabilities

The history of how society has treated individuals with developmental disabilities is marred by derogatory language and harmful ideologies (Braddock et al., 2015; Carlson, 2001; Trent Jr, 1994). Terms such as "feebleminded," "mentally defective," "idiot," "subnormal," and "retarded" were once used to describe individuals with I/DD, reflecting deep-seated prejudice. These terms have since evolved into generic insults, perpetuating negative stereotypes. The dehumanization of people with disabilities reached its zenith during the Eugenics movement in the U.S. and Nazi Germany, where such individuals were considered inferior and unworthy of life (Claeys, 2000). Although overt discrimination has decreased, the stigma and low expectations for people with I/DD persist in areas such as education, employment, healthcare, sexuality, and parenting.

Social Psychological Theories of Stigmatization

Stigmatization has deep roots in social psychology, with several theoretical perspectives seeking to explain why certain groups are marginalized. From an evolutionary standpoint, stigma often emerges from survival-related mechanisms, such as disease avoidance and concerns about non-reciprocity. Historically, social stigmas have helped ensure the survival of dominant groups by marginalizing those perceived as different or threatening to social order.

Individuals with disabilities have frequently been positioned as risks to survival or societal functioning, leading to their exclusion and marginalization. Across history, the stigmatization of individuals with intellectual disabilities has served as a means of defining social boundaries by who is considered capable, productive, or worthy of inclusion (*Mental Deficiency Act 1913;* Lau & Cheung, 1999). Dominant groups reinforce in-group cohesion and maintain power structures. This dynamic reflects how stigma operates not only as an individual bias but as a social mechanism that sustains inequality and reinforces normative assumptions about ability and value.

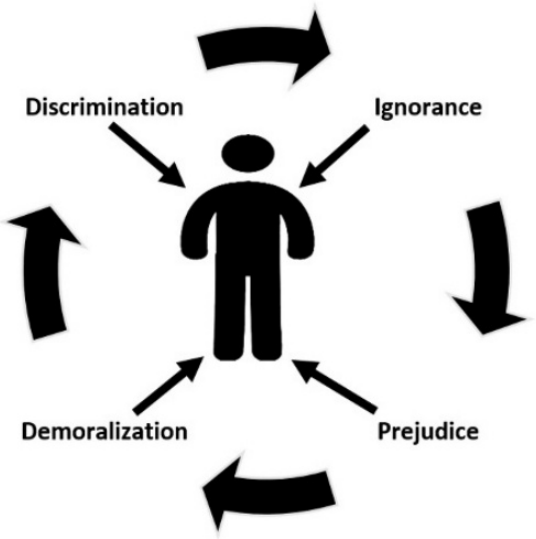


Figure 2.1. *Stigma Process Model*

Pause and Reflect: Cycle of Stigma

- Where in this cycle do you think social workers have the most power to intervene—and why?
- How might systems (e.g., education, healthcare, policy) reinforce or disrupt this cycle?
- In what ways can demoralization become internalized, and how might that affect self-advocacy and service engagement?
- Reflect on a real or observed situation: Which part of this cycle was most visible? What was missing?
- How can you apply a Person-Centered Ethics of Care (PC-EoC) approach to interrupt this cycle at multiple levels (micro, mezzo, macro)?

The **stigma process model (Figure 1)** shows how stigma is produced, reinforced, and sustained through interconnected social mechanisms that center on the individual. At the center of the model is the individual, representing a person who is the target of stigma, often someone with a disability or another marginalized identity. Surrounding the individual are four interrelated forces: *ignorance, prejudice, discrimination, and demoralization***,** each feeding into the next in a continuous loop.

The process often begins with **ignorance**, which reflects a lack of accurate knowledge, understanding, or exposure. When people do not understand a condition, identity, or lived experience, misconceptions and stereotypes are more likely to develop. This ignorance gives rise to **prejudice**, which involves negative attitudes, assumptions, or beliefs about the individual based on those misunderstandings. Prejudice then manifests behaviorally as **discrimination**, where biased beliefs translate into unequal treatment, exclusion, or denial of opportunities and resources. Discriminatory practices can occur at interpersonal, institutional, or systemic levels, reinforcing marginalization and social exclusion. As discrimination accumulates, it contributes to **demoralization**, the internal impact of stigma on the individual. This may include diminished self-

worth, reduced confidence, internalized stigma, social withdrawal, or feelings of powerlessness. Demoralization, in turn, can limit opportunities for visibility, advocacy, or self-expression, thereby perpetuating ignorance in broader society.

The arrows in the model emphasize that stigma is not linear but cyclical. Each component reinforces the others, sustaining stigma over time unless intentional interventions disrupt the cycle. The model highlights that stigma is not merely an individual experience but a socially produced process requiring systemic, educational, and relational responses to promote inclusion, equity, and dignity for individuals with disabilities.

The Cycle of Stigmatization Towards Individuals with I/DD

Stigmatization related to intellectual and developmental disabilities is maintained through a connected cycle influenced by misinformation, social separation, and power imbalances. These factors reinforce one another, allowing stigma to persist across social, institutional, and policy levels. Persistent stigma is partly caused by this cycle, which includes widespread misunderstandings and long-standing misconceptions about I/DD.

Inaccurate beliefs and stereotypes promote discriminatory attitudes and practices that hinder inclusion. In this sense, social and physical segregation worsens this by lowering visibility and limiting meaningful interaction, which deepens fear, discomfort, and false assumptions about individuals with disabilities. At the structural level, limited social, economic, and political power reduces public concern for the rights and needs of individuals with I/DD. Consequently, advocacy remains weak, investments in services stay low, and marginalization persists.

The Context of Stigma: Where Does It Occur?

Stigmatization related to intellectual and developmental disabilities does not operate in isolation; rather, it emerges and is reinforced across multiple layers of social life (Werner, 2015; Werner et al., 2012). From individual beliefs to institutional practices and broader societal structures, stigma is produced, maintained, and normalized through interconnected systems of power and meaning.

The multi-level model proposed by Werner & Scior (2016) (Figure 2.3) offers a comprehensive framework for understanding where and how stigma manifests in the lives of individuals with I/DD and their caregivers (Werner, 2016; Werner & Scior, 2016). This model identifies four interrelated levels at which stigmatization occurs.

- At the *personal level***,** stigma operates through internal beliefs, attitudes, and emotional responses. Individuals without disabilities may hold implicit biases or misconceptions, while individuals with I/DD may internalize negative societal messages, leading to self-stigma, low self-esteem, and diminished expectations for themselves.
- The *interpersonal level* captures stigma within everyday social interactions, including relationships with family members, peers, educators, healthcare providers, and service professionals. At this level, stigma may appear as patronizing behavior, lowered expectations, exclusion from decision-making, or overt discrimination, all of which directly influence lived experiences and opportunities.

- At the *community level*, stigma is embedded within social norms, organizational practices, and local institutions such as schools, workplaces, healthcare systems, and service agencies. Segregated programs, inaccessible environments, and limited community representation reinforce invisibility and marginalization by signaling who belongs and who does not.
- Finally, the societal level includes structural and cultural forces such as public policies, legal systems, media representations, and dominant ideologies about productivity, independence, and worth. At this level, stigma persists through underfunded services, restrictive eligibility criteria, exclusionary laws, and narratives that portray disability as a burden or a deficit rather than as a part of human diversity.

Scior's model demonstrates that stigma is an individual attitude and a systemic issue that requires coordinated, multilevel interventions. This framework reveals the need for social work practices that tackle both individual and systemic stigma, prioritizing anti-oppressive, rights-based, and trauma-informed methods.

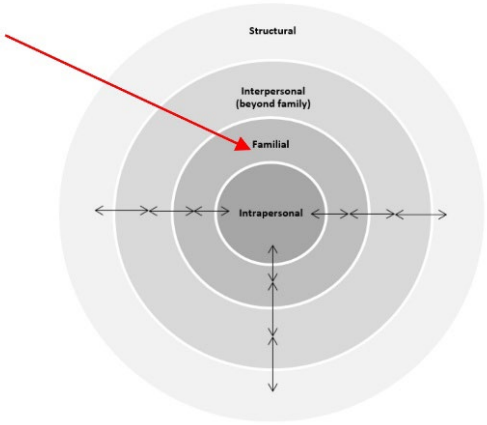


Figure 2.2. *Werner & Scior's Multi-Level Model of Stigmatization*

Theoretical Approaches to Disability

Examining stigma related to intellectual and developmental disabilities (I/DD) requires engaging with multiple theoretical frameworks that explain how society understands, constructs, and responds to disability. Each perspective offers distinct insights into the origins of marginalization and the mechanisms that produce and sustain stigma.

The social model of disability locates the problem not within the individual but within the social, political, and institutional structures that create barriers to inclusion. From this view, disability is produced by systemic factors such as personal bias, inaccessible physical and social environments, segregated educational systems, and limited employment opportunities, making stigma a consequence of social organization rather than an inherent trait of individuals with I/DD (Kattari et al., 2017; Oliver, 1990, 2013). The medical model, by contrast, conceptualizes I/DD primarily as a pathological condition requiring diagnosis, treatment, or correction. Rooted in

historically moralized understandings of disability, this approach prioritizes cure and normalization, often at the expense of acceptance and inclusion. When framing disability as a deficit within the individual, the medical model contributes to pathologization and can reinforce stigma by positioning individuals with I/DD as problems to be managed rather than as individuals entitled to full participation in society (Kattari et al., 2017; Oliver, 1990, 2013).

Theoretical Comparisons

These theoretical approaches reveal fundamentally different assumptions about disability, responsibility, and social value towards individuals with disabilities. The medical model locates disability within the individual, prioritizing diagnosis and remediation, which can inadvertently reinforce deficit-based thinking and stigma. In contrast, the social model reframes disability as a product of structural, environmental, and institutional barriers, shifting responsibility from the individual to society and emphasizing rights, access, and inclusion. Cognitive and attributional approaches further reveal how stigma is sustained through beliefs about fairness, control, and perceived burden, operating at both interpersonal and systemic levels.

These models support an anti-oppressive, person-centered approach that resists pathologization, promotes equity, and centers the dignity and self-determination of individuals with intellectual and developmental disabilities. In social work practice, these theoretical distinctions are critical, and social workers must use anti-oppressive frameworks that align most closely with the social model to challenge power imbalances, interrogate normative assumptions about ability, and advocate for structural change. At the same time, social workers must integrate insights from attributional theories to address internalized stigma, bias, and decision-making processes within individuals and institutions.

Cognitive and Attributional Approaches

Additional perspectives on the persistence of stigma are offered by cognitive and attributional theories. These examine how beliefs, assumptions, and social judgments shape attitudes toward individuals with I/DD. The belief in a just world, for example, leads some individuals to interpret social inequities as deserved or unavoidable, thereby rationalizing marginalization. Historical and contemporary eugenic ideologies further contribute to dehumanization by assigning lesser value to the lives of people with I/DD.

Cognitive mechanisms help explain how stigma is normalized and perpetuated across interpersonal and structural levels. For example, attributional processes also reinforce stigma. Assumptions that individuals with I/DD lack autonomy or decision-making capacity can legitimize exclusionary practices and paternalistic control. Moreover, perceived demands associated with providing support, combined with uncertainty in social interactions, may lead individuals and institutions to avoid inclusion altogether.

Integrating Anti-Oppressive and Trauma-Informed Frameworks

Anti-oppressive and trauma-informed frameworks offer critical lenses for responding to the stigmatization of individuals with I/DD. **Anti-oppressive practice** challenges the structural

inequities, normative assumptions about ability, and power hierarchies that sustain stigma, aligning with the social model's emphasis on systemic change. From this perspective, social workers are called to interrogate policies, practices, and professional norms that reinforce segregation, dependency, and exclusion, while actively promoting self-determination, inclusion, and rights-based advocacy.

Trauma-informed frameworks further recognize that chronic exposure to stigma, exclusion, and devaluation constitutes a form of relational and systemic trauma. Experiences such as presumed incompetence, social isolation, and vulnerability to abuse can have lasting psychological and emotional consequences. **Trauma-informed practice** emphasizes safety, trust, collaboration, empowerment, and cultural humility, principles that are essential when working with individuals and families who have been repeatedly marginalized. These frameworks can guide social workers toward interventions that mitigate the effects of stigma, actively dismantle its sources, and support healing and resilience.

The Effects of Stigmatization on Individuals with I/DD

Impact on Individuals with I/DD and their Caregivers

The effects of stigma go far beyond attitudes, significantly impacting the daily lives of individuals with I/DD and their families in meaningful and lasting ways. Individuals with I/DD are often judged as incapable, which limits their independence and chances (DeVoto & Harrison, 2015). They are frequently seen as dependent or childlike, leading to less respect, fewer choices, and being excluded from important decisions that affect them. These views directly impact their access to education, jobs, and meaningful social activities, creating ongoing cycles of reliance and exclusion.

The absence of visible, positive role models further reinforces low expectations and limits perceived opportunities for achievement and leadership for individuals with I/DD. Segregated environments, whether in education, housing, or services, also increase the risk of abuse, neglect, and hate crimes by decreasing oversight and reducing chances for self-advocacy. Prolonged exposure to stigma over time can harm mental health, causing issues such as poor self-esteem, anxiety, and depression, ultimately deepening health inequalities.

Stigma also extends to families through processes commonly described as courtesy stigma, in which negative attitudes toward individuals with I/DD are transferred to those associated with them. Family members may internalize these societal messages, leading to affiliate stigma characterized by shame, guilt, or self-blame. As a result, families may withdraw from social spaces to avoid judgment or discrimination, increasing isolation and reducing access to informal support networks.

The ongoing stress of caring for a loved one with I/DD within a stigmatizing social environment can lead to significant health and financial issues (McDaniel, 2023; Miodrag & Hodapp, 2011). Emotional strain, decreased well-being, and economic difficulties are common, especially when families must navigate fragmented systems with limited support. These effects

illustrate how stigma influences not only individuals but also entire family systems and broader social structures.

Social Work Efforts to Combat Stigma

Transition: From Stigma to Intervention

The pervasive effects of stigma on individuals with I/DD and their families reveal the urgent need for intentional, justice-oriented intervention strategies. Because stigma operates across interpersonal, institutional, and structural levels, effective responses must move beyond individual behavior change to address power imbalances, exclusionary systems, and cumulative trauma. This shift from describing harm to actively disrupting it provides a foundation for anti-oppressive and trauma-informed social work practice.

Social workers play a crucial role in addressing and reducing the stigma associated with I/DD. By understanding the theoretical underpinnings of stigma, social workers can design and implement interventions at multiple levels.

Intervention Strategies

Social workers occupy a critical role in efforts to address and reduce stigma associated with intellectual and developmental disabilities. Social workers are better equipped to design interventions that operate across multiple levels of influence and promote meaningful, lasting change, and to ground their practice in theoretical understandings of how stigma is produced and sustained.

Effective intervention begins with a clear understanding of the theoretical factors that contribute to stigmatization. Knowledge of social, cognitive, and structural explanations of stigma enables social workers to identify their sources, recognize how they are reinforced, and respond with intentional, evidence-informed strategies. This theoretical grounding supports interventions that extend beyond individual attitudes to address broader social and institutional dynamics.

Stigma reduction efforts must be implemented across micro, mezzo, and macro levels of practice. At the individual level, interventions may focus on empowerment, self-advocacy, and addressing internalized stigma. At the community level, social workers can challenge exclusionary practices, promote inclusion, and engage organizations to deliver accessible and affirming services. At the societal level, advocacy efforts target policy reform, public awareness, and systemic change. Reducing stigma requires ongoing action everywhere, supporting ethical values of dignity, social justice, and inclusion for people with intellectual and developmental disabilities.

Mapping Stigma-Reduction Strategies Across Social Work Practice Levels

Social workers address stigma associated with intellectual and developmental disabilities through coordinated interventions across micro, mezzo, and macro levels of practice, grounded in anti-oppressive and trauma-informed frameworks. At the ***micro*** level, practice emphasizes empowerment, self-determination, and relational engagement, supporting individuals in

recognizing and challenging internalized stigma, facilitating self-advocacy and supported decision-making, providing psychoeducation about rights and available supports, and helping families navigate courtesy and affiliate stigma while strengthening coping networks.

At the ***mezzo*** level, social workers intervene within organizations, service systems, and communities by consulting with schools, healthcare settings, and agencies to reduce exclusionary practices; training professionals to recognize implicit bias and diagnostic overshadowing; and collaborating with community organizations to design inclusive programs that center the voices of individuals with I/DD. At the ***macro*** level, practice shifts toward structural equity and systemic transformation, advocating for disability-inclusive policies across education, employment, housing, and healthcare; challenging laws and funding structures that sustain segregation; engaging in public education to counter harmful stereotypes; and supporting disability-led movements that shift power and representation. These strategies reflect a commitment to dismantling the structural conditions that sustain stigma while advancing the dignity, inclusion, and full participation of individuals with I/DD in all aspects of community life.

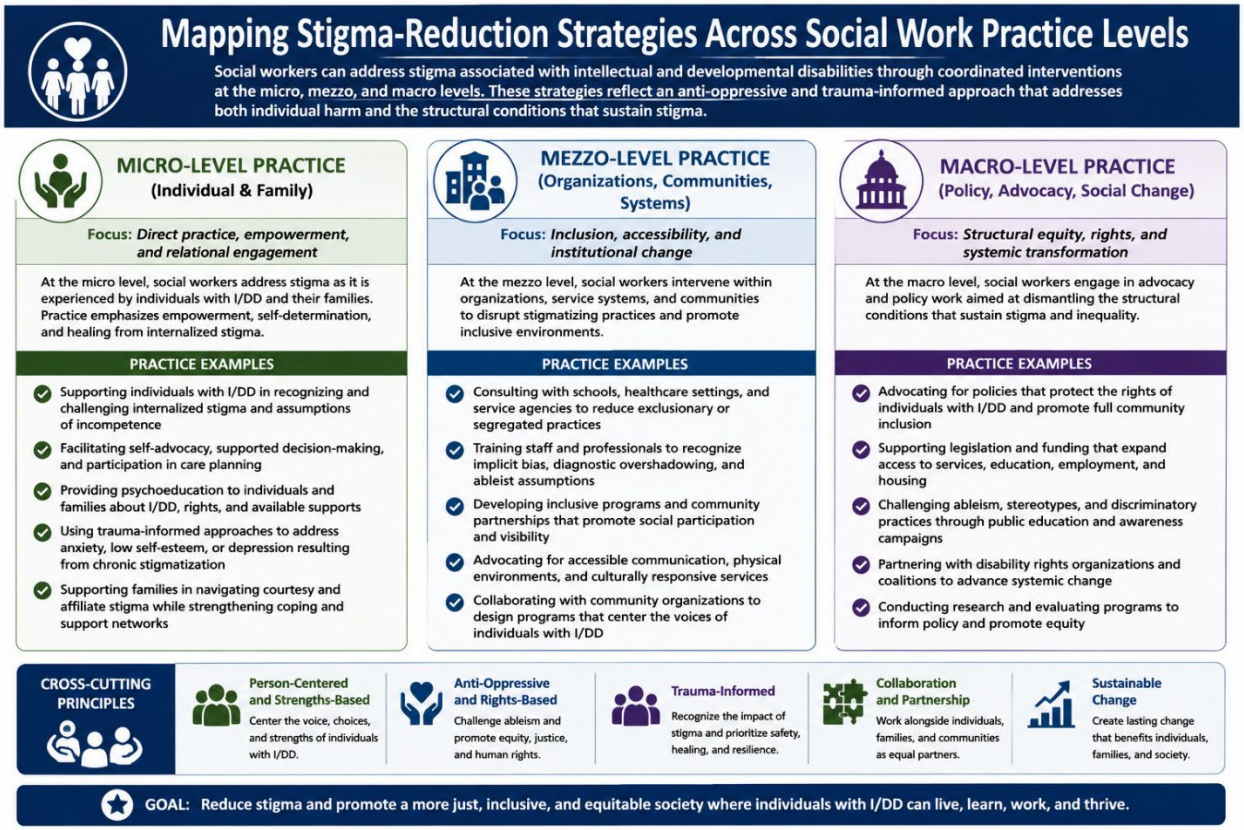


Figure 2.3. *Mapping Stigma Reduction Strategies Across Social Work Practice*

Anti-oppressive and trauma-informed social work practice requires coordinated action across all three levels to address personal harm while simultaneously transforming the structures that produce and sustain stigma. Micro, mezzo, and macro interventions reinforce one another, reflecting the understanding that stigma is a systemic process with individual consequences.

Pause and Reflect

1. How do micro-, mezzo-, and macro-level interventions complement one another in reducing stigma experienced by individuals with I/DD? Provide an example of how interventions at one level can influence change at another.
2. Reflect on a personal, professional, or field experience in which you observed stigma toward an individual with a disability. At which practice level(s) did the stigma occur, and how might a social worker have intervened differently?
3. How do anti-oppressive and trauma-informed frameworks strengthen stigma-reduction efforts across all three levels of social work practice?
4. If you were designing a stigma-reduction initiative for your agency or community, what is one intervention you would implement at the micro, mezzo, and macro levels to promote inclusion and equity for individuals with I/DD?

NASW Code of Ethics

These approaches align with the NASW Code of Ethics' core values, which emphasize dignity and a person's worth, as well as the importance of human relationships. The social model supports the NASW mandates to challenge social injustices by dismantling discriminatory practices and exclusionary policies that marginalize individuals with I/DD. In contrast, relying on the medical model risks violating ethical commitments, reinforcing deficit-based perspectives within practice, and limiting self-determination. These frameworks should guide social workers in applying an anti-oppressive lens in practice, upholding the profession's ethical responsibility to promote equity, respect autonomy, and advocate for the full inclusion and human rights of individuals with intellectual and developmental disabilities.

Understanding and Applying the Neurodiversity Framework

The neurodiversity framework provides a critical lens for challenging stigma and advancing inclusion for individuals with intellectual and developmental disabilities (Chauhan et al., 2024; McMahan, 2002). Rather than framing cognitive differences such as deficits or pathologies, this approach recognizes neurological diversity as a natural and valuable aspect of human variation.

Central to this framework is recognizing that conditions such as intellectual/developmental disabilities (I/DD) represent normal variations in human cognition. Differences in brain development and functioning are understood as part of the broad spectrum of human diversity, not as abnormalities that need to be "fixed. This perspective directly counters deficit-based narratives that have historically led to exclusion and marginalization.

Respect for language preferences is another key principle of neurodiversity-informed practice. Honoring individuals' preferences for person-first or identity-first language supports dignity, autonomy, and self-definition, reinforcing the importance of listening to how they describe their identities and experiences.

Effective application of the neurodiversity framework also requires familiarity with relevant legislation and ethical obligations. Social workers must understand and actively uphold

protections established through laws such as the Americans with Disabilities Act (ADA) (1990) and the Individuals with Disabilities Education Act (IDEA) (2004). Knowing these frameworks strengthens advocacy efforts and ensures practice aligns with legal mandates for access, accommodation, and inclusion.

In social work practice, neurodiversity-informed care emphasizes developing individualized support plans that build strength while addressing access needs. Inclusion strategies extend across educational settings, workplaces, and community environments, promoting participation rather than segregation. Therefore, collaboration is essential, requiring coordinated efforts among families, caregivers, educators, healthcare providers, and social service professionals to ensure continuity and responsiveness of support.

Finally, assistive technologies play an important role in advancing autonomy and participation. Familiarity with tools that support communication, learning, mobility, and daily living enables social workers to better advocate for accommodations that enhance independence and quality of life. These principles position the neurodiversity framework as a powerful foundation for ethical, inclusive, and person-centered social work practice.

Discussion Questions:

1. How has the historical stigmatization of individuals with I/DD influenced current societal attitudes and policies?
2. What are the key differences between the Social Model of Disability and the Medical Model of Disability, and how do these models impact the treatment of individuals with I/DD?
3. What are the effects of stigma on individuals with I/DD and their families? How can social workers help mitigate these effects?
4. How can the concept of neurodiversity be used to promote more inclusive practices for individuals with I/DD?
5. What strategies can social workers use to combat stigma at the individual, family, and societal levels?

Case Scenario: Addressing Stigmatization in School and Community Settings

Case Scenario: Michael

Michael is a 12-year-old boy with an intellectual disability who attends a public middle school in a small town. Despite receiving special education services and accommodations through his Individualized Education Plan (IEP), Michael struggles academically and socially. He has difficulty with reading and math, and often needs additional time and support to complete his work. While his teachers and parents are supportive, his classmates are not always understanding.

Michael is frequently teased at school by peers for his slower pace and for needing extra help from the teacher's aide. Other students sometimes exclude him from group activities, calling him hurtful names. These experiences have made Michael feel increasingly isolated and anxious, and he has begun withdrawing from class participation and making excuses to avoid going to school.

Similar experiences in the community compound the situation. Michael is involved in a local youth soccer league, which he loves, but the other children often ignore him on the field, passing the ball only among themselves. During practice, he overhears some parents questioning why a "special needs kid" is playing with their children. This comment deeply affects Michael's self-esteem and sense of belonging.

Michael's parents, aware of the growing impact of these experiences, reach out to the school counselor and the soccer team coach. They are concerned about the long-term effects of the stigmatization Michael is experiencing and want to ensure he feels included and supported both in school and in the community.

Intervention and Support

The school counselor meets with Michael's parents, teachers, and principal to develop a coordinated response. They agree to implement a school-wide anti-bullying program that includes education about intellectual and developmental disabilities, emphasizing empathy, respect, and inclusivity. The counselor also works with Michael's classmates through structured discussions and activities designed to foster understanding of intellectual disabilities. A peer mentoring program is introduced, pairing selected students with Michael to support him in social and academic settings and help build meaningful friendships.

The soccer team coach, after meeting with Michael's parents, addresses the players and their families about the importance of inclusivity and teamwork, emphasizing that every member of the team deserves respect regardless of ability. Practice routines are adjusted to

ensure Michael is actively involved and has genuine opportunities to contribute and succeed on the field.

Michael's parents are also connected with a local support group for families of children with intellectual disabilities. Through this group, they gain insights into how other families have navigated similar challenges and learn about resources and strategies to help Michael build confidence and navigate social situations.

Discussion Questions

1. How has the stigmatization Michael experiences at school and in the community affected his self-esteem and overall well-being?
2. What role do the school and community play in addressing the stigmatization of individuals with intellectual disabilities, and what specific actions can they take to foster inclusivity?
3. How does the Social Model of Disability apply to Michael's situation, and how might it inform the interventions implemented by the school and community?
4. Why is it important to educate Michael's peers about intellectual disabilities, and how can this education reduce stigma and improve his social experiences?
5. What long-term strategies can be put in place to ensure that Michael and other students with I/DD are supported and included in all aspects of school and community life?

Key Terms and Definitions

Term	Definition
Stigma Process Model	A cyclical framework that explains how stigma is produced and sustained through four interconnected mechanisms: ignorance, prejudice, discrimination, and demoralization. Applied to individuals with I/DD, the model illustrates how societal misunderstanding and negative attitudes translate into harmful behaviors and internalized harm.
└ **Ignorance**	A lack of accurate knowledge or understanding about intellectual and developmental disabilities that contributes to stereotypes, fear, and misinformation within communities and institutions.
└ **Prejudice**	Negative attitudes or beliefs about individuals with I/DD arising from misinformation, bias, or social conditioning that position disability as a deficiency or deviation from the norm.
└ **Discrimination**	Behavioral expressions of stigma involving unequal treatment, exclusion, or denial of rights and opportunities at interpersonal, institutional, or systemic levels.
└ **Demoralization**	The internal psychological impact of stigma on individuals with I/DD, including reduced self-worth, confidence, agency, and social participation resulting from chronic exposure to stigmatizing attitudes and behaviors.
Multi-Level Model of Stigmatization	A framework developed by Werner and Scior identifying stigma as occurring across personal, interpersonal, community, and societal levels, emphasizing its systemic and structural nature and the need for multilevel intervention.
└ **Personal Level**	Internalized stigma experienced by individuals with I/DD, including shame, low self-esteem, and reduced self-determination resulting from absorbing negative societal beliefs about disability.
└ **Interpersonal Level**	Stigma experienced in direct interactions with others, including discriminatory behaviors, social exclusion, and negative assumptions made by peers, professionals, and community members.
└ **Community Level**	Stigma embedded in organizational and community practices, including exclusionary policies, inaccessible environments, and limited opportunities for participation in community life.
└ **Societal Level**	Structural stigma embedded in laws, policies, media representations, and cultural norms that systematically

	disadvantage individuals with I/DD across institutions and social systems.
Affiliate Stigma	The internalization of societal stigma by family members of individuals with I/DD, resulting in shame, guilt, self-blame, or social withdrawal due to their association with a stigmatized person.
Ableism	A system of beliefs and social practices that positions non-disabled people as the norm and devalues, excludes, or discriminates against individuals with disabilities, often manifesting in institutional structures, cultural attitudes, and interpersonal interactions.
Courtesy Stigma	Stigma experienced by family members, caregivers, or close associates of individuals with I/DD due to their relationship or association with a stigmatized person, leading to social exclusion or negative treatment.
Dehumanization	The process of denying the full humanity, worth, or agency of individuals with I/DD, often used historically to justify exclusion, institutionalization, neglect, or abuse.
Disability Stigma	Negative beliefs, attitudes, and practices directed toward individuals with disabilities that result in social exclusion, lowered expectations, and structural inequities across life domains.
Eugenics	A historical and ideological movement promoting beliefs of biological superiority that justified forced sterilization, segregation, and dehumanization of people with disabilities, leaving lasting impacts on disability policy and public attitudes.
Internalized Stigma	The process by which individuals with I/DD absorb and accept negative societal beliefs about disability, negatively impacting self-esteem, identity formation, and mental health outcomes.
Structural Vulnerability	Heightened exposure to harm resulting from social, economic, and political systems that limit access to resources, protections, and opportunities for individuals with I/DD.

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Chapter 3: I/DD Delays in Early Childhood and Early Intervention

Learning objectives

- Recognize early signs of I/DD across physical, cognitive, communication, social-emotional, behavioral, and sensory domains in early childhood.
- Explain diagnostic pathways for children ages 0–3, including prenatal, perinatal, postnatal, and late-identification considerations.
- Describe the purpose and structure of Early Intervention (EI), including eligibility, IDEA funding, and family-centered service delivery.
- Identify the roles of multidisciplinary EI providers, including social workers, therapists, medical professionals, and service coordinators.
- Summarize core EI service areas, including physical, cognitive, communication, adaptive, social-emotional, and sensory development.
- Explain the IFSP process and components, emphasizing collaboration, measurable outcomes, and ongoing review.
- Apply the Person-Centered Ethics of Care Model to early childhood systems and service coordination.
- Assess the importance of early intervention and transition planning for long-term child and family outcomes.

Table 3.1. *Alignment of Chapter 3 Learning Objectives With CSWE EPAS (2022) Competencies*

Chapter 3 Learning Objective	CSWE EPAS (2022) Competency	Alignment Rationale
Recognize early signs of I/DD across developmental domains	***Competency 1:*** Demonstrate Ethical and Professional Behavior	Requires professional judgment, ethical screening practices, and accurate identification of developmental concerns in early childhood.
Explain diagnostic pathways for children ages 0–3	***Competency 4:*** Engage in Practice-Informed Research and Research-Informed Practice	Draws on developmental science, diagnostic standards, and evidence-based assessment practices to inform identification.
Describe the purpose and structure of Early Intervention (EI)	***Competency 5:*** Engage in Policy Practice	Connects IDEA policy, eligibility criteria, and funding structures to service access and implementation.
Identify the roles of multidisciplinary EI providers	***Competency 6:*** Engage with Individuals, Families, Groups, Organizations, and Communities	Emphasizes interdisciplinary collaboration and family engagement in early childhood systems.

Summarize core EI service areas	***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities	Focuses on developmental assessment domains used to determine service needs and outcomes.
Explain the IFSP process and components	***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities	Aligns with intervention planning, service coordination, and measurable outcome development.
Apply the Person-Centered Ethics of Care Model	***Competency 2:*** Engage Diversity and Difference in Practice	Centers family systems, cultural humility, and individualized planning within diverse early childhood contexts.
Assess the importance of early intervention and transition planning	***Competency 3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Highlights equity in early access to services and long-term developmental opportunities.

Recognizing Developmental Delays in Early Childhood

Early childhood is a critical period for development, and recognizing the signs of developmental delays is essential for timely and effective intervention (Poon et al., 2010; Singh & Anekar, 2018). During this period, developmental delays may emerge across cognitive, communication, social-emotional, behavioral, sensory, or physical realms. Recognizing early indicators of developmental delays is critical so your children can receive adequate support that promotes development and long-term outcomes.

Diagnosing Children Ages 0–3

Identifying intellectual and developmental disabilities (I/DD) in early childhood can occur at multiple points before and after birth. Understanding these pathways is essential for timely referral and intervention.

Children may receive a **prenatal diagnosis** through genetic testing or prenatal imaging. Conditions such as Down syndrome are often identified before birth. This allows families and providers to plan for early interventions or support. **Premature birth**, defined as delivery before 37 weeks of gestation, also increases the risk for I/DD and related conditions, including vision and hearing impairments. In other cases, **postnatal developmental delays** appear after birth. While some children show temporary delays that improve with appropriate services, others may later meet diagnostic criteria for conditions such as intellectual disability, autism spectrum disorder, cerebral palsy, or epilepsy.

A formal diagnosis of I/DD depends on the condition and requires assessment by qualified clinicians. Licensed psychologists or neuropsychologists diagnose *intellectual disability* using standardized tests of intelligence and adaptive functioning (Tassé & Mehling, 2016). Psychologists, physicians, or interdisciplinary teams can diagnose autism. Cerebral palsy is typically diagnosed by a physician or medical team using clinical evaluation and neuroimaging,

while epilepsy is diagnosed by pediatric neurologists. However, **late identification** remains a concern, particularly for children with mild I/DD. In some cases, delays are not recognized until elementary school, when academic and social expectations exceed a child's unsupported abilities. Therefore, **early detection** is critical across all pathways because it allows for timely intervention and significantly improves developmental, educational, and family outcomes.

What Is Early Intervention (EI)?

Early Intervention (EI) is a system of services that supports children from birth to age three who have developmental delays or diagnosed disabilities, as well as their families. These services play a key role in encouraging early development and helping families during a crucial period of growth. **Individualized services** are tailored to each child and family, addressing each child's developmental needs while helping caregivers understand and respond to their child's strengths and challenges. These programs are publicly funded and available in every U.S. state and territory, providing services at no or low cost to eligible families.

The goal of EI is to improve a child's ability to learn new skills, lessen the effects of developmental challenges, and support long-term success in school and daily life. Services vary in intensity, ranging from brief weekly sessions to intensive support of up to 40 hours per week, depending on the child's needs. Providers work closely with families, focusing on strategies that can be easily incorporated into daily routines to foster ongoing learning and development. It is important to note that EI services differ by state.

Focus Areas of Early Intervention

Early Intervention services target key areas of adaptive and developmental functioning to help young children reach developmental milestones and increase independence. These services develop **gross and fine motor skills**, including balance, strength, coordination, grasping, and object manipulation. **Communication** abilities are also a central focus, with support that enhances speech, language, and the use of alternative or assistive communication methods when needed. In addition, Early Intervention emphasizes the development of **adaptive living skills** (ADL) and self-help/independent living skills that support daily activities such as feeding, dressing, and toileting, promoting functional participation in everyday routines.

Specialists Involved in Early Intervention

Early Intervention (EI) services are delivered through a **multidisciplinary team** of professionals who collaborate to support young children's developmental needs and their families' well-being. Each team member brings specialized expertise that contributes to comprehensive, coordinated care. For example, social workers assess social-emotional development, help families navigate systems of care, and connect caregivers to community resources and benefits. Speech and language pathologists address communication and language development, including speech production, early literacy, and feeding or swallowing concerns. Physical therapists focus on gross motor development, balance, strength, coordination, and mobility, and may recommend assistive or adaptive technology to enhance participation. Occupational therapists support fine motor skills, sensory processing, play, and environmental adaptations that promote self-care, independence, and

engagement in daily routines. Psychologists and developmental therapists address behavioral concerns, emotional regulation, learning, and relationship development, supporting both child functioning and family understanding. Nurses manage medical needs, including medication administration, health monitoring, mobility supports, and care for medically complex children. Nutritionists provide guidance on feeding, growth, and dietary needs that may affect development. Audiologists assess and address hearing-related needs that can impact communication and learning, while vision specialists support children with visual impairments and related developmental challenges through assessment and targeted interventions.

Skill Development Services

Building on the service framework outlined in Section: *What Is Early Intervention?* and the multidisciplinary roles described in Section: *Who Provides Early Intervention Services?*, Early Intervention targets core developmental domains that support functional growth in early childhood. These domains include physical skills, such as gross and fine motor abilities involved in reaching, crawling, walking, drawing, and building; cognitive skills related to early thinking, learning, attention, and problem-solving; and communication skills, including expressive and receptive language, early literacy, and the use of alternative or assistive communication when needed.

Early Intervention also focuses on developing self-help and adaptive skills that promote independence in daily activities such as eating and dressing, as well as social and emotional skills that support play, interaction, and emotional regulation. In addition, sensory processing skills are addressed to help children respond to and manage sensory input such as textures, sounds, tastes, and movement. These developmental domains guide the selection of services and interventions described earlier in the chapter and inform individualized planning for each child and family. With these developmental domains guiding service selection and intensity, the following section examines how Early Intervention services are funded and how eligibility for services is determined.

Early Intervention Funding and Eligibility

Early Intervention (EI) services are funded under the Individuals with Disabilities Education Act (IDEA) (2004), with specific funding mechanisms and eligibility criteria determined at the state level. Although IDEA establishes a federal framework for EI, each state administers its own program. Therefore, resulting in differences in regulations, procedures, and service delivery models.

Funding for EI is provided through federal grants allocated to each state. This ensures that eligible children can receive services at no or low cost. While private or public insurance may help offset some expenses, eligibility for EI services is not dependent on insurance coverage or family income. Access is based on developmental need rather than financial status. This reflects the law's emphasis on early support and equity.

Eligibility criteria vary by state but generally fall into two categories. Some children automatically qualify for EI services based on a diagnosis known to be associated with

developmental delays, such as Intellectual Disability, Down syndrome, autism spectrum disorder, or cerebral palsy. Other children qualify by demonstrating significant developmental delays, often defined by scoring below established thresholds on standardized assessments across one or more developmental domains.

Theoretically, EI is designed as an **intervention** program rather than a **prevention** program. As a result, not all children with mild or emerging delays meet eligibility requirements, even though they may benefit from additional support. For these children, ongoing developmental surveillance, monitoring, and periodic re-evaluation remain important to ensure that services can be initiated promptly if delays become more pronounced.

The Individualized Family Service Plan (IFSP)

The Individualized Family Service Plan (IFSP) is a key component of the Early Intervention process. It serves as the formal document that outlines the specific services a child will receive and how those services will be delivered. The IFSP is designed to be both comprehensive and individualized, detailing the child's current level of functioning, identified needs, and clearly defined treatment goals, referred to as outcomes. These outcomes guide service delivery and provide a framework for monitoring developmental progress over time.

The IFSP is grounded in a **family-centered approach** and recognizes that supporting the family is essential to supporting the child's development. Families play a central role in shaping the plan by contributing their knowledge of the child's daily routines, strengths, challenges, and priorities. The IFSP reflects a holistic understanding of early development in everyday life and addresses family needs alongside child outcomes.

The IFSP is developed through a collaborative process involving a multidisciplinary team. This team typically includes family members, a family advocate, a service coordinator, and relevant professionals involved in the child's care. Federal regulations require that the IFSP be developed within 30 days of the child's EI eligibility being determined. During this process, the team gathers and reviews all relevant information, including prior evaluations and assessments. Families are encouraged to communicate openly, share concerns and goals, keep meeting records, and prepare questions to ensure meaningful participation in decision-making.

Each IFSP includes several required components. These include a list of all individuals and organizations involved in the child's care, a detailed description of the child's current functioning across developmental domains, and specific, measurable outcomes that the child is expected to achieve. Outcomes commonly focus on areas such as motor development, communication, and self-help skills and serve as the foundation for selecting appropriate services and supports.
After the IFSP

The IFSP is reviewed and revised regularly to reflect the child's developmental progress and evolving needs. By law, the IFSP team must meet at least every six months to review outcomes, assess progress, and make any necessary adjustments to services or goals. Families may also request additional reviews at any time if they believe the plan is not adequately meeting their child's needs or if there has been a significant change in the child's condition or circumstances.

Early Intervention services typically continue until the child reaches age three, although some states may offer extended services beyond this age. As children approach the end of EI eligibility, the team begins planning for the transition to preschool services. This transition process helps ensure continuity of support as children with I/DD move into early childhood education programs that continue to promote development, learning, and participation.

With the IFSP guiding individualized goals and coordinated services, the following section reintroduces the Person-Centered Ethics of Care Model, which provides a broader framework for understanding how multiple systems shape early intervention experiences.

SAMPLE IFSP OUTCOME

Individualized Family Service Plan — Communication Domain

CHILD	AREA OF DEVELOPMENT
24-month-old	Communication — expressive language delay

OUTCOME STATEMENT

Within six months, the child will use at least 10 functional words or signs to express wants and needs during daily routines — such as meals, playtime, and dressing — as observed and reported by caregivers and Early Intervention providers.

STRATEGIES AND SUPPORTS

1	Speech-language therapy sessions twice per week
2	Caregiver coaching on modeling language during daily routines
3	Use of gestures or picture supports as needed to facilitate communication

MEASUREMENT OF PROGRESS

Progress will be measured through caregiver reports, therapist observation, and documentation during Early Intervention visits.

FAMILY PRIORITY

> *"Caregivers would like the child to better communicate needs to reduce frustration during daily activities."*

IFSP TEAM MEMBER SIGNATURE	PARENT/GUARDIAN SIGNATURE
DATE	DATE

Figure 3.1. *Sample IFSP outcome illustrates a family-centered, measurable goal within the communication domain. Alford & Son, 2026.*

Understanding Early Intervention Through the I/DD Person-Centered Ethics of Care Model Framework (I/DD PC-EoC)

The Intellectual and Developmental Disabilities Person-Centered Ethics of Care Model (Alford & Son, 2025) Figure 3.1 provides a theoretical framework for understanding how Early Intervention operates within a network of interconnected systems that shape a child's development and family experience. Reintroduced here, this model builds on earlier chapters by emphasizing that early intervention does not occur in isolation but within overlapping medical, educational, social service, and governmental systems. The model highlights the ethical importance of centering the child and family, recognizing relationships, responsiveness, and coordination as essential elements of effective and equitable care.

Within the context of Early Intervention, the I/DD PC-EoC Model reveals the importance of viewing development through a holistic lens. Rather than focusing solely on discrete skills or diagnoses, the model prioritizes the lived experiences of children and families, the quality of collaboration and relationships with service providers, and the accessibility of systems that support growth and well-being. From this perspective, families are often navigating a diagnosis for the first time and must interact with multiple systems simultaneously; it is critical.

Service providers play a critical role within this framework. The EI process often begins with a referral from a professional, such as a physician, a social worker, or an early childhood provider, who identifies potential developmental concerns. Parents and caregivers may also initiate the process by contacting their county health department for developmental surveillance and EI assessment. Once a child is referred, families work closely with service providers, therapists, and service coordinators to navigate the complex systems that influence their child's development. These systems may include healthcare providers, early childhood education programs, social services, and state agencies responsible for funding and insurance coverage.

Through the lens of the I/DD Person-Centered Ethics of Care Model (IDD PC-EoC), effective Early Intervention depends on the availability of services and on how well systems communicate, collaborate, and adapt to family needs. This model reinforces professionals' ethical responsibility to reduce fragmentation, support continuity of care, and empower families as active partners in decision-making. Situating EI within this broader ethical and systems-based framework, social workers and other providers are better equipped to support children with I/DD and their families in ways that are responsive, coordinated, and grounded in respect and equity.

The Early Intervention Person-Centered Systems Model (EI-PCSM)

The Early Intervention Person-Centered Systems Model (EI-PCSM) provides a comprehensive framework for understanding how early intervention services for young children with intellectual and developmental disabilities (I/DD) are organized, delivered, and experienced. Structured as a series of interconnected layers, the model captures the dynamic relationship between development, family engagement, service delivery, and ethical practice.

At the center of the model is the child within the family system, emphasizing that development occurs within relationships, daily routines, and caregiving environments. This

positioning reflects a core principle of the Person-Centered Ethics of Care (PC-EoC) (Alford & Son, 2025): individuals are inherently relational and cannot be understood outside their social and familial contexts. By centering the family alongside the child, the model reinforces relational autonomy and the importance of shared decision-making.

Surrounding the core are the developmental domains—motor, cognitive, communication, social-emotional, behavioral, and sensory functioning—which guide assessment and intervention planning. Within a PC-EoC lens, these domains are not viewed as deficits to be corrected, but as areas of growth shaped by environmental supports, relationships, and access to resources. This perspective promotes dignity and respect by recognizing the whole child rather than reducing them to a diagnosis.

The next layer represents early intervention processes, including early identification, the development of the Individualized Family Service Plan (IFSP), and service delivery in natural environments. This layer operationalizes PC-EoC principles by ensuring that services are family-centered, functional, and embedded in everyday life. The IFSP process reflects relational collaboration, as families actively contribute to goal setting and decision-making, reinforcing their role as experts in their child's life.

Encircling these processes is the multidisciplinary system of care, which includes social workers, therapists, medical providers, and service coordinators. This layer supports PC-EoC by emphasizing interdependence and coordinated care, recognizing that no single provider can meet all needs in isolation. Effective collaboration reduces fragmentation and ensures that services are responsive, continuous, and aligned with family priorities.

The outermost layer is grounded explicitly in the Person-Centered Ethics of Care (PC-EoC) (Alford & Son, 2025), which serves as the ethical and theoretical foundation for the entire model. Principles such as dignity and respect, relational autonomy, cultural responsiveness, and structural awareness shape how services are delivered across all layers. This ensures that care is technically appropriate, ethically grounded, culturally attuned, and responsive to systemic inequities that may affect access and outcomes.

Importantly, the EI-PCSM model aligns with PC-EoC and operates it. Each layer translates ethical principles into practice: relationships guide assessment, families shape decision-making, environments contextualize intervention, and systems are coordinated to support continuity of care. In this way, the model demonstrates how PC-EoC moves from a theoretical framework to a lived practice within early intervention systems.

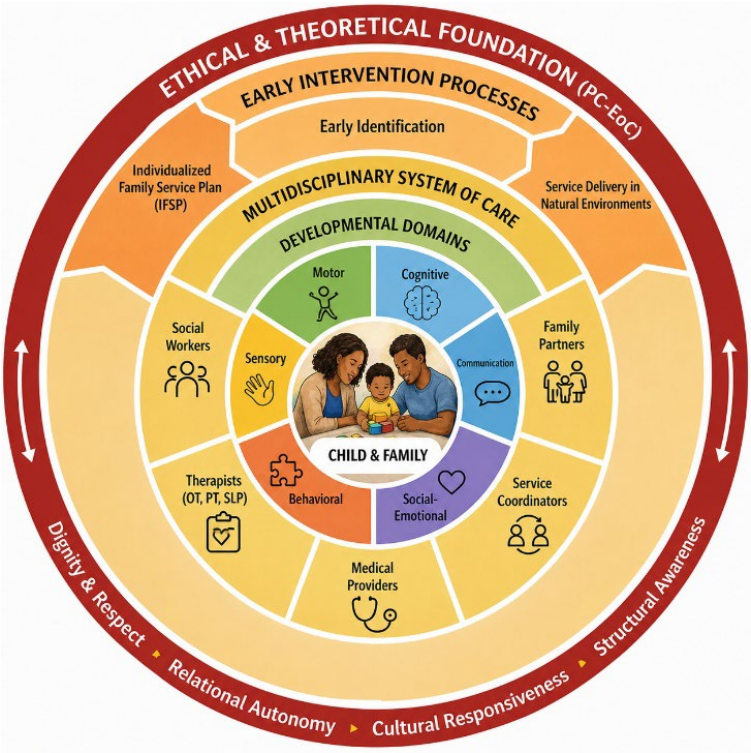


Figure 3.2. *The Early Intervention Person-Centered Systems Model (EI-PCSM) (Alford & Son, 2026)*

Pause and Reflect

- How does the Early Intervention Person-Centered Systems Model (EI-PCSM) expand upon traditional approaches to Early Intervention? In what ways does centering the child and family strengthen interdisciplinary collaboration and service delivery?
- The EI-PCSM integrates ethical principles, developmental domains, service providers, and Early Intervention processes into one framework. Why is it important for social workers to understand how these components interact rather than viewing them as separate parts of the intervention process?
- Imagine you are the social worker assigned to a two-year-old child recently referred to Early Intervention. Using the EI-PCSM, describe how you would coordinate with the family and interdisciplinary team to develop a comprehensive, person-centered plan of care.
- The outer ring of the EI-PCSM emphasizes ethical principles, including dignity and respect, relational autonomy, cultural responsiveness, and structural awareness. How can these principles guide decision-making when families have different cultural values, priorities, or expectations regarding their child's development and services?

Conclusion

Early Intervention is a critical component in the care of children with I/DD. Grounded in the Person-Centered Ethics of Care (PC-EoC)**,** early intervention emphasizes relational, family-

centered, and ethically responsive practice that honors the dignity, autonomy, and lived experiences of children and their families. Within this framework, social workers play a central role in engaging families, conducting holistic assessments, coordinating services, and advocating for equitable access to care across systems. As illustrated in the Early Intervention Person-Centered Systems Model (EI-PCSM), these practices are operationalized through the integration of developmental domains, early identification, IFSP planning, multidisciplinary collaboration, and service delivery within natural environments. Social workers and interdisciplinary teams can significantly improve outcomes and help build a strong foundation for children's long-term development and family well-being by recognizing developmental delays early and providing targeted, coordinated support.

Discussion Questions:

1. Why is early intervention crucial for children with I/DD, and what are the long-term benefits of these services?
2. What role do social workers play in supporting families during the early intervention process?
3. How does the Individualized Family Service Plan (IFSP) differ from an Individualized Education Plan (IEP), and why is this distinction important?
4. What are the key components of the Person-Centered Ethics of Care Model, and how does this model guide early intervention practices?
5. How can social workers help families navigate the early intervention system and advocate for their child's needs?

Case Scenario: Early Intervention for a Child with Developmental Delays

Case Scenario: Ava

Ava is a 2-year-old girl who lives with her parents, Emily and David, in a suburban neighborhood. Emily and David first noticed that Ava was not meeting her developmental milestones when she was about 18 months old. While other children her age were starting to speak in simple sentences and engage in more complex play, Ava was using only a few words and preferred repetitive activities, such as lining up her toys. She also seemed less interested in interacting with other children and often avoided eye contact.

Concerned about Ava's development, Emily and David brought her to their pediatrician, who conducted a developmental screening. The screening revealed delays in several areas, including language, social interaction, and fine motor skills. The pediatrician referred Ava to an early intervention program for a comprehensive assessment.

Assessment and Early Intervention

A multidisciplinary team evaluated Ava, including a speech therapist, an occupational therapist, and a developmental psychologist. The assessment confirmed significant delays in communication and social skills, as well as challenges with fine motor coordination. The team recommended early intervention services, and Emily and David developed an Individualized Family Service Plan (IFSP) in collaboration with the team. The IFSP outlined specific goals, including increasing Ava's vocabulary, improving her ability to engage in social play, and developing fine motor skills, along with strategies for Emily and David to reinforce these goals at home.

Ava began weekly speech therapy sessions using play-based techniques, including picture books and toys, to encourage her to name objects and express her needs. The speech therapist also coached Emily and David in how to use these approaches during their daily routines. Through occupational therapy, Ava engaged in activities such as stacking blocks, drawing, and using playdough to build hand strength and coordination, and the family received home exercises to practice together.

The early intervention team also included a social worker who met regularly with Emily and David to address their concerns, provide emotional support, and connect them with a local parent support group where they could share experiences and learn from other families navigating similar challenges.

Progress and Family Involvement

Over the following months, Ava made significant progress. She began using more words to communicate, became more engaged during play, and demonstrated meaningful improvement in her fine motor skills. Emily and David were active and consistent participants throughout the process, applying the strategies provided by the therapy team and attending monthly meetings to review Ava's progress and adjust the IFSP as needed.

As Ava approached her third birthday, the early intervention team began planning her transition to preschool services through the Committee on Preschool Special Education (CPSE). Working closely with the family, the team took deliberate steps to ensure a smooth transition and continuity of support for Ava's ongoing development.

Discussion Questions

1. How did early intervention services benefit Ava, and why is it crucial to address developmental delays as early as possible?
2. What are the key components of an Individualized Family Service Plan (IFSP), and how did it guide the intervention strategies developed for Ava?
3. How did Emily and David's involvement in Ava's early intervention plan contribute to her progress? What role do parents play in the success of early intervention?
4. Why is a multidisciplinary team important in assessing and providing services for children with developmental delays? How did each professional contribute to Ava's development?
5. What steps should be taken to ensure a smooth transition from early intervention services to preschool services for children like Ava?

Key Terms and Definitions

Term	Definition
Adaptive Skills	Everyday skills that support independence and functioning including self-care, communication, and social participation.
Assistive Technology	Devices or tools used to support communication, mobility, learning, or daily functioning in young children with developmental delays.
Audiologist	A healthcare professional who assesses and treats hearing-related concerns that may impact communication and development.
Care Coordination	The process of organizing and managing services across multiple systems to ensure cohesive, continuous support for a child and family.
Cerebral Palsy	A group of neurological conditions affecting movement, posture, and coordination, often identified in early childhood.
Developmental Delay	A significant lag in one or more areas of development — such as motor, cognitive, communication, social-emotional, or adaptive skills, compared to age-expected norms.
Developmental Domains	Areas of child development commonly assessed in Early Intervention include physical, cognitive, communication, social-emotional, adaptive, and sensory functioning.
Early Detection	Identifying developmental concerns as early as possible enables timely assessment and intervention.
Early Intervention (EI)	A federally supported system of services for children from birth to age three with developmental delays or disabilities and their families, designed to promote development and family capacity.
Epidemiology	The study of the distribution, prevalence, and patterns of health and developmental conditions within populations.
Gross Motor Skills	Large-muscle movements such as sitting, crawling, standing, walking, and maintaining balance.
Fine Motor Skills	Small muscle movements involving the hands and fingers, such as grasping, drawing, and manipulating objects.
Individualized Family Service Plan (IFSP)	A written, family-centered plan that outlines a child's developmental needs, services, and measurable outcomes within the Early Intervention system.

Individuals with Disabilities Education Act (IDEA), Part C	Federal legislation that authorizes and funds Early Intervention services for infants and toddlers with developmental delays or disabilities.
Late Identification	The delayed recognition of developmental delays or disabilities often occurs when environmental or academic demands exceed a child's unsupported abilities.
Multidisciplinary Team	A group of professionals from different disciplines who collaborate to assess, plan, and deliver Early Intervention services.
Occupational Therapist (OT)	A professional who supports fine motor skills, sensory processing, play, self-care, and environmental adaptations in young children.
Person-Centered Ethics of Care Model	A theoretical framework emphasizing dignity, relationships, collaboration, and coordinated systems of care, centering the child and family in decision-making.
Physical Therapist (PT)	A professional who supports gross motor development, strength, coordination, mobility, and movement-related participation.
Premature Birth	Birth occurring before 37 weeks of gestation, associated with increased risk for developmental delays and disabilities.
Referral	The process by which a child is directed to Early Intervention for evaluation, often initiated by healthcare providers, educators, or family members.
Screening	A preliminary process for identifying potential developmental concerns and determining the need for further assessment.
Sensory Processing	The way the nervous system receives, interprets, and responds to sensory input such as sound, touch, movement, taste, and visual stimuli.
Service Coordinator	A designated professional who helps families navigate Early Intervention services, facilitates IFSP development, and ensures coordination across providers.
Social Worker (EI)	A professional who supports social-emotional development, family well-being, resource access, and system navigation within Early Intervention.
Speech and Language Pathologist (SLP)	A professional who assesses and treats communication, language, early literacy, and feeding or swallowing concerns.

Transition Planning	The process of preparing a child and family to move from Early Intervention services to preschool or other early childhood programs.
Vision Specialist	A professional who assesses and supports visual functioning and related developmental needs in young children.

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Chapter 4: Preschool and School-Aged Children with I/DD and the IEP

Learning Objectives

After completing this chapter, students will be able to:

- Describe the IEP process across preschool and school-age education.
- Identify eligibility pathways and federal protections under IDEA and Section 504.
- Explain the roles and responsibilities of CPSE and CSE teams.
- Evaluate the function of related services in supporting meaningful participation in school.
- Reflect on strategies to promote equity, family partnership, and self-determination in the IEP process.

Table 4.1. *Alignment of Chapter 4 Learning Objectives with CSWE EPAS (2022) Competencies:*

Chapter 4 Learning Objective	CSWE EPAS (2022) Competency	Alignment Rationale
Describe the IEP process across preschool and school-age education.	**Competency 1:** Demonstrate Ethical and Professional Behavior **Competency 6:** Engage with Individuals, Families, Groups, Organizations, and Communities	Understanding the IEP process prepares social workers to navigate educational systems professionally, while effectively engaging children, families, and interdisciplinary teams.
Identify eligibility pathways and federal protections under IDEA and Section 504.	**Competency 3:** Advance Human Rights and Social, Racial, Economic, and Environmental Justice **Competency 5:** Engage in Policy Practice	Knowledge of IDEA and Section 504 enables social workers to advocate for equitable access to education, protect students' legal rights, and apply educational policy to practice.
Explain the roles and responsibilities of CPSE and CSE teams.	**Competency 1:** Demonstrate Ethical and Professional Behavior **Competency 8:** Intervene with Individuals, Families, Groups, Organizations, and Communities	Understanding interdisciplinary roles strengthens ethical collaboration, role clarity, and coordinated intervention within educational systems.
Evaluate the function of related services in	**Competency 4:** Engage in Practice-informed	Evaluating related services promotes evidence-informed decision-making and interdisciplinary interventions that

supporting meaningful participation in school.	Research and Research-informed Practice **Competency 8:** Intervene with Individuals, Families, Groups, Organizations, and Communities	enhance students' educational participation and inclusion.
Reflect on strategies to promote equity, family partnership, and self-determination in the IEP process.	**Competency 2:** Advance Anti-racism, Diversity, Equity, and Inclusion (ADEI) in Practice **Competency 3:** Advance Human Rights and Social, Racial, Economic, and Environmental Justice **Competency 6:** Engage with Individuals, Families, Groups, Organizations, and Communities	Emphasizes culturally responsive practice, equitable educational opportunities, collaborative family engagement, and promotion of self-determination for children and youth with developmental disabilities.

Entering Preschool: Understanding the Committee on Preschool Special Education (CPSE) and the Transition from Early Intervention

As children with intellectual and developmental disabilities (I/DD) turn three, they transition from Early Intervention (Part C of IDEA) to preschool special education under Part B. This transition shifts from a family-centered care model to a school-centered educational planning model, guided by the CPSE. Families now collaborate with school professionals to develop an Individualized Education Program (IEP) that outlines services, goals, and supports for their child. Research underscores that supportive school–family collaboration during this transition leads to better engagement and developmental outcomes (Bruder, 2010; Fish, 2008).

The Role of the Committee on Preschool Special Education (CPSE)

The Committee on Preschool Special Education (CPSE) determines the special education needs and services for children aged three to five. Every school district maintains a CPSE committee charged with crafting an Individualized Education Plan (IEP) for each eligible child. The IEP is a legally mandated document that guides the child's educational experience and is periodically reviewed and adapted as the child grows and their needs evolve.

Eligibility for Special Education Services

To receive special education services, a child must be identified as having one or more specific disabilities that adversely affect educational performance and require specially designed

instruction. The IEP team determines eligibility through a comprehensive evaluation process that considers the child's developmental, academic, and functional needs. To qualify, the IEP team must determine that a child has one of the following recognized disability categories under IDEA: intellectual disability, autism spectrum disorder, blindness or visual impairment, deafness or hearing impairment, emotional disturbance, speech or language impairment, orthopedic impairment, traumatic brain injury, specific learning disability, other health impairment, multiple disabilities, deaf-blindness, or developmental delay for children ages three through nine.

The eligibility categories most commonly associated with intellectual and developmental disabilities include intellectual disability, autism spectrum disorder, multiple disabilities, developmental delay, and other health impairment, each reflecting distinct but often overlapping profiles of need that require individualized, specially designed instruction and support. Eligibility decisions are made using multidisciplinary evaluation, including cognitive, functional, behavioral, communication, and adaptive assessments (Yell et al., 2006).

Children may qualify for special education if a disability adversely affects educational performance and requires specially designed instruction. Eligibility categories commonly associated with I/DD include:

Eligibility Category	Examples of Related Needs
Intellectual Disability	Support for cognitive & adaptive functioning
Autism Spectrum Disorder	Communication, social, sensory supports
Multiple Disabilities	Complex medical/learning needs
Blindness/Deafness/Hearing Impairment	Access supports & communication services
Emotional/Behavioral Disability	Behavioral support planning
Other Health Impairments	Chronic conditions affecting learning

When Can a Child Have an IEP?

Children who are at least three years of age and meet the criteria for special education services are eligible for an IEP. For children with I/DD, the IEP is critical. Without an IEP, they cannot access the specialized services and support available through the public school system. Children who do not qualify for an IEP may still be eligible for a 504 plan, which provides targeted accommodations such as extended test times or preferential seating. While a 504 plan can meaningfully support student participation, it does not include the comprehensive, individualized services that an IEP provides and is therefore a more limited option for students with significant support needs.

The IEP Process

The IEP is a comprehensive, individualized document that outlines the special education services and supports a child will receive. Developed through a structured collaborative process, the IEP is designed to ensure that each eligible student receives an education tailored to their unique needs, strengths, and goals. The IEP team typically includes parents or caregivers as equal partners, a special education teacher, a general education teacher, a school psychologist or evaluator, relevant

service providers such as occupational therapists, physical therapists, and speech-language pathologists, a school administrator, and the student. Each team member contributes a distinct perspective so the IEP reflects the whole child rather than any single assessment or professional viewpoint.

Creating and Reviewing the IEP

The IEP is developed through a shared process which includes educators, specialists, and the child's family. This team effort ensures the plan reflects the child's unique needs, strengths, and educational goals. Parental involvement is a cornerstone of this process. Federal law under IDEA requires parents to be active participants in IEP meetings, not passive recipients of decisions made by professionals. Parents bring invaluable knowledge of their child's history, preferences, behaviors, and daily experiences that assessment alone cannot capture; therefore, their active participation helps ensure the IEP reflects the whole child rather than a narrow clinical picture.

Social workers play an important role in supporting families throughout this process, helping parents understand their rights, prepare for meetings, communicate their concerns effectively, and advocate for services that align with their child's needs and goals. To remain relevant and responsive, the IEP is reviewed at least once a year, allowing the full team, including the family, to evaluate the child's progress, address emerging challenges, and make necessary adjustments as the child grows and their needs evolve.

Steps of the IEP Process

The IEP process involves several key steps, each designed to ensure that the child receives appropriate services and support. The process typically begins with identification and pre-referral, during which initial concerns about a child's development may prompt early interventions initiated by the family or the school before pursuing a formal evaluation. If pre-referral interventions prove insufficient, a formal referral for special education evaluation is made. The child is then evaluated to determine whether they meet the eligibility criteria for special education services.

If the child is found eligible, the IEP team convenes to develop an individualized plan that outlines the specific services, supports, accommodations, and goals the child will receive. Once finalized, the IEP is implemented, with teachers and service providers delivering the prescribed services and supports in the child's educational setting. Throughout this process, the child's progress is monitored on an ongoing basis, and the IEP is reviewed at least annually to ensure it reflects the child's evolving needs, strengths, and goals.

The Six-Step Individualized Education Program (IEP) Process Model

The Six-Step Individualized Education Program (IEP) Process Model is a collaborative and cyclical process designed to identify educational needs, determine eligibility, develop individualized support, implement specialized services, and continuously monitor student progress. At the center of the model is the student, reflecting the person-centered philosophy of the Individuals with Disabilities Education Act (IDEA), in which educational decisions are guided by the student's strengths, needs, preferences, and goals. The six interconnected stages emphasize

ongoing collaboration among families, educators, related service providers, and the student to promote meaningful educational participation, inclusion, and long-term success.

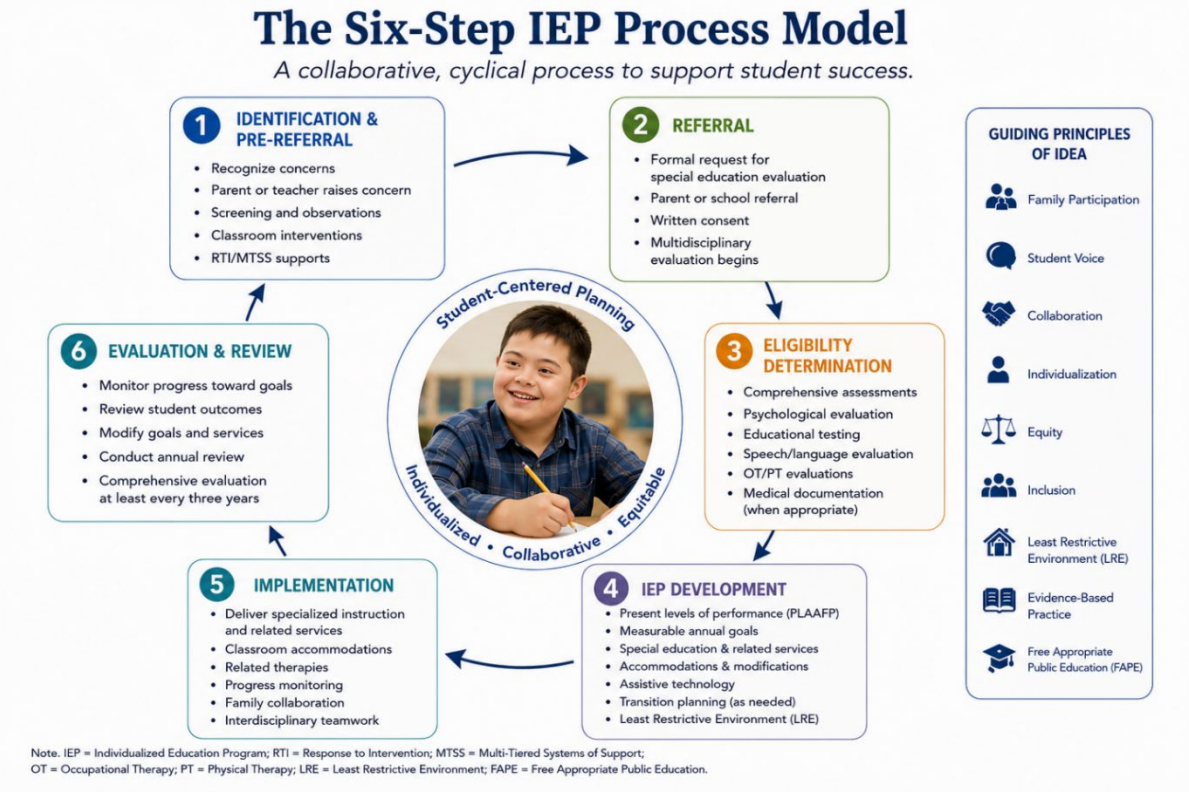


Figure 4.1. *The six-step Individualized Education Program (IEP) process, including identification, referral, evaluation, IEP development, implementation, and ongoing review* (Alford & Son, 2026)

Equity Note: Research shows that families from marginalized communities may face disparities in access, placement decisions, and service intensity, making advocacy and cultural responsiveness critical (Mueller & Buckley, 2014; Son & Kim, 2025).

The IEP Team, Document, and Related Services

The IEP team is central to creating and implementing an effective educational plan for each child. Bringing together diverse expertise, including educators, specialists, administrators, and family members, the team ensures that all aspects of the child's development are considered and that the IEP reflects the most current understanding of their needs and progress. Family involvement is not incidental to this process; it is essential. Parents and caregivers contribute knowledge of the child that no assessment alone can capture, and their active participation helps ensure that planning decisions are grounded in the child's full life experience.

A high-quality IEP is a detailed, individualized document that includes the child's present levels of academic and functional performance, measurable annual goals, special education and

related services, accommodations and assistive technology, and a plan for monitoring progress. Best-practice IEPs are strengths-based, clear and measurable, culturally and linguistically responsive, and focused on promoting participation rather than cataloging deficits.

The IEP is a living document, valid for one year from the date it is written and reviewed annually before its anniversary date. Prior to the annual meeting, notice is sent to all team members to schedule the review, at which the team assesses the child's progress, updates goals, and adjusts services as needed. Family members are strongly encouraged to participate in this review to ensure that the plan continues to meet the child's evolving needs.

In addition to educational services, the IEP may include related services that support the child's ability to benefit from special education. These may include speech therapy to build communication skills, occupational or physical therapy to support functional participation, transportation services, and behavioral intervention programs developed in collaboration with teachers and specialists. These services improve participation, communication, independence, and overall well-being (Case-Smith & Arbesman, 2008).

Conclusion: The Role of the IEP in Supporting Children with I/DD

The IEP is a cornerstone of special education, providing a structured, individualized approach to supporting the educational and developmental needs of children with I/DD. Through the collaborative efforts of the IEP team, including family members, educators, and specialists, the IEP ensures that each child receives the services and supports necessary to thrive in the school environment. As children progress through preschool and beyond, the IEP is continually adapted to meet their evolving needs, laying the foundation for lifelong learning and success.

Discussion Questions:

1. What are the main differences between the Committee for Preschool Special Education (CPSE) and the Committee for Special Education (CSE), and how do these committees impact on the services provided to children with I/DD?
2. How does the IEP process support the educational and developmental needs of children with I/DD? What challenges might arise during this process?
3. Why is it important for youth to participate in their IEP meetings, particularly as they approach adolescence?
4. How can social workers ensure that an IEP is effectively implemented and meets the individual needs of the child?
5. What role do related services, such as speech therapy and occupational therapy, play in supporting children with I/DD in the school setting?

Case Scenario: Developing and Implementing an IEP for a Child with Autism

Case Scenario: Jacob

Jacob is a 3-year-old boy diagnosed with Autism Spectrum Disorder (ASD) who has been receiving early intervention services since age two, including speech and occupational therapy. As Jacob approaches his fourth birthday, his parents, Maria and Carlos, are preparing for his transition from early intervention to preschool special education services under Part B of the Individuals with Disabilities Education Act (IDEA).

The Committee on Preschool Special Education (CPSE) schedules a meeting with Jacob's parents, his early intervention team, and school representatives to develop his first Individualized Education Plan (IEP). The team includes a special education teacher, a speech therapist, an occupational therapist, a psychologist, and the school principal.

During the meeting, Maria and Carlos share their concerns about Jacob's communication difficulties, challenges with social interaction, and sensory sensitivities. They also highlight his strengths — his ability to focus on tasks he enjoys, his natural curiosity, and his love for puzzles and building blocks.

Developing the IEP

The team agrees on several goals for Jacob's IEP, focusing on improving his communication skills, increasing his engagement in social play, and addressing his sensory needs. The IEP includes the following specific services and accommodations:

- **Speech Therapy: Jacob** will participate in speech therapy sessions twice per week to develop his expressive and receptive language skills.
- **Occupational Therapy:** Jacob will receive occupational therapy to support sensory integration and fine motor development, including using utensils during meals and holding a pencil.
- **Social Skills Development:** Jacob will participate in a small-group social skills program designed to help him practice turn-taking, sharing, and peer interaction in a structured environment.
- **Classroom Accommodations:** The IEP outlines supports including a designated quiet area, visual schedules to assist with transitions, and sensory tools such as noise-canceling headphones.

Implementing the IEP

Jacob begins preschool, and his teachers and therapists work together to implement the IEP. His speech therapist focuses on expanding his vocabulary and helping him use simple sentences to communicate his needs. The occupational therapist introduces activities to

strengthen fine motor skills and incorporates sensory breaks to help Jacob regulate his emotions throughout the school day.

The special education teacher collaborates closely with Maria and Carlos to promote consistency between school and home. Following the school's approach, the family introduces visual schedules at home to help Jacob understand daily routines and prepare for transitions.

Monitoring Progress

The IEP team meets with Jacob's parents every six months to review progress and make adjustments as needed. The team observes steady growth in Jacob's communication and social skills, though he continues to need support with sensory integration. The IEP is updated to include additional sensory activities and a stronger focus on helping Jacob initiate interactions with peers.

Discussion Questions

1. How does the IEP process ensure that Jacob's educational and developmental needs are met? What role do parents play in developing and implementing the IEP?
2. How are specific, measurable goals and objectives important in an IEP, and how do they guide the interventions provided to Jacob?
3. Why are classroom accommodations crucial for Jacob's success, and how can they be tailored to his individual needs?
4. How does collaboration between the IEP team, parents, and teachers contribute to Jacob's progress and well-being?
5. Why is it important to regularly review and adjust the IEP, and how does this process support Jacob's continuous development?

Key Terms and Definitions

Term	Definition
Committee on Preschool Special Education (CPSE)	A multidisciplinary committee responsible for determining the eligibility and special education needs of children ages 3–5 and developing their Individualized Education Programs (IEPs).
Individualized Education Program (IEP)	A legally required written plan developed under the Individuals with Disabilities Education Act (IDEA) that outlines a student's educational goals, special education services, accommodations, and related supports to meet their unique needs.
Part B of the Individuals with Disabilities Education Act (IDEA)	The section of IDEA that provides special education and related services to eligible children and youth with disabilities, ages 3–21, through an Individualized Education Program (IEP).
Related Services	Supportive services provided as part of an Individualized Education Program (IEP) to help a child benefit from special education and participate meaningfully in school. Examples include speech-language pathology, occupational therapy, physical therapy, counseling, transportation, and other specialized services.
The Six-Step IEP Process Model	A structured, legally grounded process through which eligible students with disabilities are identified, evaluated, and provided with individualized educational planning and services. The six steps are: identification, referral, evaluation, IEP development, implementation, and ongoing review.
└ Step 1: Identification	The process of recognizing that a student may have a disability that is affecting their educational performance, initiated by parents, educators, or other professionals who observe developmental concerns.
└ Step 2: Referral	A formal request submitted to the IEP team or school district requesting that a student be evaluated for special education eligibility, triggered when initial concerns cannot be resolved through general education interventions.
└ Step 3: Evaluation	A comprehensive, nondiscriminatory assessment conducted by a multidisciplinary team using multiple tools and sources to determine whether the student has a qualifying disability and to identify their educational needs.
└ Step 4: IEP Development	The collaborative process through which the IEP team — including parents, educators, and specialists — develops the

	student's individualized plan, establishing goals, services, accommodations, and placement.
└ **Step 5: Implementation**	The delivery of the services and supports outlined in the IEP by teachers, therapists, and other providers across educational settings, with consistency maintained between school and home where possible.
└ **Step 6: Ongoing Review**	The annual review and ongoing monitoring of the student's progress toward IEP goals, with adjustments made as needed to ensure the plan continues to meet the student's evolving needs and circumstances.

Note. *Related services are provided when necessary to support a child's educational participation and progress under the Individuals with Disabilities Education Act (IDEA).*

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Chapter 5: School-Aged Children and Teens with I/DD and Transition to Adulthood

Learning Objectives

By the end of this chapter, readers will be able to:

1. Explain the transition from preschool to school-age special education services and the role of the Committee on Special Education (CSE).
2. Describe federal requirements for IEP transition planning and best-practice recommendations.
3. Analyze guardianship and supported decision-making options using a rights-based framework.
4. Identify pathways for accessing adult services through state developmental disability systems.
5. Apply transition-planning concepts to real-world case scenarios.

Table 5.1. *Alignment of Chapter 5 Learning Objectives with CSWE EPAS (2022) Competencies*

Chapter 5 Learning Objectives	CSWE EPAS (2022) Competency	Alignment Rationale
Explain the transition from preschool to school-age special education services and the role of the Committee on Special Education (CSE)	***Competency 1*****:** Demonstrate Ethical and Professional Behavior ***Competency 5:*** Engage in Policy Practice	Understanding the transition process and the role of the CSE prepares social workers to navigate educational systems professionally and apply federal and state policies that support continuity of services.
Describe federal requirements for IEP transition planning and best-practice recommendations	***Competency 5:*** Engage in Policy Practice ***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities	Evaluating decision-making options through a rights-based lens promotes self-determination, autonomy, equitable treatment, and protection of the civil rights of individuals with I/DD.
Analyze guardianship and supported decision-making options using a rights-based framework	***Competency 2:***Advance Anti-racism, Diversity, Equity, and Inclusion (ADEI) in Practice **Competency *3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Evaluating related services promotes evidence-informed decision making and interdisciplinary interventions that enhance students' educational participation and inclusion.
Identify pathways for accessing adult services through state developmental disability systems.	**Competency *5*:** Engage in Policy Practice **Competency *8*:** Intervene with Individuals, Families,	Understanding eligibility and service systems enables social workers to connect individuals and families with community resources and coordinate effective transition services.

	Groups, Organizations, and Communities	
Apply transition-planning concepts to real-world case scenarios.	***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities ***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities ***Competency 9:*** Evaluate Practice with Individuals, Families, Groups, Organizations, and Communities	Applying transition-planning concepts to case scenarios strengthens the assessment, intervention, interdisciplinary collaboration, and evaluation skills needed for an effective transition to adulthood.

From Preschool to School Age: The Role of the CSE

As children with intellectual and developmental disabilities (I/DD) transition from preschool to school age, responsibility for their education and services shifts from the Committee on Preschool Special Education (CPSE) to the Committee on Special Education (CSE). Under the Individuals with Disabilities Education Act (IDEA), school-aged students who meet eligibility criteria are entitled to a Free Appropriate Public Education (FAPE), delivered through an Individualized Education Program (IEP) (IDEA, 2004). In New York State, the CSE oversees services for children ages 5 through 21, conducts annual IEP reviews, makes placement decisions, and coordinates services across educational settings. Research consistently demonstrates that youth participation in IEP meetings, usually beginning around age 14, is strongly associated with improved self-determination, post-school employment, and community participation outcomes (Martin & Zhang, 2020).

Future Planning for the School-Aged Child

Transition planning is a critical component of the educational process for students with I/DD, focusing on preparing them for life after high school. Although IDEA requires transition planning to begin no later than age 16, many states, including New York, initiate planning at age 14 to allow sufficient time for skill development, self-determination, and coordination of postsecondary services (Kellems & Morningstar, 2010). Best practice recommends beginning this planning process as early as age 12, ensuring students are well prepared for the challenges and opportunities of adulthood well before their school years conclude.

What Is IEP Transition Planning?

IEP transition planning is a legally required, student-centered process governed by IDEA. It involves setting meaningful goals for the student's post-high school life and identifying the services and support needed to achieve them. Postsecondary goals are typically organized across four domains: vocational training, postsecondary education, employment, and independent living. Once these goals are established, the IEP team identifies the transition services necessary to

support the students in reaching them. These services may include specially designed instruction, related services such as therapy or counseling, community-based learning experiences, career and college counseling, and daily living skills instruction. Taken together, these components form a comprehensive, individualized roadmap that reflects each student's strengths, interests, and vision for their future, ensuring that transition is not an afterthought but a central and ongoing priority throughout the school years.

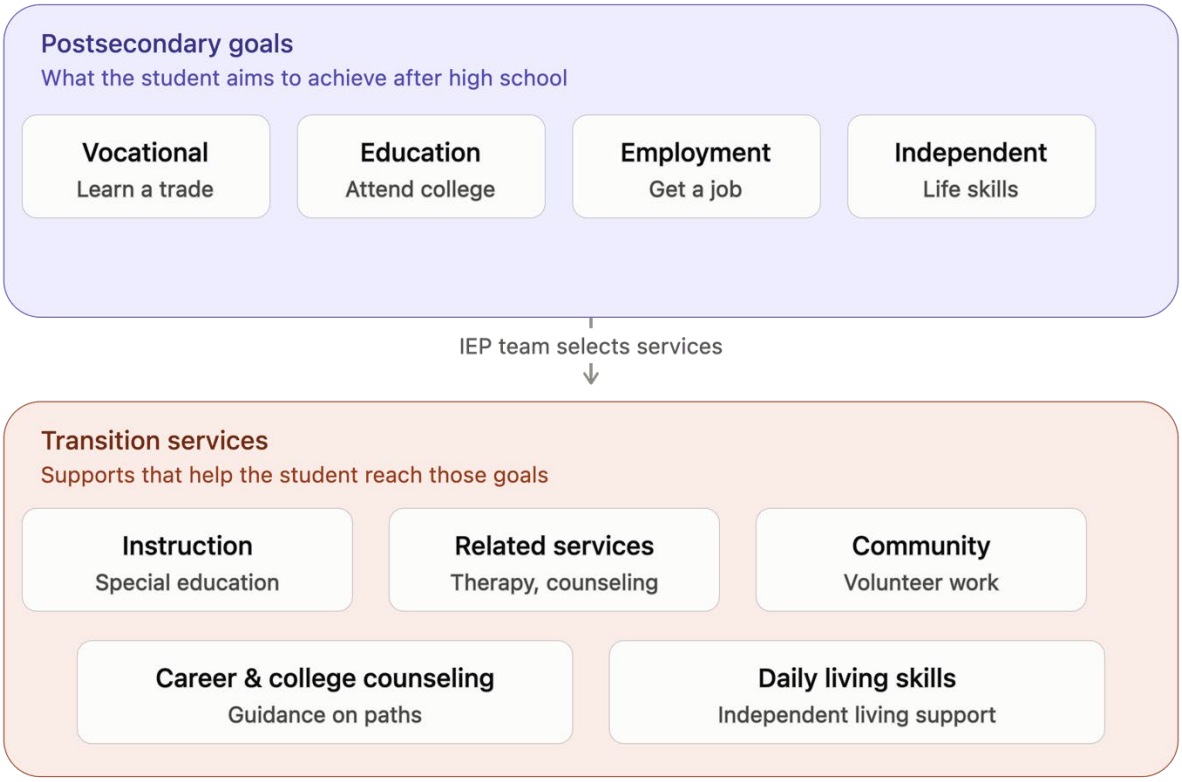


Figure 5.1. *Key Components of Transition Planning*

Best practice emphasizes strengths-based, culturally responsive, and person-centered planning, particularly for young people from racially, linguistically, and socioeconomically diverse backgrounds.

Guardianship: An Essential Part of Transition

Guardianship is an important consideration during the transition planning process, particularly for families of students with I/DD who may need ongoing support in making decisions as adults. Guardianship is a legal process through which a court appoints an individual or institution to manage the legal rights of an incapacitated person, including decisions related to finances, healthcare, or other aspects of daily life. Guardianship can be limited in scope, covering only specific areas such as medical decisions or financial management, or comprehensive, encompassing all aspects of an individual's life.

Alternative to Guardianship: Supported Decision-Making

Supported Decision-Making (SDM) offers a rights-affirming alternative to guardianship, allowing individuals with disabilities to retain their legal authority while receiving assistance from trusted supporters to understand information, weigh options, and communicate decisions. SDM is increasingly recognized in disability law and policy, consistent with the UN Convention on the Rights of Persons with Disabilities (Martinis et al., 2023). Social workers play a central role in helping families navigate the legal, ethical, and cultural considerations involved in choosing between guardianship and SDM, always centering the individual's right to self-determination.

The Office for People With Developmental Disabilities (OPWDD)

OPWDD is the primary state agency in New York responsible for coordinating services and supporting individuals with I/DD and their families across the lifespan. Partnering with approximately 500 nonprofit providers, OPWDD delivers a broad range of services including employment support, residential programs, day services, community living assistance, family support, respite care, assistive technology, and behavioral services. These services are primarily funded through Medicaid via the Home and Community-Based Services (HCBS) Waiver, offering flexibility and choice to individuals and families. The self-direction model, which allows families to manage their own staff, budgets, and schedules, reflects core social work values of self-determination, dignity, and person-centered practice (NASW, 2021).

Accessing OPWDD Services: The Front Door

Access to OPWDD services begins through the Front Door, the primary intake portal for individuals and families seeking services in New York State. Families must attend an information session, submit clinical and educational documentation, and complete an eligibility review and needs assessment. Early engagement is essential, as service waitlists and the complexity of Medicaid HCBS waivers make advance planning critical. To access programs through a Developmental Disabilities Service Organization (DDSO), a child must have a documented developmental disability that significantly limits their independence, along with eligibility for Medicaid and the HCBS waiver program. Once enrolled, individuals receive ongoing support through school and into adulthood, with services evolving to meet changing needs over time. Families can initiate the process by contacting their regional DDSO or state office of developmental disabilities directly.

Conclusion: Preparing for Adulthood

The transition from school to adulthood is a defining period for individuals with I/DD , one that requires careful planning, coordinated services, and the sustained involvement of families, educators, and professionals. Through person-centered transition planning, access to state services such as those provided by OPWDD, and the flexibility of self-directed supports, students with I/DD can work toward their postsecondary goals and build meaningful, independent lives in their communities. Social workers are uniquely positioned to guide this process, connecting families to resources, advocating for rights, and ensuring that each individual's voice remains central to every decision made on their behalf.

Discussion Questions:

1. How does transition planning within the IEP process prepare students with I/DD for life after high school? What are the key components of a successful transition plan?
2. What is the significance of guardianship for individuals with I/DD as they transition into adulthood, and what factors should families consider when deciding whether to pursue guardianship?
3. How do services provided by the Office for People with Developmental Disabilities (OPWDD) support the transition from school to adult life for individuals with I/DD?
4. What is the difference between traditional guardianship and supported decision-making, and why might a family choose one over the other?
5. How can social workers assist families in planning for the transition from school to work for individuals with I/DD?

Case Scenario: Transition Planning for a Teen with Intellectual Disability

Case Scenario: Samantha

Samantha is a 16-year-old girl with a mild intellectual disability who has been receiving special education services since elementary school. As she approaches the end of high school, her parents, her special education teacher, and the school's transition coordinator begin discussing her transition to adulthood. Samantha has expressed a clear interest in working with animals and hopes to find a job at a pet store or animal shelter after graduation.

Developing the Transition Plan

The transition coordinator schedules a meeting with Samantha, her parents, and her IEP team to develop a transition plan that will guide her through the next several years. The plan includes the following coordinated components:

Vocational Training: Samantha participates in a vocational training program offering hands-on experience working with animals. She spends part of her school day in this program and the remainder in the classroom, continuing to develop her academic skills.

Life Skills Training: Samantha receives instruction in essential life skills including managing money, using public transportation, and preparing simple meals, foundational competencies that support her growing independence as she prepares for adulthood.

Work-Based Learning: Samantha participates in a work-based learning program that places her in a local pet store one afternoon per week, allowing her to gain practical skills and explore the responsibilities of working in a retail environment.

Community Integration: The plan includes activities to support Samantha's involvement in her community, including volunteering at an animal shelter and participating in a social group for teens with disabilities.

Implementing the Transition Plan

Samantha begins her vocational training and work-based learning programs. She enjoys working with the animals, builds relationships with coworkers, and begins developing confidence in her abilities. Life skills training supports her independence at home, and she becomes more capable in managing everyday tasks.

The transition coordinator meets regularly with Samantha and her parents to monitor progress and adjust the plan as needed. Together, they discuss her long-term goals and

explore options for supported employment after high school, including job coaching services available through the Office for People With Developmental Disabilities (OPWDD).

Preparing for Adulthood

As Samantha approaches her 18th birthday, her parents begin exploring guardianship options to ensure they can continue supporting her in making important decisions. They also work with the transition coordinator to apply for adult services through OPWDD, including supported employment and residential options.

By the time Samantha graduates, she is well-prepared to transition into a supported employment program and pursue work in a field she is genuinely passionate about. The transition plan has provided her with the skills, experiences, and confidence needed to take this important step toward adult life.

Discussion Questions

1. What are the key components of a successful transition plan for a teen with an intellectual disability, and how do they prepare Samantha for adulthood?
2. How do vocational training and life skills instruction contribute to Samantha's independence and future employment opportunities?
3. Why is work-based learning important for Samantha's transition, and how does it help her develop practical skills and build confidence?
4. How does community involvement support Samantha's transition to adulthood, and what are the benefits of volunteering and participating in social activities?
5. What considerations should Samantha's parents consider when exploring guardianship and adult services, and how can these supports help Samantha achieve her goals?

Key Terms and Definitions

Term	Definition
Committee for Special Education (CSE)	A multidisciplinary team responsible for determining eligibility, developing, reviewing, and maintaining Individualized Education Programs (IEPs) for school-age children and youth with disabilities, typically ages 5–21.
Free Appropriate Public Education (FAPE)	A legal right under the Individuals with Disabilities Education Act (IDEA) that ensures eligible students with disabilities receive special education and related services at no cost to families and tailored to meet their unique educational needs.
Transition Planning	A process within the IEP that prepares students with I/DD for life after high school, focusing on postsecondary education, employment, and independent living.
Guardianship	A legal process where a court appoints a guardian to make decisions on behalf of an individual who is deemed unable to manage their own affairs due to disability.
Supported Decision-Making (SDM)	An alternative to guardianship where individuals with disabilities retain decision-making capacity with the support of trusted individuals.
Self-Directed Services	Medicaid-funded services allowing individuals and families to control service delivery.

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Chapter 6: I/DD: Behavioral Treatment and Management

Learning Objectives

By the end of this chapter, readers will be able to:

- Explain the challenges involved in diagnosing and managing challenging behaviors in individuals with intellectual and developmental disabilities (I/DD) and identify strategies social workers can use to support the assessment process.
- Describe the role of the Functional Behavior Assessment (FBA) in guiding the development of a Behavioral Intervention Plan (BIP) within the IEP process.
- Analyze the ethical considerations involved in using Applied Behavior Analysis (ABA) as a treatment approach for individuals with I/DD.
- Compare Positive Behavioral Supports (PBS) and Applied Behavior Analysis (ABA), including the circumstances under which each approach may be most appropriate.
- Identify alternative evidence-based behavioral programs, such as TEACCH, and evaluate how they complement or differ from ABA and PBS.

Table 6.1. *Alignment of Chapter 6 Learning Objectives with CSWE EPAS (2022) Competencies:*

Chapter Learning Objective	CSWE EPAS (2022) Competency	Alignment Rationale
Explain the challenges involved in diagnosing and managing challenging behaviors in individuals with I/DD and identify strategies social workers can use to support the assessment process.	***Competency 1:*** Demonstrate Ethical and Professional Behavior ***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities	Recognizing the diagnostic complexity of challenging behaviors in individuals with I/DD requires professional judgment and sound assessment skills, preparing social workers to contribute accurately and ethically to interdisciplinary evaluation processes.
Describe the role of the Functional Behavior Assessment (FBA) in guiding the development of a Behavioral Intervention Plan (BIP) within the IEP process.	***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities Competency 8: Intervene with Individuals, Families, Groups, Organizations, and Communities	Understanding how FBA findings translate into an individualized BIP strengthens the assessment-to-intervention continuum, ensuring behavioral supports are function-based, data-driven, and responsive to the student's needs.

Analyze the ethical considerations involved in using Applied Behavior Analysis (ABA) as a treatment approach for individuals with I/DD.	***Competency 1:*** Demonstrate Ethical and Professional Behavior Competency 2: Advance Anti-racism, Diversity, Equity, and Inclusion (ADEI) in Practice	Evaluating informed consent, potential for harm, and cultural responsiveness in ABA reflects core social work values of dignity, self-determination, and worth of the person, requiring practitioners to weigh intervention benefits against individual and family autonomy.
Compare Positive Behavioral Supports (PBS) and Applied Behavior Analysis (ABA), including the circumstances under which each approach may be most appropriate.	***Competency 3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice Competency 8: Intervene with Individuals, Families, Groups, Organizations, and Communities	Distinguishing PBS's lifestyle- and environment-centered approach from ABA's consequence-based methods equips social workers to select and advocate for interventions that affirm autonomy and rights while effectively addressing behavioral needs.
Identify alternative evidence-based behavioral programs, such as TEACCH, and evaluate how they complement or differ from ABA and PBS.	***Competency 4:*** Engage in Practice-Informed Research and Research-Informed Practice Competency 9: Evaluate Practice with Individuals, Families, Groups, Organizations, and Communities	Applying research on structured, strengths-based programs like TEACCH prepares social workers to critically evaluate the fit and effectiveness of behavioral interventions for individuals with differing strengths, needs, and learning styles.

Introduction

Individuals with intellectual and developmental disabilities (I/DD) often experience psychiatric disorders and engage in challenging behaviors that can be aggressive, destructive, or dangerous. These behaviors, coupled with difficulties in adaptive coping, can significantly impact quality of life, community integration, and overall well-being. Common examples include pica (the ingestion of non-food items), self-injurious behavior, aggression, socially inappropriate sexual behaviors, and fecal smearing. When left untreated, challenging behaviors can lead to the breakdown of community placements, social isolation, segregation, and hospitalization. Effective, evidence-based management of these behaviors is therefore essential not only for individual well-being but for sustaining meaningful participation in community life. This chapter examines the psychological and pharmacological approaches to behavioral intervention, providing students with a comprehensive foundation for evidence-based practice with individuals with I/DD.

What Is Behavior Therapy?

Behavior therapy is a foundational approach to managing challenging behaviors in individuals with I/DD. Rooted in the science of learning, it focuses on understanding the individual's learning history and the environmental antecedents and consequences that trigger or maintain behavior. Rather than viewing challenging behavior as a symptom to be suppressed, behavior therapy examines the functions those behaviors serve, such as gaining attention, accessing preferred items, or escaping from demands, and uses that understanding to design effective, individualized interventions (Lloyd & Kennedy, 2014). A thorough assessment identifies the factors reinforcing or maintaining the behavior, enabling clinicians to modify environmental contingencies and help the individual learn more adaptive alternatives. When integrated into a school-aged child's Individualized Education Program (IEP), behavior therapy can be delivered systematically across settings, ensuring continuity of support between home, school, and community.

Functional Behavior Assessment (FBA)

Functional Behavior Assessment (FBA) is a systematic, evidence-based process for identifying the underlying causes and functions of challenging behaviors. The FBA begins with a clear and operationalized definition of the **target behavior**, the specific behavior to be assessed and addressed. From there, the process involves a thorough analysis of the antecedents that trigger the behavior and the consequences that reinforce or maintain it. A critical component of the FBA is determining how the target behavior interferes with the individual's educational progress and broader participation in daily life. These elements provide clinicians, educators, and social workers with the information needed to design function-based, individualized behavioral interventions aligned with the person's needs and goals.

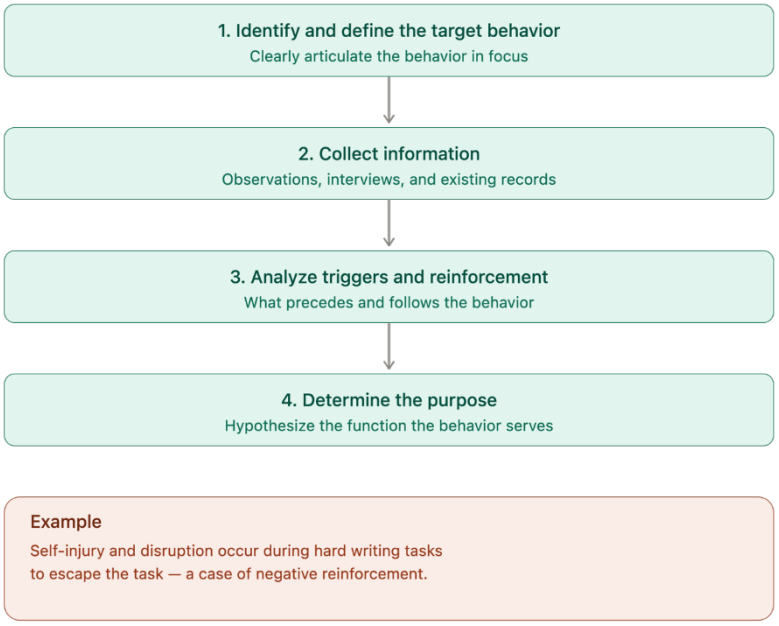


Figure 6.1. *Steps in Conducting a Functional Behavior Assessment (FBA)*

Pause & Reflect

- Looking at the steps of a Functional Behavior Assessment, how might a social worker's role differ from that of a behavior specialist or psychologist when conducting or contributing to an FBA? What unique perspective does a social worker bring to this process?
- Consider a student whose challenging behavior functions as a way to escape difficult tasks. How might an educator or clinician inadvertently reinforce that behavior, and what would need to change in the environment to interrupt that cycle?
- The chapter notes that ABA is not appropriate for everyone and should never be applied as a one-size-fits-all solution. What ethical considerations should guide a social worker's recommendation of ABA to a family, and how would you ensure the family is making a fully informed decision?
- How do the principles of Positive Behavioral Supports (PBS) align with the Person-Centered Ethics of Care Model? In what ways does PBS reflect a strengths-based, dignity-affirming approach to behavioral intervention?
- Challenging behaviors are sometimes described as a form of communication. What does this perspective suggest about how social workers and educators should respond when a child with I/DD exhibits aggressive or disruptive behavior?

Steps in Conducting an FBA

Conducting a Functional Behavior Assessment involves several sequential steps. The process begins by clearly identifying and defining the target behavior, articulating precisely what behavior will be the focus of assessment. Information is then gathered through direct observation, interviews with the child, parents, and staff, and a review of existing records to understand when, where, and under what conditions the behavior occurs. The next step involves analyzing the antecedents that precede the behavior and the consequences that follow and reinforce it. From this analysis, the team formulates a hypothesis about the purpose the behavior serves for the individual. For example, a student who engages in self-injury and classroom disruption when presented with a challenging writing task may be using that behavior to escape the difficult demand, a function known as **negative reinforcement**. This hypothesis then guides the development of a targeted, function-based intervention.

IEP Assessment Meeting

The FBA results are central to developing an effective Behavioral Intervention Plan (BIP). During the IEP meeting, the team may complete the FBA directly if sufficient information is available or determine how to gather additional data if needed. Once the FBA is complete and the function of the behavior is understood, the IEP team develops a BIP that incorporates positive behavioral support tailored to the student's specific needs, strengths, and learning context.

Behavioral Intervention Plans (BIP)

A **Behavioral Intervention Plan** is developed following the FBA and is designed to address specific behaviors that interfere with a student's learning and participation. The need for a BIP must be formally documented within the IEP, which must also specify any devices, services, or accommodations required to address the target behavior. The BIP is reviewed at least annually to assess its effectiveness and make necessary adjustments as the students' needs evolve. Like the IEP itself, the BIP is a living document, intended to be responsive and adaptive rather than static.

Applied Behavior Analysis (ABA)

Applied Behavior Analysis (ABA) is one of the most widely used and researched behavioral interventions, particularly for autism spectrum disorder (Luiselli, 2015). Often referred to as the gold standard for autism treatment, ABA focuses on increasing helpful behaviors and decreasing harmful ones through systematic use of rewards and consequences, with the goal of improving language, communication, and daily living skills. ABA therapists, including social workers with behavioral training, use FBA to design and implement individualized interventions, which are often intensive and may span several years.

Ethical considerations are essential in the application of ABA. The approach is not appropriate for all individuals and should never be applied as a one-size-fits-all solution. Clinicians must carefully consider the potential for exacerbating behavior or causing psychological harm, and interventions must be culturally sensitive and respectful of the autonomy of both the individual and their family. Families should be fully informed about alternative behavior management approaches before consenting to ABA services.

Positive Behavioral Supports (PBS)

Positive Behavioral Supports (PBS) offers an alternative framework for managing challenging behaviors, one that emphasizes lifestyle and environmental change rather than direct behavioral consequences (LaVigna & Willis, 2012). Unlike ABA, which centers on targeted behavioral interventions, PBS focuses on creating supportive environments and reducing the conditions that give rise to challenging behaviors. PBS interventions are outcome-oriented, prioritize results meaningful to the individual and their family, and rely on reinforcement rather than punishment to promote positive behavior change. In practice, PBS involves teaching replacement behaviors that are more adaptive than the target behavior, rewarding neutral or socially appropriate responses, helping individuals recognize and avoid behavioral triggers, and building on individual strengths to support success in school and community settings.

Other Behavioral Programs: TEACCH

In addition to ABA and PBS, other structured behavioral programs are available for individuals with I/DD. The **Treatment and Education of Autistic and Related Communication Handicapped Children (TEACCH)** program is an evidence-supported approach (Zhou et al., 2024) designed specifically for autistic children that leverages their characteristic strengths in visual detail processing and memory. TEACCH employs visual schedules, concrete visual learning

materials, and environmental adaptations to support both learning and behavioral regulation, creating structured, predictable environments in which autistic individuals can thrive.

Conclusion

Effective behavioral management for individuals with I/DD requires a multifaceted approach that integrates thorough assessment, individualized intervention planning, and ongoing review. Whether through ABA, PBS, TEACCH, or other evidence-based programs, the goal is always to improve quality of life, promote adaptive functioning, and support the individual's full participation in school and community. Ethical considerations and cultural sensitivity are not ancillary concerns but foundational principles that must guide every stage of the assessment and intervention process, ensuring that behavioral supports affirm rather than diminish the dignity and autonomy of each person served.

Discussion Questions:

1. What are the challenges in diagnosing and managing challenging behaviors in individuals with I/DD, and how can social workers support this process?
2. How does the Functional Behavior Assessment (FBA) guide the development of Behavioral Intervention Plans (BIP) for students with I/DD?
3. What are the ethical considerations involved in using Applied Behavioral Analysis (ABA) as a treatment approach for individuals with I/DD?
4. How do Positive Behavioral Supports (PBS) differ from ABA, and in what situations might PBS be more appropriate?
5. What are some alternative behavioral programs, such as TEACCH, and how do they complement or differ from ABA and PBS?

Case Scenario: Addressing Challenging Behavior in a Child with I/DD

Case Scenario: Ethan

Ethan is a 10-year-old boy with moderate intellectual disability and a diagnosis of Attention-Deficit/Hyperactivity Disorder (ADHD). Over the past year, his parents and teachers have become increasingly concerned about his aggressive behavior, which includes hitting and kicking classmates when frustrated, throwing objects when upset, and refusing to follow instructions. These behaviors are significantly impacting his ability to learn and socialize, and have resulted in several incidents where he was sent home from school. His parents, Amanda and John, worry that if the behavior continues, Ethan may lose his placement in a mainstream classroom.

Functional Behavior Assessment (FBA)

The school's special education team, including a behavior specialist, conducts a Functional Behavior Assessment to identify the underlying causes of Ethan's behavior. The FBA includes direct observations across settings, interviews with his parents and teachers, and a review of his behavior records.

The FBA identifies several key triggers for Ethan's aggressive behavior, including difficulty with transitions between activities, frustration with tasks that exceed his current skill level, and sensory overload in noisy environments. Critically, the assessment also reveals that Ethan's aggressive behavior frequently results in removal from the classroom, which inadvertently reinforces the behavior by allowing him to escape overwhelming situations.

Developing a Behavioral Intervention Plan (BIP)

Based on the FBA findings, the school develops a Behavioral Intervention Plan tailored to Ethan's specific triggers and needs. The BIP includes the following strategies:

- **Positive Reinforcement:** Ethan is rewarded with praise, stickers, or extra playtime for using appropriate behaviors, such as asking for help or taking deep breaths when frustrated.
- **Structured Routines:** A visual schedule with pictures and consistent cues is implemented to help Ethan understand the daily routine and reduce anxiety around transitions.
- **Task Modification:** Academic tasks are adjusted to match Ethan's current skill level, with additional support provided for challenging work to reduce frustration and increase engagement.

- **Sensory Breaks:** Ethan receives regular sensory breaks in a quiet area of the classroom, where he has access to calming tools such as noise-canceling headphones and a weighted blanket.
- **Teaching Coping Skills:** The school counselor works with Ethan to develop strategies for managing frustration and anger, including deep breathing, counting to ten, and using a stress ball.

Monitoring and Adjusting the BIP

Ethan's parents, teachers, and the behavior specialist meet regularly to review his progress and make necessary adjustments to the BIP. Over time, Ethan begins to show meaningful improvement in managing his behavior. He uses the coping strategies he has learned, and positive reinforcement helps him understand the benefits of appropriate responses. While occasional incidents still occur, the frequency and intensity of his aggressive behavior decrease significantly, supporting his continued participation in the mainstream classroom.

Discussion Questions

1. **Functional Behavior Assessment:** How does the FBA process help identify the triggers and reinforcements of Ethan's challenging behavior, and why is this information crucial for developing an effective intervention plan?
2. **Behavioral Intervention Plan:** What are the key components of Ethan's BIP, and how do they directly address the specific triggers identified through the FBA?
3. **Positive Reinforcement:** How does positive reinforcement play a role in changing Ethan's behavior, and what are the benefits of using this approach over punitive alternatives?
4. **Sensory Needs and Coping Strategies:** Why is it important to address Ethan's sensory needs and teach him coping strategies, and how do these interventions support his ability to manage frustration and anger?
1. **Monitoring and Adjustment:** Why is it necessary to regularly monitor and adjust the BIP, and how does this ongoing process contribute to Ethan's long-term success in managing his behavior?

Key Terms and Definitions

Term	Definition
Applied Behavior Analysis (ABA)	A widely researched behavioral intervention that uses systematic rewards and consequences to increase helpful behaviors and decrease harmful ones, with the goal of improving language, communication, and daily living skills, particularly in individuals with autism spectrum disorder.
Behavior Therapy	A foundational intervention approach rooted in the science of learning that examines the environmental antecedents and consequences maintaining challenging behavior, using that understanding to design individualized, function-based interventions.
Behavioral Intervention Plan (BIP)	A formal plan developed following a Functional Behavior Assessment that outlines targeted strategies, positive behavioral supports, and accommodations designed to address specific challenging behaviors and promote more adaptive alternatives.
Challenging Behaviors	Behaviors exhibited by individuals with I/DD that can be aggressive, destructive, or dangerous, including pica, self-injury, aggression, and socially inappropriate behaviors, which may negatively impact quality of life and community participation when left untreated.
Functional Behavior Assessment (FBA)	A systematic, evidence-based process for identifying the underlying causes and functions of challenging behaviors, including defining the target behavior, analyzing antecedents and consequences, and formulating a hypothesis to guide intervention planning.
Negative Reinforcement	A behavioral mechanism in which a behavior is strengthened because it results in the removal of an aversive or unwanted stimulus, such as a student engaging in disruption to escape a difficult task.
Pica	A challenging behavior characterized by the repeated ingestion of non-food items, commonly observed in individuals with intellectual and developmental disabilities and associated with significant health risks.
Positive Behavioral Supports (PBS)	An alternative behavioral framework that emphasizes lifestyle and environmental change, teaching replacement behaviors, building on individual strengths, and using

	reinforcement rather than punishment to reduce challenging behaviors and promote quality of life.
Target Behavior	The specific, clearly defined behavior that is the focus of a Functional Behavior Assessment and Behavioral Intervention Plan, operationalized in observable and measurable terms to guide assessment and intervention.
TEACCH	The Treatment and Education of Autistic and Related Communication Handicapped Children program, an evidence-supported approach that leverages visual learning strengths and structured environmental adaptations to support behavioral regulation and independent task completion in autistic individuals.

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Chapter 7: Social Work Practice and Service Delivery for Individuals with Intellectual and Developmental Disabilities

Learning Objectives

By the end of this chapter, students will be able to:

- Describe the role and function of OPWDD in coordinating services for individuals with I/DD in New York State.
- Explain the five components of the Person-Centered Ethics of Care (PC-EoC) Model and their application in service delivery.
- Identify key state-wide agencies and resources available to individuals with I/DD and their families.
- Apply principles of systems navigation, interagency collaboration, and advocacy in social work practice with individuals with I/DD.
- Articulate how person-centered practice promotes dignity, autonomy, and quality of life across the lifespan

Table 7.1. *Alignment of Chapter Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	CSWE Competency	Alignment Rationale
Describe the organization and delivery of I/DD services	Competency 5: Engage in Policy Practice	Examines disability service systems, policies, and public programs.
Explain the Person-Centered Ethics of Care Model	Competency 1: Demonstrate Ethical and Professional Behavior	Applies ethical, person-centered principles to professional practice.
Identify major state agencies serving individuals with I/DD	Competency 6: Engage with Individuals, Families, Groups, Organizations, and Communities	Emphasizes systems navigation and interdisciplinary collaboration.
Apply person-centered care coordination strategies	Competency 8: Intervene with Individuals, Families, Groups, Organizations, and Communities	Focuses on service planning, care coordination, and advocacy.
Analyze the role of social workers in service delivery	Competency 9: Evaluate Practice with Individuals, Families, Groups, Organizations, and Communities	Encourages evaluation of intervention effectiveness and service outcomes.
Evaluate strategies for equitable access to services	Competency 3: Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Promotes equitable access, inclusion, and disability rights.

Introduction to Social Work Practice and Service Delivery

Successful social work practice extends far beyond connecting individuals to available resources. Social workers serving individuals with **intellectual and developmental disabilities (I/DD)** must understand complex service systems, coordinate interdisciplinary care, advocate across multiple agencies, and ensure that services remain person-centered throughout the lifespan. Individuals with I/DD often interact with educational, healthcare, governmental, financial, and community-based systems simultaneously; therefore, effective practice requires an integrated understanding of how these systems function together rather than independently.

This chapter examines the organization and delivery of services for individuals with I/DD through the lens of the **Person-Centered Ethics of Care (PC-EoC) Model** (Alford & Son, 2025), which serves as the organizing framework for disability service delivery throughout this textbook. Attention is given to the role of the New York State Office for People With Developmental Disabilities (OPWDD), interdisciplinary collaboration, care coordination, systems navigation, and social work advocacy, which prepare social workers to promote equitable access to services while supporting the dignity, autonomy, and quality of life of individuals with I/DD and their families.

Understanding the Office for People With Developmental Disabilities (OPWDD)

The **New York State Office for People With Developmental Disabilities (OPWDD)** (Figure 9.1) is the primary governmental agency responsible for coordinating services for individuals with developmental disabilities who were born before age 22 (*Office for People With Developmental Disabilities*, n.d.). OPWDD's mission is grounded in promoting person-centered services that maximize independence, community inclusion, self-determination, and overall quality of life.

OPWDD offers a comprehensive continuum of support, including residential services, day habilitation programs, supported employment, community habilitation, respite services, family support services, self-direction, and clinical support. Rather than expecting individuals to fit into predetermined service models, OPWDD emphasizes flexible, person-centered planning that allows them to receive support in the communities where they choose to live. Although OPWDD directly administers selected programs, most services are delivered through partnerships with hundreds of nonprofit organizations, healthcare providers, residential agencies, and community organizations throughout New York State. This provider network enables services to be tailored to each person's strengths, preferences, goals, and support needs.

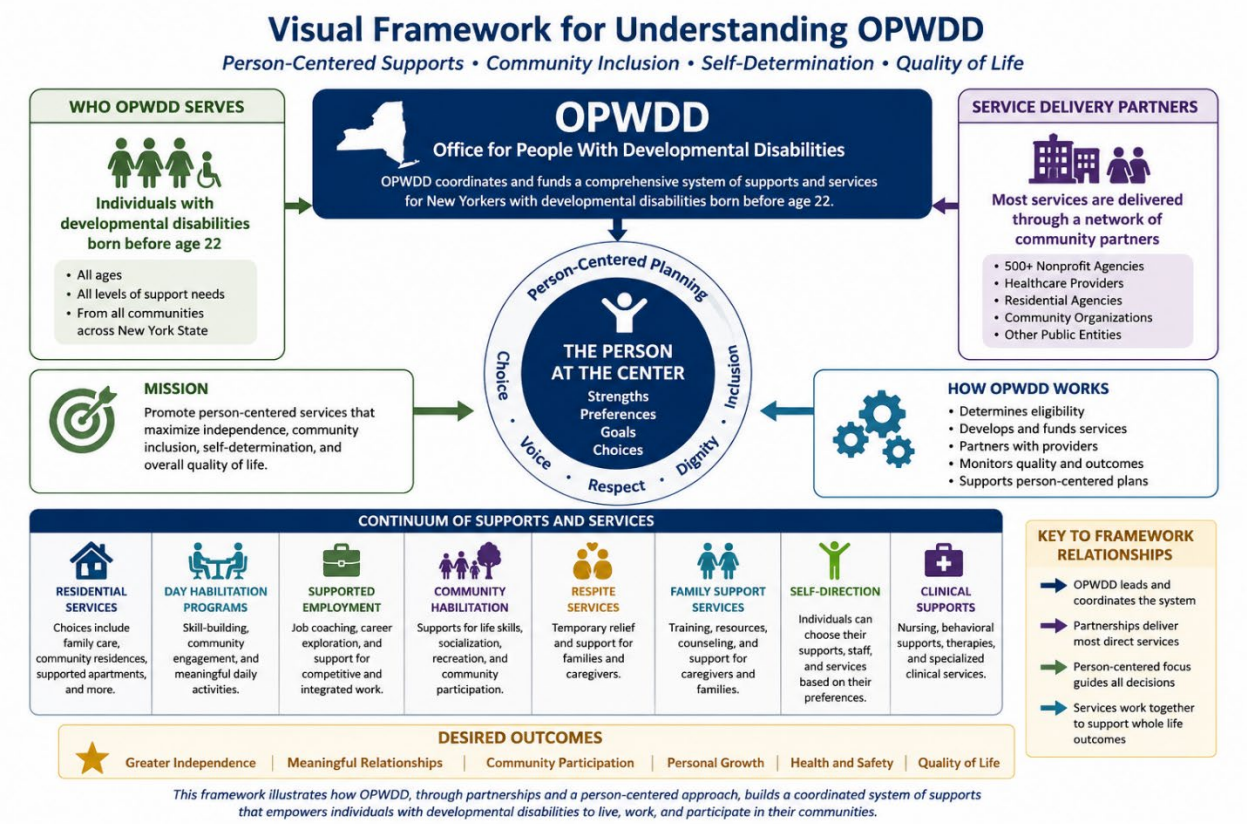


Figure 7.1. *Visual Framework for Understanding the Office of People with Developmental Disabilities*

Pause and Reflect

1. How does this framework demonstrate the principles of person-centered planning? Identify examples within the figure that show how an individual's strengths, preferences, goals, and choices guide service delivery.
2. The framework illustrates that OPWDD works through partnerships with nonprofit agencies, healthcare providers, residential programs, and community organizations. Why is interdisciplinary and interagency collaboration essential for achieving positive outcomes for individuals with I/DD?
3. Imagine you are the social worker supporting an individual who has recently become eligible for OPWDD services. Using this framework, describe how you would help the individual and family navigate available support while ensuring that services remain person-centered and culturally responsive.
4. The desired outcomes highlighted in the figure include greater independence, meaningful relationships, community participation, personal growth, health and safety, and quality of life. Which of these outcomes do you believe is most foundational to person-centered practice, and how can social workers contribute to achieving it through advocacy, care coordination, and systems navigation?

Reintroducing the Person-Centered Ethics of Care Model

Introduced in earlier chapters, the **Person-Centered Ethics of Care (PC-EoC) Model** serves as the conceptual foundation for understanding disability service delivery. Rather than viewing disability services as isolated programs, the model recognizes that successful outcomes emerge from coordinated interactions among multiple systems that collectively influence the lives of individuals with I/DD and their families.

The PC-EoC model emphasizes five interconnected systems: educational, medical, governmental, financial, and social service systems**.** Each system provides unique support while also relying on collaboration with interdisciplinary teams and family members. Social workers occupy a central position within this framework, serving as the linchpin by facilitating communication, connecting systems, and ensuring services remain responsive to the individual's goals and preferences.

Within the educational system, social workers advocate for appropriate educational programming, transition planning, vocational preparation, and lifelong learning opportunities. The medical system focuses on ensuring access to comprehensive healthcare that addresses both primary and specialty needs while promoting continuity of care. Governmental and financial systems provide policies, funding mechanisms, and public benefits that enable disability services, while social service systems connect individuals and families to community resources, behavioral support, counseling, housing, and other essential programs. These interconnected systems show that effective service delivery extends beyond any single agency or provider, and successful outcomes depend upon well-coordinated, ethical, and person-centered collaboration across systems.

Applying the Person-Centered Ethics of Care Model in Social Work Practice

The Person-Centered Ethics of Care Model provides a practical framework for guiding social work interventions across the lifespan, encouraging practitioners to consider the broader ecological systems shaping an individual's daily life rather than focusing solely on immediate service needs. In practice, this means advocating for appropriate educational services, transition planning, and vocational supports; coordinating care among physicians, specialists, therapists, and families to reduce fragmentation and improve health outcomes; navigating complex eligibility requirements, Medicaid systems, OPWDD services, and public benefits to secure the financial resources that sustain long-term stability and independence; and facilitating access to recreational programs, employment, housing, and civic participation so that individuals with I/DD remain active members of their communities rather than isolated recipients of services.

Other Statewide Agencies Supporting Individuals with I/DD

Although OPWDD serves as the primary coordinating agency for developmental disability services in New York State, comprehensive care requires collaboration across multiple governmental systems. The **Department of Health (DOH)** oversees Early Intervention services, Medicaid administration, and public health initiatives; the **Department of Education (DOE)** ensures access to special education, related services, transition planning, and educational

accommodations; the **Office of Mental Health (OMH)** provides behavioral health services for individuals with co-occurring psychiatric conditions; and the **Department of Social Services (DSS)** administers housing assistance, financial supports, public benefits, and case management. Because effective service delivery often requires simultaneous engagement across these agencies, social workers must understand eligibility requirements, referral pathways, application procedures, and interagency communication strategies that promote coordinated rather than fragmented care.

Interdisciplinary Collaboration in I/DD Practice

Because individuals with I/DD often receive services from physicians, nurses, psychologists, speech-language pathologists, occupational therapists, physical therapists, educators, behavioral specialists, care managers, and residential staff, effective disability services depend on interdisciplinary collaboration, with each contributing specialized expertise toward shared person-centered goals. Social workers play a central role in this process, facilitating interdisciplinary communication, ensuring appropriate information sharing, reducing service duplication, and promoting coordinated intervention planning. Importantly, collaboration extends beyond professionals to include family members, caregivers, and individuals with I/DD themselves, recognizing that each brings essential knowledge to the planning process.

Advocacy Within Disability Service Systems

Advocacy is a defining responsibility of social work practice with individuals with I/DD, operating across multiple levels of intervention. At the micro level, social workers empower individuals and families, promote self-determination, and support informed decision-making. At the mezzo level, the focus shifts to improving organizational policies, expanding inclusive programming, and strengthening interdisciplinary collaboration. At the macro level, social workers influence legislation, disability policy, public funding, and broader societal attitudes toward disability. Grounded in anti-oppressive and person-centered practice, advocacy at every level seeks to improve access to services, challenge ableism, advance equity, and ensure the full inclusion of individuals with I/DD in all aspects of community life.

Conclusion

Effective service delivery for individuals with I/DD requires more than knowledge of available programs; it demands an integrated understanding of how multiple service systems interact, how interdisciplinary care is coordinated, and how advocacy operates across organizational and policy contexts. The Person-Centered Ethics of Care Model provides the framework for bringing these elements together, integrating educational, medical, governmental, financial, and social service systems into practice that is coordinated, ethical, and genuinely individualized. By combining systems knowledge with advocacy, collaboration, and cultural humility, social workers help individuals with I/DD achieve greater independence, community inclusion, and quality of life across the lifespan.

Discussion Questions:

1. What are the key components of the Person-Centered Ethics of Care Model, and how can this model be applied to service delivery for individuals with I/DD?
2. How does the Office for People With Developmental Disabilities (OPWDD) coordinate services for individuals with I/DD, and what role do social workers play in this process?
3. Why is it important for social workers to understand the various educational, medical, governmental, financial, and social service components that impact the lives of individuals with I/DD?
4. How can social workers help families navigate state-wide agencies and resources to access the services they need?
5. What strategies can social workers use to advocate for individuals with I/DD within the broader service delivery system?

Case Scenario: Coordinating Services for a Child with I/DD through OPWDD

Case Scenario: Lily

Lily is an 8-year-old girl with Down syndrome and moderate intellectual disability. Her parents, Jessica and Mark, have been managing her care since birth but are finding it increasingly difficult to coordinate her growing service needs , including speech therapy, occupational therapy, special education, and frequent appointments with medical specialists. Seeking support, the family contacts the Office for People With Developmental Disabilities (OPWDD), where they are assigned a social worker named Sarah.

Sarah begins by conducting a strengths-based psychosocial assessment, meeting with Jessica and Mark to understand Lily's needs alongside the family's strengths, challenges, and goals. Drawing on this assessment, Sarah works collaboratively with the family to develop a Life Plan that coordinates Lily's services across systems.

Life Plan: Coordinated Services

Medical Care Coordination: Sarah helps Jessica and Mark schedule and coordinate Lily's specialist appointments, ensuring all providers are informed of one another's recommendations. She also assists the family in applying for Medicaid to offset the costs of ongoing care.

Educational Support: Sarah collaborates with Lily's school team to ensure her Individualized Education Plan (IEP) reflects her current needs. She attends IEP meetings alongside Jessica and Mark, providing advocacy and helping the family understand their rights and available options.

Respite Care: Recognizing the caregiving demands on the family, Sarah connects Jessica and Mark with a local respite care provider, offering them regular breaks while ensuring Lily receives consistent, safe support.

Community Engagement: Sarah introduces the family to a parent support group for families of children with Down syndrome and facilitates Lily's enrollment in recreational programs where she can socialize and participate in activities she enjoys.

Ongoing Support and Monitoring

Following the development of Lily's Life Plan, Sarah continues to meet regularly with the family to review progress, adjust services as Lily's needs evolve, and provide ongoing emotional support during periods of transition. Through coordinated, person-centered

practice, Jessica and Mark feel more confident navigating Lily's care , knowing they have a knowledgeable advocate supporting their family every step of the way.

Discussion Questions

1. How does Sarah's role as a care coordinator help Jessica and Mark manage the complexities of Lily's care, and what are the benefits of having a comprehensive Life Plan?
2. Why is it important for Sarah to collaborate with both medical and educational professionals, and how does this collaboration benefit Lily's overall development?
3. What is the significance of providing respite care to families of children with I/DD, and how does this service support Jessica and Mark's ability to care for Lily?
4. How can connecting families with community resources such as support groups and recreational programs enhance the quality of life for children with I/DD and their families?
5. Why is it necessary for Sarah to provide ongoing support and monitoring, and how does this contribute to Lily's long-term well-being?

Key Terms and Definitions

Term	Definition
Community Engagement	The practice of facilitating access to social services and community resources to promote inclusion and enhance the quality of life for individuals with I/DD.
Intellectual and Developmental Disabilities (I/DD)	A broad category of lifelong conditions characterized by significant limitations in intellectual functioning and adaptive behavior, originating before adulthood, that affect learning, communication, social participation, and independent living.
Non-Profit Service Providers	The network of over 500 non-profit agencies and providers through which approximately 80% of OPWDD services are delivered, enabling a diverse and flexible range of supports for individuals with I/DD.
Office for People With Developmental Disabilities (OPWDD)	The New York State agency responsible for coordinating and delivering services and supports for individuals with developmental disabilities, serving as a primary resource for individuals with I/DD and their families.
Person-Centered Ethics of Care (PC-EoC) Model	A theoretical framework that integrates the key components of an individual's life — educational, medical, governmental, financial, and social — to provide a holistic and ethical approach to service delivery for individuals with I/DD.
└ Department of Education (DOE)	The New York State agency responsible for ensuring that children with I/DD receive appropriate educational services, including special education and related supports.
└ Department of Health (DOH)	A New York State agency that provides a range of health services and supports, including early intervention programs essential for young children with developmental delays.
└ Department of Social Services (DSS)	A New York State agency that offers financial assistance, housing support, and case management services vital for families caring for individuals with I/DD.
└ Office of Mental Health (OMH)	A New York State agency that provides services for individuals with co-occurring mental health conditions, which are common among the I/DD population.

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Chapter 8: Intellectual/Developmental Disabilities: Young Adulthood and the Guardianship Process

Learning Objectives

By the end of this chapter, students will be able to:

- Explain the transition from adolescence to adulthood for individuals with I/DD, including legal changes in decision-making authority at age 18.
- Describe the purpose, process, and requirements of guardianship, including Article 17-A Guardianship in New York State.
- Analyze the ethical considerations involved in guardianship, including tensions between protection and self-determination.
- Evaluate Supported Decision-Making (SDM) as an alternative to guardianship, including its benefits and limitations.
- Identify the roles of courts, caregivers, and social workers in determining capacity and implementing guardianship or alternatives.
- Apply transition planning strategies to support young adults with I/DD in achieving meaningful participation in adult life.

Table 8.1. *Alignment of Chapter Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	CSWE Competency	Alignment Rationale
Explain the transition to adulthood and legal decision-making changes at age 18	***Competency 3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Addresses rights, autonomy, and legal recognition of individuals with I/DD during the transition to adulthood
Describe the guardianship process, including Article 17-A requirements	***Competency 5:*** Engage in Policy Practice	Focuses on legal frameworks, court systems, and policies governing guardianship
Analyze ethical considerations in guardianship decision-making	***Competency 1:*** Demonstrate Ethical and Professional Behavior	Examines ethical tensions between protection, autonomy, and informed consent in practice
Evaluate Supported Decision-Making as an alternative to guardianship	***Competency 2:*** Advance Human Rights and Social Justice	Promotes self-determination, autonomy, and least restrictive alternatives to legal control
Identify roles of courts, caregivers, and professionals in capacity determination	***Competency 6:*** Engage with Individuals, Families, Groups, Organizations, and Communities	Highlights collaborative roles and systems involved in supporting individuals with I/DD
Apply transition planning strategies for young adults with I/DD	***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities	Emphasizes practical application of planning, service coordination, and support strategies

Introduction to Guardianship and Transition to Adulthood

The transition from high school to adulthood represents a critical developmental period for young adults with Intellectual and Developmental Disabilities (I/DD). At age 18, individuals are legally recognized as adults, regardless of disability status. This means they are presumed to have the capacity to make decisions about their healthcare, finances, education, and living arrangements.

While this legal independence promotes autonomy, it may also create challenges for individuals who require support with complex decision-making. During this time, individuals with I/DD and their families must navigate complex decisions related to independence, services, and long-term planning. One of the most significant considerations is whether guardianship is necessary to support decision-making and ensure well-being.

This chapter examines the guardianship process, with a particular focus on New York State, where caregivers may petition the court to be appointed as legal guardians. In addition, this chapter introduces Supported Decision-Making (SDM) as an alternative approach that prioritizes autonomy while still providing necessary support (Martinis et al., 2023). Together, these frameworks highlight the importance of balancing protection with self-determination as individuals with I/DD transition into adulthood (Blanck & Martinis, 2015; Vargas, 2013).

Legal Adulthood and the Need for Guardianship

When individuals with I/DD turn 18, they are legally recognized as adults and are presumed to have full decision-making capacity. This **capacity for legal competence** includes the ability to make independent personal, medical, and financial decisions. While this legal presumption supports autonomy, it may present challenges for individuals with significant cognitive or adaptive impairments.

For some individuals, difficulties with understanding information, evaluating options, or communicating decisions may compromise their ability to make informed choices. In these cases, families or caregivers may seek guardianship to ensure that important decisions are made in the individual's best interest. Guardianship provides a legal mechanism for a designated person to assume responsibility for decision-making while promoting safety and stability. This approach should be carefully evaluated because it involves transferring decision-making authority, which may limit the individual's autonomy.

Understanding Article 17-A Guardianship

In New York State, Article 17-A Guardianship provides a legal pathway for appointing a guardian for individuals with I/DD. This process is designed to be relatively streamlined compared to other forms of guardianship and allows caregivers to obtain authority over key areas of decision-making.

Article 17-A Guardianship typically covers decisions related to healthcare, finances, and personal welfare. Families may initiate the process with an attorney's assistance or independently through court-provided resources, including do-it-yourself (DIY) forms. The Surrogate Court provides guidance and documentation to support families throughout the application process.

Filing for Article 17-A Guardianship (Ages 18+)

Filing for guardianship requires both legal documentation and professional verification of the individual's needs. Eligibility for Article 17-A Guardianship is limited to individuals diagnosed with intellectual or developmental disabilities as defined by New York State law. The **court petition** must include medical certification from two qualified professionals. One certification must be completed by a physician experienced in working with individuals with developmental disabilities, while the second must be provided by a psychiatrist or psychologist. These professionals must attest that the individual is unable to manage their personal affairs independently due to their disability.

In addition to medical documentation, the petition must include information about the petitioner, their relationship to the individual, and a detailed description of the individual's functioning and support needs.

Obtaining Court-Appointed Guardianship

Once the petition is submitted, the court reviews the documentation to determine whether guardianship is appropriate. The petition must clearly demonstrate the individual's need for support and the proposed guardian's ability to fulfill their responsibilities.

Key elements of the petition include **diagnoses and evidence** of the individual's disability, a description of the individual's disability, evidence of functional limitations, and confirmation that the proposed guardian is suitable and capable. The court evaluates whether guardianship is necessary and whether it aligns with the individual's best interests.

The Court Hearing

The guardianship process includes a formal court hearing during which a judge reviews the evidence and considers any **objections**. The **petitioner** must provide sufficient documentation demonstrating that the individual lacks the capacity to make independent, responsible decisions. Evidence presented during the **hearing** may include **IQ test** results, diagnostic documentation, and professional evaluations that establish the relationship between the individual's disability and their decision-making challenges. The court also evaluates the proposed guardian's fitness to ensure they can responsibly carry out their duties. Objections from individuals or other parties prompt further legal proceedings to ensure all views are considered before a final decision

Guardian Powers and Responsibilities

If guardianship is granted, the court assigns specific powers and responsibilities to the **guardian**. These responsibilities are typically limited to areas necessary to support the individual's well-being, such as healthcare decisions, financial management, and personal care. The court defines the scope of authority, which may vary depending on the individual's needs. Guardians are expected to act in the individual's best interest while promoting dignity, safety, and quality of life.

Supported Decision-Making as an Alternative

Supported Decision-Making (SDM) offers an alternative to traditional guardianship by allowing individuals with **Intellectual and Developmental Disabilities (I/DD)** to retain decision-making authority while receiving guidance from trusted supporters (Blanck & Martinis, 2015; Martinis et al., 2023; Vargas, 2013) Rather than transferring decision-making power to another person, SDM emphasizes autonomy and collaborative support. The core principles of SDM are that the individual retains decision-making authority, receives support in understanding available options, and makes the final decisions independently. Rather than transferring legal decision-making power to another person, SDM promotes autonomy through collaborative and individualized support (Blanck & Martinis, 2015). **Supporters (SDM)** may include family members, friends, caregivers, mentors, or community members who assist the individual in understanding options, weighing decisions, and communicating choices. Importantly, the individual makes the final decision.

This approach builds confidence and promotes self-determination by helping individuals with I/DD make informed choices. It reflects core social work values, including person-centered practice and respect for individual rights, by prioritizing empowerment rather than replacing the individual's voice in decision-making.

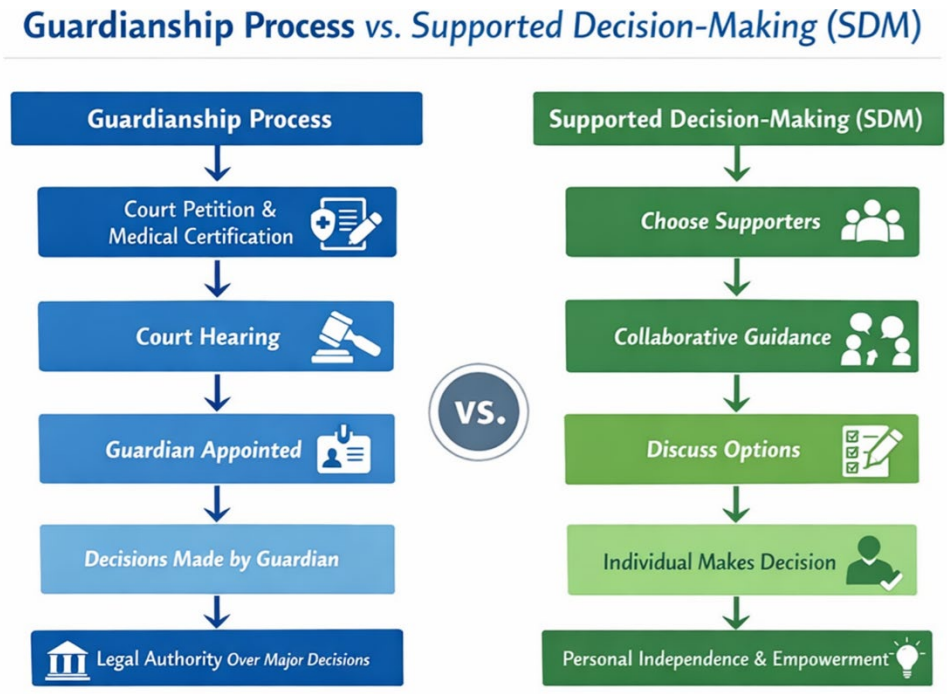


Figure 8.1. *Guardianship Process vs. Supported Decision-Making (SDM)*

Pause & Reflect

- When might guardianship be necessary, and when might Supported Decision-Making (SDM) be more appropriate?
- How do both approaches balance protection and autonomy differently?

- As a social worker, how would you support a family in deciding between guardianship and SDM while still honoring the individual's voice and rights?

Role of Social Workers

Social workers play a central role in supporting individuals with I/DD and their families as they navigate transitions in decision-making and long-term planning. Guided by the principles of person-centered practice, self-determination, and social justice, social workers help ensure that individuals with I/DD remain at the center of decisions affecting their lives. Rather than assuming incapacity, social workers promote strength-based approaches that maximize autonomy while ensuring that appropriate supports are in place to safeguard health, safety, and well-being.

- **Educating individuals and families about legal and decision-making options**: Social workers provide accurate, accessible information about guardianship, supported decision-making (SDM), powers of attorney, healthcare proxies, representative payees, and other legal mechanisms. They help families understand the benefits, limitations, and long-term implications of each option so they can make informed decisions.

- **Assessing and facilitating decision-making capacity:** Social workers recognize that decision-making capacity exists on a continuum and can vary with the complexity of the decision. They assess an individual's strengths, preferences, communication methods, and support needs, while identifying strategies and accommodations that enable meaningful participation in decision-making.

- **Advocating for the least restrictive alternatives:** Consistent with disability rights principles, social workers advocate for approaches that preserve the individual's rights and autonomy whenever possible. They encourage the use of SDM and other less restrictive alternatives before pursuing full guardianship, ensuring that any limitations on legal rights are justified, individualized, and proportionate.

- **Ensuring that the individual's voice and rights remain central:** Social workers use accessible communication strategies, supported communication tools, and person-centered planning techniques to ensure that individuals with I/DD actively participate in decisions affecting their lives. They advocate for respecting the individual's preferences, values, cultural background, and life goals, even when additional decision-making supports are needed.

Transitioning from High School to Adult Life

Preparing for meaningful engagement in work or community activities is especially important to prevent social isolation and promote inclusion. The transition from high school to adulthood is a pivotal stage that requires careful planning and coordination. Early preparation can significantly improve outcomes and reduce stress for both individuals and their families. **Transition planning** should begin well before graduation and may include exploring options such as higher education, employment, **vocational training**, or supported living arrangements. Not all individuals will follow the same pathway, and planning must be individualized based on strengths, needs, and

goals. Structured planning helps ensure that individuals with I/DD have opportunities to participate fully in society.

Conclusion

Social workers play a key role in helping individuals with I/DD and their families navigate the transition into adulthood. One important part of this process is understanding guardianship, which can provide protection and support when individuals have difficulty making certain decisions.

At the same time, social workers must help families balance protection with the individual's right to autonomy and self-determination. Alternatives such as Supported Decision-Making show that individuals can still lead their own lives with the right support and guidance. Social workers help ensure that the individual's voice remains central and that decisions are made with dignity, respect, and care throughout the process, so they can transition to adulthood in meaningful, empowering ways aligned with their personal goals.

Discussion Questions:

1. Why is guardianship an important consideration for families of young adults with I/DD, and what are the legal implications of guardianship?
2. How does Article 17-A Guardianship in New York State address the needs of individuals with I/DD, and what are the steps involved in filing for guardianship?
3. What are the ethical considerations surrounding guardianship, and how does supported decision-making offer an alternative approach?
4. How can social workers assist families in understanding the guardianship process and exploring alternatives that best meet the needs of the individual with I/DD?
5. What challenges might arise during the transition from high school to adult life for individuals with I/DD, and how can social workers support a smooth transition?

Case Scenario: Navigating Guardianship and Supported Decision-Making

David

David is a 17-year-old with a moderate intellectual disability and epilepsy who is nearing his 18th birthday, at which point he will be legally recognized as an adult. His parents, Linda and Tom, have always been closely involved in his care and decision-making, but are concerned about how David will manage important decisions as an adult, particularly regarding his medical care and finances. Unsure whether pursuing full guardianship is the right choice, they meet with a social worker named Karen, who specializes in working with families of young adults with I/DD, to explore their options.

Exploring Guardianship

Karen explains the guardianship process under Article 17-A in New York State, which allows parents to petition the court to become legal guardians of their adult child with I/DD. This would give Linda and Tom the legal authority to make decisions on David's behalf, particularly in areas where he may struggle to understand the implications of his choices, such as medical treatments or financial agreements.

However, Karen also introduces the concept of Supported Decision-Making (SDM), which allows individuals with I/DD to retain their decision-making capacity while receiving support from trusted individuals. Through SDM, David could make his own decisions with his parents' guidance rather than having those decisions made for him, preserving his autonomy while ensuring he has the support he needs.

Decision-Making Process

Karen works collaboratively with Linda, Tom, and David to assess David's decision-making capacity across different areas of his life. They discuss scenarios where he might need support, such as understanding complex medical information or managing his bank account. David expresses a clear desire for greater independence while acknowledging that he will need help with certain decisions.

Together, the family develops a Supported Decision-Making agreement in which David identifies his parents as his supporters. The agreement outlines how Linda and Tom will assist David in making decisions, ensuring that his preferences and choices are respected throughout the process. Karen helps them formalize the agreement, which can be presented in court if needed.

Legal and Emotional Support

Karen helps Linda and Tom understand the legal protections available through Supported Decision-Making, emphasizing that it can serve as a flexible, rights-affirming alternative to full guardianship. She connects the family with a disability law specialist to ensure they have the legal guidance necessary to make an informed decision.

In addition to the legal dimensions of this transition, Karen provides ongoing emotional support to the family, helping them navigate the complexities of moving into adulthood with I/DD. She encourages Linda and Tom to actively involve David in all discussions about his future, reinforcing his right to self-determination and his role as the central voice in decisions that affect his own life.

Discussion Questions

1. **Guardianship vs. Supported Decision-Making:** What are the key differences between guardianship and Supported Decision-Making, and how do these options impact David's autonomy and right to self-determination?
2. **Assessing Decision-Making Capacity:** How can families and social workers assess an individual's capacity to make decisions, and what factors should be considered in determining the appropriate level of support?
3. **Legal Protections:** What legal protections are available through Supported Decision-Making, and how can they ensure that David's rights are respected while providing necessary support?
4. **Involving the Individual:** Why is it important to involve David in discussions about his future and decision-making, and how can this involvement empower him as he transitions to adulthood?
5. **Long-Term Considerations:** What are the long-term considerations for David's parents as they plan for his adulthood, and how can Karen continue to support them through this process?

Key Terms and Definitions

Term	Definition
Article 17-A Guardianship	A legal process in New York State that allows a court to appoint a guardian for an individual with intellectual or developmental disabilities to make personal, medical, and financial decisions on their behalf.
Autonomy	The ability and right of an individual to make their own decisions and direct their own life.
Capacity (Legal Competence)	The legal ability of an individual to understand information, make decisions, and appreciate the consequences of those decisions.
Court Petition	A formal legal request submitted to the court seeking guardianship or another legal determination.
Decision-Making Capacity	The ability to understand, evaluate, and communicate decisions in a meaningful way.
Diagnosis Evidence	Documentation confirming that an individual's intellectual or developmental disability contributes to limitations in decision-making capacity.
Guardianship	A legal arrangement in which one person is granted authority by a court to make decisions for another person who lacks decision-making capacity.
└ **Guardian**	A court-appointed individual authorized to make decisions on behalf of a person who is unable to manage their own affairs independently.
└ **Guardian Fitness**	The court's assessment of whether a proposed guardian is capable, responsible, and appropriate to serve in the role.
└ **Hearing (Guardianship Hearing)**	A court proceeding in which evidence is presented to determine whether guardianship is necessary and appropriate.
└ **Petitioner**	The individual (often a parent or caregiver) who files a request with the court seeking guardianship.
└ **Scope of Authority**	The specific powers granted to a guardian by the court, which may include decisions related to healthcare, finances, and personal welfare.
└ **Surrogate Court**	The court in New York State that handles guardianship petitions under Article 17-A.

Intellectual and Developmental Disabilities (I/DD)	Disabilities characterized by significant limitations in intellectual functioning and adaptive behavior, originating before age 18.
IQ Testing	Standardized assessment used to evaluate cognitive functioning and inform determinations of capacity.
Legal Independence at Age 18	The legal status in which individuals are presumed to have full decision-making rights upon reaching adulthood.
Medical Certification	Required documentation from qualified professionals confirming that an individual is unable to manage personal affairs due to disability.
Objection (Legal)	A formal disagreement raised during a guardianship proceeding, which may require additional review by the court.
Supported Decision-Making (SDM)	An alternative to guardianship that allows individuals with I/DD to make their own decisions with guidance and support from trusted individuals.
└ **Supporters (SDM)**	Individuals who assist a person with I/DD in understanding options, weighing decisions, and communicating choices while preserving autonomy.
Transition Planning	The process of preparing a young adult with I/DD for adulthood, including education, employment, housing, and independent living supports.
Vocational Training	Programs designed to develop job-related skills and prepare individuals for employment.

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Chapter 9: I/DD: Young Adults, Employment, and Decision-Making

Learning Objectives

By the end of this chapter, students will be able to:

- Explain the role of person-centered planning in promoting self-determination for individuals with I/DD transitioning to adulthood
- Describe the responsibilities of social workers in fostering decision-making, independence, and self-advocacy
- Identify strategies that support a successful transition from school to employment
- Examine the importance of work and volunteer experiences in shaping employment outcomes
- Recognize key components of work readiness, including social skills, communication, and job-related competencies
- Understand the role of multidisciplinary collaboration in supporting employment and transition planning
- Analyze how social workers collaborate with employment agencies and employers to create opportunities
- Apply person-centered approaches to real-world transition planning scenarios
- Assess the importance of ongoing support in promoting long-term independence and community integration

Table 9.1 *Alignment of Chapter 3 Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	Competency	Alignment Rationale
Explain person-centered planning in promoting self-determination	Competency 2: Engage Diversity and Difference in Practice	Centers individual identity, strengths, and lived experience
Describe social worker roles in decision-making and self-advocacy	***Competency 6***: Engage with Individuals, Families, Groups, Organizations, and Communities	Emphasizes relational engagement and empowerment
Analyze strategies for transition to employment	***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities	Focuses on applied intervention and transition planning
Examine work/volunteer experiences and outcomes	***Competency 4:*** Engage in Practice-Informed Research and Research-Informed Practice	Draws on evidence linking early exposure to employment success
Identify work readiness skills	***Competency 7:*** Assess Individuals, Families, Groups, Organizations, and Communities	Focuses on assessment of strengths, skills, and readiness
Evaluate multidisciplinary collaboration	***Competency 9:*** Evaluate Practice with Individuals, Families, Groups, Organizations, and Communities	Highlights coordination and evaluation of systems-level practice

Apply person-centered approaches to scenarios	***Competency 1:*** Demonstrate Ethical and Professional Behavior	Reinforces ethical, client-centered decision-making
Assess systemic barriers and access **to employment**	***Competency 3:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Promotes equity, inclusion, and access to opportunity

Introduction to Young Adults with I/DD

As young adults with Intellectual and Developmental Disabilities (I/DD) transition into adulthood, they encounter a variety of challenges related to community integration, employment, and decision-making. This time represents an important developmental stage in which individuals begin to define their goals, exercise autonomy, and engage more fully in society. Key to this process is **person-centered planning**, which promotes self-determination by supporting individuals in making choices that align with their strengths, values, goals, and preferences.

Self-determination is not a single milestone; however, it is a **lifelong developmental process**. Social workers play a key role in supporting this process by helping individuals build decision-making skills, access resources, and navigate systems that influence their transition into adulthood.

Social Worker Roles and Responsibilities

Social workers play a critical role in promoting **self-determination** and supporting individuals with I/DD as they transition into adulthood, emphasizing empowerment, strengthening family involvement, and fostering meaningful participation in decision-making.

One key responsibility is the **early incorporation of decision-making skills**. Integrating decision-making goals into the Individualized Education Plan (IEP) (see Chapter 4) allows individuals to begin practicing decision-making and self-advocacy early in life. This sets the foundation for independence. Families also play an essential role in this process. Social workers support families in creating opportunities at home and in communities, such as through Community Habilitation services, that reinforce decision-making skills in everyday contexts.

A central approach guiding this work is **Person-Centered Planning (PCP)**, which focuses on the individual's strengths, goals, and aspirations, promoting self-determination. Social workers collaborate with **multidisciplinary teams** to develop plans that reflect the individual's vision for their life, ensuring that individuals have opportunities to make choices, build skills, and exercise autonomy.

Social workers also support **individualized-directed learning.** This enables individuals with I/DD to participate in shaping their educational and life goals based on their cognitive abilities. Additionally, they play a key role in developing **self-advocacy skills** that help them access information, express their needs, and actively participate in decision-making. Providing time, space, and support for individuals to make their own decisions is essential in promoting confidence and independence.

Transitioning to Work

Transitioning from school to employment (see Figure 11) is a significant milestone for young adults with I/DD that requires planning and coordination. Staying informed about available resources, training programs, and services during planning is essential to effectively support both individuals and their families. Person-centered planning is most effective when it is tailored to the individual and actively facilitated by social workers. The transition begins with understanding the individual, for which comprehensive information on strengths, interests, preferences, and support needs is needed to guide planning.

Social workers play a key role in coordinating Life Plan meetings**;** here is where goals are discussed, resources are identified, and steps toward employment are outlined. Collaborating with the IEP team (see Chapter 4), including special education teachers, therapists, and transition specialists, is important in refining and implementing transition plans. Equally important is ensuring that plans are effectively implemented, with social workers regularly following up to monitor progress and make adjustments as needed.

Exploring Work and Volunteer Opportunities

Work and volunteer experiences are essential components of successful transition planning. These experiences provide individuals with opportunities to explore their interests, build skills, and develop a sense of identity within the workforce and the community. Early exposure to work environments is highly associated with positive employment outcomes in adulthood, whether through volunteer roles or paid positions. These experiences help individuals envision themselves as workers, identify their strengths, and gain confidence in their abilities.

Schools often support this process through **work-readiness programs,** which may include job shadowing, field trips to work sites, and structured volunteer opportunities. These experiences serve as a bridge between school and employment, offering practical exposure that prepares individuals for future career pathways and a smoother transition.

Preparing for Work and Volunteer Opportunities

Preparing for employment involves identifying opportunities that require intentional skill-building and readiness development. One of the most important areas of preparation is **social skill development**, as interpersonal communication and interaction are essential for employment success. Social workers play a critical role in this area, equipping individuals with the skills needed to succeed in the workplace. Structured learning opportunities that focus on practical, job-related skills help individuals build competence and confidence.

Early preparation is key. Beginning the transition process by the student's junior year allows sufficient time for skill development and career exploration. For students with intellectual disabilities in the NYC DOE system, “junior year” may look different than the traditional 11th grade. Because these students with I/DD are entitled to a free and appropriate public education through age 21 (with services extending to age 22), junior year is best understood as the midpoint of their school experience, typically around ages 17 to 18, when transition planning moves from

foundational goal-setting to active skill-building and real-world exploration. This extended timeline is an opportunity, not a delay, giving students and families more time to identify vocational interests, connect with adult service providers such as **OPWDD,** and practice the independent living skills that will support long-term success.

Social workers also support the development of foundational employment skills, such as time management, communication, and task completion, using strategies tailored to each individual's needs, including visual supports and personalized communication tools. This work extends beyond the school setting through collaboration with employment agencies that specialize in supporting individuals with I/DD, ensuring that job opportunities are appropriate, well-supported, and aligned with each student's unique goals.

Collaborating with Employment Agencies and Employers

Meaningful employment outcomes for individuals with I/DD depend on strong partnerships between social workers, employment agencies, and employers. Social workers play a central role in building these relationships, identifying employers who are open to offering job shadowing and work experiences, and actively involving them in the planning process to ensure opportunities are a genuine fit for both the individual and the workplace. Beyond making connections, social workers also focus on removing employment barriers, advocating for reasonable accommodation, facilitating clear communication among all stakeholders, and providing ongoing support that helps individuals thrive in a work environment.

Conclusion

The transition to adulthood for individuals with I/DD is a complex and critical period, one that requires planning, coordinated support, and a genuine commitment to each person's goals, needs, and preferences. Social workers play a central role in this journey, promoting independence and self-determination, bridging gaps among families, schools, and employment systems, and supporting individuals with I/DD in building not just careers but meaningful, self-directed lives and full participation in their communities.

Discussion Questions:

1. How can person-centered planning promote self-determination and independence for individuals with I/DD as they transition to adulthood?
2. What role do social workers play in fostering decision-making skills and self-advocacy among individuals with I/DD?
3. How can work and volunteer experiences during high school contribute to successful employment outcomes for individuals with I/DD?
4. What are the key challenges in preparing individuals with I/DD for employment, and how can social workers support this process?
5. How can collaboration with employment agencies and employers enhance work opportunities for individuals with I/DD, and what strategies can social workers use to facilitate this collaboration?

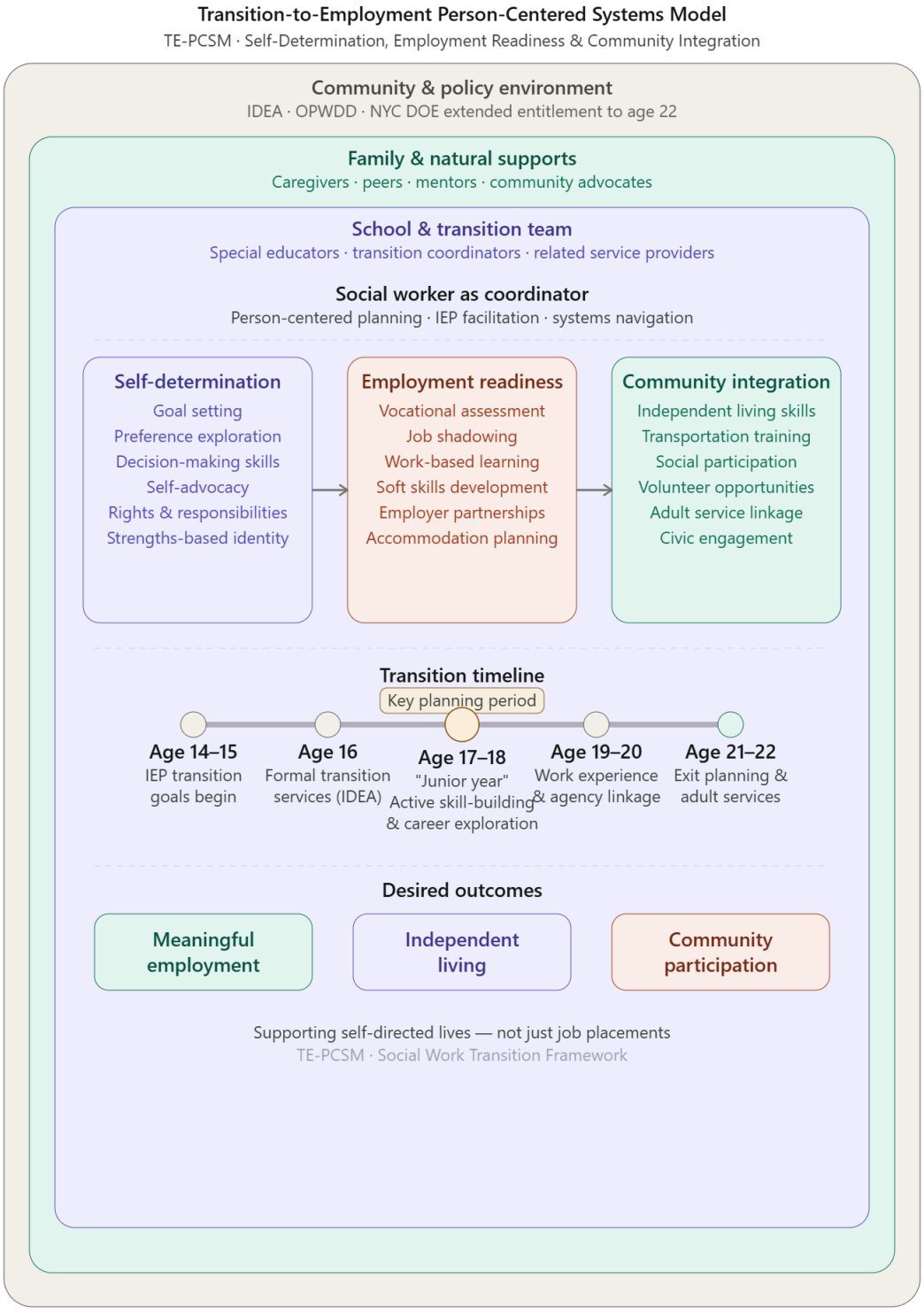


Figure 9.1. *The Transition-to-Employment Person-Centered Systems Model (TE-PCSM): A Framework for Supporting Self-Determination, Employment Readiness, and Community Integration Among Individuals with Intellectual and Developmental Disabilities.*

Pause and Reflect

1. How does the Transition-to-Employment Person-Centered Systems Model (TE-PCSM) demonstrate that successful transition planning involves more than obtaining employment? Identify the relationships among self-determination, employment readiness, and community integration within the model.
2. The TE-PCSM identifies the social worker as the coordinator of person-centered planning and systems navigation. What unique roles and responsibilities does the social worker assume during the transition process, and why are these responsibilities essential for achieving successful adult outcomes?
3. Transition planning is presented as a developmental process that begins in early adolescence rather than at graduation. Why is early planning important, and how might delayed transition planning affect employment, independent living, and community participation for individuals with I/DD?
4. Imagine you are the transition social worker for a 17-year-old student with I/DD who hopes to obtain competitive employment after high school. Using the TE-PCSM, describe how you would collaborate with the student, family, school personnel, adult service agencies, and community partners to develop a person-centered transition plan that promotes meaningful employment, independent living, and full community participation.

Case Scenario: Alex's Transition to Employment

Case Scenario: Alex

Alex is a 20-year-old with a mild intellectual disability who recently graduated from high school and is navigating the transition to employment, independence, and community participation. During high school, Alex participated in a work-readiness program, gaining experience at a grocery store and an animal shelter. These experiences helped him identify his strengths; he is friendly, detail-oriented, and thrives in structured environments, while also revealing areas for continued growth, including time management, completing multi-step tasks, and adjusting to new settings.

Using a person-centered approach, Alex's social worker, Rachel, begins by centering his voice and goals. Alex expresses a clear interest in working in a retail setting, particularly a clothing store where he can organize merchandise and assist customers. His preferences shape every step of the planning process.

Transition Plan: Coordinated Support

- **Employment Planning:** Rachel connects Alex to a supported employment program, assists with resume development and interview preparation, and enrolls him in skill-building workshops focused on communication, customer service, and time management.
- **Collaborative Support:** Alex's transition is sustained through close collaboration among Rachel, his family, an employment agency, and a job coach. Together, they ensure that planning is coordinated, consistent, and responsive to Alex's evolving needs.
- **Job Placement and Coaching:** With this support network, Alex secures a part-time position at a clothing store. His job coach provides on-site assistance, helping him establish routines and complete tasks with increasing independence, while Rachel continues to monitor progress and coordinate services across systems.
- **Promoting Independence:** As Alex gains confidence, supports are gradually reduced. Ongoing planning focuses on increasing work hours, strengthening skills, and exploring long-term career opportunities aligned with his interests.

Framework Connection: TE-PCSM

This case reflects the Transition-to-Employment Person-Centered Systems Model (TE-PCSM): Alex's self-determination guides the process, skill development shapes his readiness, structured transition supports translate goals into action, and coordinated systems

ensure continuity of care , all grounded in the Person-Centered Ethics of Care, which emphasizes dignity, autonomy, and relational support throughout the transition to adulthood.

Discussion Questions

1. How does centering Alex's goals and preferences influence his employment outcomes and sense of self-determination?
2. How do the strengths and areas for growth identified during high school shape Rachel's intervention planning?
3. What role does collaboration among Rachel, the family, the employment agency, and the job coach play in supporting a successful transition?
4. How is person-centered, ethical practice demonstrated throughout this case, and how does it reflect the principles of the PC-EoC Model?
5. Why is ongoing support and gradual reduction of services important for promoting long-term independence for individuals with I/DD?

Key Terms and Definitions

Term	Definition
Community Habilitation	A support service that assists individuals with I/DD in developing and maintaining skills needed for community participation and independent living, often provided in home and community-based settings.
Community Integration	The process of supporting individuals with I/DD in participating meaningfully in community life, including civic, social, and employment activities, alongside people without disabilities, as a goal of transition planning.
Employment Readiness	The development of foundational skills and competencies, including time management, communication, task completion, and interpersonal skills, that prepare individuals with I/DD to succeed in a work environment.
Extended Entitlement	The right of students with I/DD in New York State to receive a free and appropriate public education through age 21, with services extending to age 22, providing additional time for transition planning and skill development beyond the traditional school timeline.
Individualized Education Plan (IEP)	A legally mandated, written document developed for eligible students with disabilities that outlines individualized goals, services, and supports, including transition planning goals, to guide their education.
Intellectual and Developmental Disabilities (I/DD)	A broad category of conditions characterized by significant limitations in intellectual functioning and adaptive behavior, originating before adulthood, that affect an individual's everyday social and practical skills.
Life Plan Meeting	A structured meeting coordinated by a social worker in which the individual with I/DD, their family, and support team discuss goals, identify resources, and outline steps toward employment and community integration.
Office for People With Developmental Disabilities (OPWDD)	The New York State agency that coordinates and funds services and supports for individuals with I/DD, including residential, employment, and community-based programs.
Person-Centered Planning (PCP)	An approach to supporting individuals with I/DD that focuses on their unique strengths, goals, and aspirations. Social workers collaborate with multidisciplinary teams to develop plans that reflect the individual's vision for their life.

Reasonable Accommodations	Modifications or adjustments to a job, work environment, or the way tasks are performed that enable individuals with disabilities to participate fully and equitably in the workplace without fundamentally altering the nature of the work.
Self-Advocacy	The ability of an individual with I/DD to understand and communicate their needs, access information, express preferences, and actively participate in decisions that affect their life.
Self-Determination	A lifelong developmental process through which individuals with I/DD build the skills, confidence, and autonomy to make choices aligned with their own strengths, values, goals, and preferences.
Transition Planning	A coordinated, person-centered process that prepares students with I/DD for the move from school to adult life, encompassing employment, independent living, and community participation. In the NYC DOE, this process extends through age 22.
Work-Based Learning	Structured experiences, such as job shadowing, internships, and volunteer roles, that provide individuals with I/DD practical exposure to work environments, helping them build skills and explore career interests before formal employment.

Chapter 10: Older Adults with Intellectual Disabilities

Learning Objectives

By the conclusion of this chapter, students will be able to:

- Describe the aging process among older adults with intellectual and developmental disabilities (I/DD), including biological, psychological, and social changes.
- Examine the health disparities, chronic conditions, and age-related syndromes commonly experienced by older adults with I/DD.
- Analyze the impact of aging on families, caregivers, residential systems, and long-term supports.
- Apply person-centered, strengths-based, and trauma-informed approaches to assessment, intervention, and care planning for older adults with I/DD.
- Evaluate the role of social workers in promoting healthy aging, advocacy, interdisciplinary collaboration, and end-of-life planning.

Table 10.1 *Alignment of Chapter Learning Objectives with CSWE EPAS (2022)*

Learning Objective	CSWE Competency	Alignment Rationale
Explain the biological, psychological, social, and environmental aspects of aging among older adults with intellectual and developmental disabilities (I/DD), including health disparities, chronic conditions, and age-related changes.	**Competency 4:** Engage in Practice-Informed Research and Research-Informed Practice; **Competency 7:** Assess Individuals, Families, Groups, Organizations, and Communities	Requires students to integrate current research on aging, disability, and health disparities while conducting comprehensive biopsychosocial assessments that recognize the multidimensional needs of older adults with I/DD.
Examine the impact of aging on individuals with I/DD, families, caregivers, residential systems, and long-term supports, emphasizing interdisciplinary collaboration and coordinated systems of care.	**Competency 6:** Engage with Individuals, Families, Groups, Organizations, and Communities; **Competency 7:** Assess Individuals, Families, Groups, Organizations, and Communities	Emphasizes engagement with individuals, families, and interdisciplinary teams while assessing how family systems, residential environments, healthcare, and community supports influence healthy aging and quality of life.
Apply person-centered, strengths-based, and trauma-informed approaches to healthy aging, future planning, residential	**Competency 1:** Demonstrate Ethical and Professional Behavior; **Competency 6:** Engage with Individuals, Families, Groups,	Reinforces ethical practice by promoting self-determination, dignity, informed decision-making, and culturally responsive interventions.

planning, supported decision-making, and service coordination for older adults with I/DD.	Organizations, and Communities; C***ompetency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities	Students learn to apply person-centered strategies that strengthen independence, empower families, and coordinate services across systems.
Analyze the role of social workers in promoting healthy aging through advocacy, interdisciplinary collaboration, healthcare navigation, and policy initiatives that address ageism, ableism, and inequities experienced by older adults with I/DD.	***Competency 2:*** Advance Human Rights and Social, Racial, Economic, and Environmental Justice; ***Competency 5*****:** Engage in Policy Practice	Highlights the social worker's responsibility to challenge systemic barriers, advocate for equitable access to healthcare and aging services, promote disability rights, and influence policies that improve long-term outcomes for older adults with I/DD.
Evaluate evidence-informed interventions and apply critical thinking to case scenarios involving aging, caregiver support, residential transitions, end-of-life planning, and quality-of-life outcomes for older adults with I/DD.	***Competency 4*****:** Engage in Practice-Informed Research and Research-Informed Practice; ***Competency 8:*** Intervene with Individuals, Families, Groups, Organizations, and Communities; ***Competency 9:*** Evaluate Practice with Individuals, Families, Groups, Organizations, and Communities	Encourages students to synthesize research, theory, and practice through case-based learning while evaluating the effectiveness of interventions designed to support healthy aging, continuity of care, and person-centered outcomes across the lifespan.

Introduction to Older Adults with I/DD

Due to advancements in healthcare and community-based support, adults with I/DD are experiencing significantly increased life expectancies, with the population of older adults with I/DD projected to reach 1.2 million by 2030 (Heller, 2019). While the general population is typically considered "older" at age 65, aging-related changes in the I/DD population are often observed as early as age 40 to 50. Older adults with I/DD face a distinct set of challenges, including accelerated aging, clinical complexities such as diagnostic overshadowing, and cross-system fragmentation (Foster et al., 2025; Gordon, 2017; Lahti Anderson & Pettingell, 2023).

Functional, cognitive, and sensory changes frequently manifest 15 to 20 years earlier in the I/DD population compared to neurotypical cohorts, often presenting not as explicit complaints but as behavioral changes, increased irritability, or sudden loss of adaptive skills. Individuals with Down syndrome face a particularly **accelerated aging** trajectory, due to overexpression of the APP gene on chromosome 21; the majority develop neuropathological changes characteristic of Alzheimer's disease by age 40, with clinical dementia symptoms frequently emerging in their 50s (Gordon, 2017). Additionally, complex long-term medication regimens, including psychotropic

medications, become increasingly risky as liver and kidney function decline, raising the likelihood of adverse drug interactions, over-sedation, and falls.

Psychosocial transitions further compound these challenges. Many adults with I/DD have lived at home with aging parents throughout their lives, and a critical, often crisis-driven milestone occurs when parental caregivers become frail or pass away. Older adults with I/DD frequently experience complicated grief when parents, siblings, or residential peers die, as paternalistic assumptions about cognitive capacity often result in their exclusion from funerals and illness-related conversations, leading to unresolved grief that may manifest as behavioral regression (Samuel et al., 2022). Retirement from structured day programs presents an additional hurdle, as finding meaningful, low-impact activities that prevent social isolation remains a significant late-life challenge.

At the systemic level, the intersection of the aging network, including Senior Centers, Medicare, and Area Agencies on Aging, and the disability network, including Medicaid waivers and state developmental disability offices, creates significant service gaps. The disability sector is historically oriented towards habilitation and community integration, while the aging sector focuses on managing progressive decline; when an individual with I/DD enters late life, these systems rarely integrate smoothly, resulting in fragmented care (Heller et al., 2015). Residential settings designed for younger, mobile adults often lack the physical accessibility required to support individuals with advanced mobility loss or dementia, while skilled nursing facilities frequently lack staff trained in the behavioral and communication needs of individuals with developmental disabilities.

Understanding Aging Among Older Adults with I/DD

Advances in medicine, public health, nutrition, and disability services have dramatically improved life expectancy for individuals with I/DD (Krahn & Trollor, 2026), with many now living well into their sixties, seventies, and beyond. This demographic shift has transformed aging into one of the most consequential emerging priorities within disability services, demanding new frameworks, specialized knowledge, and coordinated systems of care. Older adults with I/DD experience many of the same biological, psychological, and social changes associated with aging in the general population, including changes in physical health, cognitive functioning, sensory abilities, mobility, and social networks, but confront these alongside lifelong disability, accumulated health disparities, and decades of systemic exclusion (Araten-Bergman & Bigby, 2024). These compounding factors accelerate the aging process, complicate detection and treatment of age-related conditions, and increase vulnerability to isolation, depression, and functional decline.

The risk of early-onset Alzheimer's disease is significantly elevated among individuals with Down syndrome, with dementia-related symptoms emerging as early as the forties or fifties in many cases (Rubenstein et al., 2024). Other age-related conditions, including arthritis, cardiovascular disease, sensory impairments, and reduced mobility, also tend to emerge at younger ages than in the general population, requiring proactive, preventive, and interdisciplinary approaches to care. Many older adults with I/DD have also outlived their primary caregivers, leaving them vulnerable to disruptions in housing, care continuity, and social connection,

particularly when transitions to residential or community-based settings have not been planned in advance (Samuel et al., 2022).
Successful aging for individuals with I/DD requires coordinated, person-centered support that is attentive to each individual's full life history, health profile, and evolving preferences. Social workers are uniquely positioned to lead this work, advocating across systems, facilitating advance planning, coordinating care, and ensuring that aging with I/DD is approached with the dignity, intentionality, and respect every person deserves.

Biological Changes and Health Conditions

Aging is accompanied by gradual physiological changes affecting nearly every body system, including cardiovascular function, musculoskeletal integrity, neurological health, sensory acuity, immune response, and metabolic regulation (Taffet, 2024). For older adults with I/DD, these changes often emerge earlier and with greater complexity than in the general population, a phenomenon increasingly recognized in literature as premature or accelerated aging (Lin & Tseng, 2022). The cumulative effect of lifelong disability alongside the biological processes of aging places many individuals at increased risk for chronic illness, functional decline, and reduced quality of life.

Common health concerns include cardiovascular disease, diabetes, obesity, osteoporosis, arthritis, epilepsy, sleep disorders, hearing loss, vision impairment, dental disease, and chronic pain. Many of these conditions are preventable or manageable with early intervention, yet they frequently go undetected or undertreated due to diagnostic overshadowing, communication barriers, limited access to specialized care, and the historical marginalization of individuals with I/DD within mainstream healthcare systems. Social workers play a critical role in ensuring that health concerns are identified early, communicated effectively across providers, and addressed within a coordinated plan of care.

Polypharmacy, the concurrent use of multiple medications, becomes increasingly common as individuals age and accumulate multiple diagnoses (Nicholson et al., 2024). For individuals with I/DD who may already be prescribed medications for seizure management or behavioral health conditions, the addition of age-related medications significantly increases the risk of adverse drug interactions and medication-related complications. Careful monitoring by interdisciplinary healthcare teams, in collaboration with social workers and family caregivers, is essential to managing polypharmacy safely (Houseworth et al., 2023).

Individuals with Down syndrome face a substantially elevated risk of early-onset Alzheimer's disease, with the vast majority developing its neuropathological hallmarks by their forties and clinical symptoms frequently emerging in their fifties or sixties , decades earlier than typical onset in the broader population (Mannan et al., 2025; Maure-Blesa et al., 2025). Because early symptoms in this population often present as subtle behavioral changes, withdrawal, functional decline, or shifts in mood rather than the memory complaints more commonly reported in neurotypical adults, accurate diagnosis can be particularly challenging. Social workers, in collaboration with neurologists, psychologists, and other specialists, must be prepared to facilitate comprehensive assessments that distinguish between normal aging, dementia, depression, grief, and situational stressors. Early and accurate identification is essential to ensuring individuals

receive appropriate supports, families are adequately prepared, and advance planning occurs in a timely and person-centered manner.

Early Detection and Interdisciplinary Collaboration

Social workers are often among the first to notice subtle changes in an individual's functional or cognitive abilities. Specialized tools such as the National Task Group Early Detection Screen for Dementia (NTG-EDSD) provide a structured framework for establishing functional baselines and documenting early signs of cognitive decline or atypical aging presentation (Gordon, 2017). When used consistently and in coordination with interdisciplinary teams, these screening tools enable earlier intervention, more informed care planning, and stronger outcomes for individuals with I/DD navigating the challenges of late life.

Mental Health and Cognitive Aging

Older adults with I/DD experience depression, anxiety, grief, loneliness, and social isolation at rates that are frequently overlooked and undertreated (Stokes et al., 2025). The loss of parents, caregivers, siblings, and peers, many of whom served as primary sources of support throughout the individual's life, can trigger profound grief responses that go unaddressed within systems focused primarily on functional and medical needs. Social isolation, reduced community participation, and the loss of meaningful roles and routines further compound these emotional challenges. Social workers are well-positioned to identify these concerns early, provide therapeutic support, and advocate for mental health services that are accessible and responsive to each individual's communication style and life history.

A persistent barrier to appropriate care is diagnostic overshadowing, the tendency to mistake behavioral and emotional changes in an individual for their disability rather than evaluating them as potential indicators of depression, anxiety, physical illness, pain, trauma, or early dementia (Dell'Armo & Tassé, 2024). When changes are dismissed rather than investigated, diagnosis is delayed, treatment is withheld, and suffering is prolonged. Social workers play a critical role in challenging **diagnostic overshadowing** by advocating within interdisciplinary teams, educating caregivers and providers, and ensuring that concerns receive thorough clinical evaluation.

A comprehensive mental health assessment must take a broad view, examining biological, psychological, environmental, and social factors when evaluating changes in mood, cognition, behavior, or daily functioning. Standardized tools adapted for use with individuals with I/DD, combined with input from caregivers, family members, and direct support professionals, are essential to thorough evaluation. Social workers coordinate this process, gathering information across systems, facilitating communication among providers, and ensuring findings translate into actionable, person-centered plans. Trauma-informed and person-centered approaches must guide this work at every stage, recognizing the cumulative weight of lifelong discrimination and exclusion that many older adults with I/DD carry, while affirming their dignity and right to compassionate, individualized care.

Health Disparities and Healthcare Access

Despite increased longevity, older adults with I/DD continue to experience significant healthcare disparities that compromise their health, functioning, and quality of life (Okoye et al., 2026). Access to appropriate medical care remains a persistent challenge shaped by provider shortages, particularly among physicians, specialists, and mental health professionals with training in both disability and geriatric care, alongside fragmented healthcare systems, inadequate insurance coverage, inaccessible facilities, and limited transportation options. These barriers restrict the ability of older adults with I/DD to receive timely, consistent, and appropriate care.

Preventive healthcare services also remain persistently underutilized. Routine physical examinations, cancer screenings, dental care, hearing and vision assessments, vaccinations, and chronic disease management are associated with significantly better health outcomes; however, many older adults with I/DD do not receive these services at recommended intervals (Ricciardi et al., 2025). This gap reflects both systemic barriers to access and the historical tendency of healthcare providers to focus exclusively on disability-related concerns while overlooking age-related health needs, with the consequence that undetected conditions progress more rapidly and become increasingly difficult to manage.

Social determinants of health further compound these disparities. Poverty, housing instability, food insecurity, social isolation, limited education, and unequal access to healthcare resources disproportionately affect individuals with I/DD throughout their lives, with their increasing impact becoming more pronounced in older adulthood (Aidoo, 2023; Scott, 2025). Decades of navigating underfunded service systems, experiencing discrimination within healthcare settings, and managing the financial consequences of limited employment and reliance on public benefits are not incidental to poor health outcomes; they are among their primary drivers, and addressing them requires systemic change alongside individualized support.

Social workers are uniquely positioned to bridge the gap between individuals with I/DD and the healthcare systems designed to serve them. At the micro level, they conduct health screenings, coordinate care across providers, assist individuals in communicating their needs, and help families navigate insurance and benefits systems. At the mezzo level, they advocate within healthcare organizations for accessible facilities, disability-competent staff training, and inclusive policies. At the macro level, they engage in policy advocacy to strengthen funding for disability and aging services, expand Medicaid coverage, address transportation inequities, and advance systemic reforms that promote equitable access across the lifespan.

Cross-System Navigation and Service Coordination

Because the aging and disability service sectors frequently operate in silos, social workers play a critical bridging role between these systems. They coordinate between developmental disability agencies, often funded through **Medicaid Home and Community-Based Services (HCBS) waivers**, and the aging network, including Area Agencies on Aging, senior centers, and Medicare, ensuring that individuals do not lose disability-specific funding or behavioral supports when they begin accessing traditional eldercare services. When physical or cognitive decline requires a change in living arrangements, social workers manage the transition, advocating for environmental

modifications such as universal design features and ramps when individuals remain in group home settings, and conducting **pre-admission screening and referral (PASRR)** and training nursing home staff on communication styles and behavioral baselines when transitions to skilled nursing facilities are necessary.

Families, Caregivers, and Aging

Caregiving for individuals with intellectual and developmental disabilities is a prolonged endeavor that can last 60 years or more and, for the vast majority of families, becomes a lifelong commitment (Milberger et al., 2023). Many older adults with I/DD continue to reside with aging parents or family members who may themselves be managing declining health, chronic illness, cognitive changes, and reduced physical capacity. As caregivers age, their own declining abilities increase the burden of care, while the aging process simultaneously produces significant social impacts, including isolation, loneliness, and reduced access to support networks (Samuel, 2024).

In these households, the caregiving relationship is often reciprocal and deeply interdependent, with aging parents and their adult children with I/DD relying on one another in ways that make transition planning both emotionally complex and practically urgent (Samuel et al., 2026). Financial strain, caregiver burnout, and uncertainty about the future further compound the stress experienced by families at this stage of life (Marsack-Topolewski et al., 2024). Aging family caregivers of adults with I/DD therefore face a distinct set of challenges that set them apart from the broader caregiving population, challenges that demand proactive planning, coordinated support, and social work practice that is attentive to the needs of both the individual with I/DD and the family members who have devoted their lives to their care.

When aging parents are no longer able to provide care due to illness, hospitalization, or death, the impact on the individual with I/DD can be profound. The loss of a primary caregiver frequently disrupts not only living arrangements but also daily routines, emotional support systems, medical management, and community connections that the individual may have relied upon for decades. Without proactive planning, these disruptions can precipitate crises including emergency residential placements, lapses in service coordination, and significant declines in emotional and physical well-being. Increasingly, siblings and extended family members step into caregiving roles following the death or incapacitation of parents, often with limited preparation, knowledge of existing services, or understanding of the individual's history, preferences, and legal arrangements.

Social workers play a central role in helping families plan proactively and compassionately for these transitions before a crisis occurs. This includes facilitating family meetings, providing counseling to both individuals with I/DD and their caregivers, assisting with future planning documents, and supporting families in making informed decisions about guardianship, Supported Decision-Making, residential options, and long-term financial security. Social workers also connect families with community resources, including respite care, peer support networks, legal services, and OPWDD planning support, which can reduce **caregiver burden** and strengthen the family's capacity to plan ahead. When transitions occur, social workers coordinate continuity of services, advocate for appropriate placements, and provide emotional support to both the individual with I/DD and family members navigating loss, change, and new caregiving

responsibilities. Throughout this work, a person-centered and trauma-informed lens ensures that the individual's voice, preferences, and well-being remain at the center of every decision.

Residential Supports and Aging in Place

Many older adults with I/DD express a strong desire to remain in familiar homes and communities as they age (Steele et al., 2026). This advocacy reflects an attachment to place and the importance of routine, relationships, and a sense of community belonging for overall well-being. **Aging in place** for individuals with I/DD emphasizes maintaining independence and quality of life through environmental modifications, assistive technology, home and community-based services (HCBS), and individualized supports tailored to the person's evolving needs. When carefully planned and adequately resourced, aging in place can promote dignity, autonomy, and continued community participation for individuals with I/DD well into later adulthood.

Social workers play a critical role in this process, facilitating conversations about future housing, connecting families with available resources, advocating for accessible and appropriate placements, and supporting individuals through residential transitions with continuity, care, and respect for their life history. **Residential planning** requires careful, proactive consideration of health needs, physical accessibility, safety, social participation, and personal preferences, ideally beginning well before a crisis causes an abrupt transition. Options exist across a continuum of support intensity, including family homes with in-home supports, supervised or supportive apartments, certified residential settings, intermediate care facilities, and skilled nursing facilities for individuals with higher medical needs. Each option carries distinct implications for autonomy, community integration, and quality of life, and no single model is appropriate for all individuals. Regardless of setting, person-centered planning remains essential, ensuring that the individual’s voice, preferences, and goals remain central to every housing decision and that residential supports are regularly reviewed and adjusted as needs change over time.

End-of-Life Planning

Advances in healthcare and increased life expectancy have made end-of-life planning an increasingly important dimension of social work practice with individuals with I/DD (Alford, 2025; S. Alford & Holmes, 2025). However, these conversations are frequently delayed or inadequately addressed within disability service systems, leaving individuals and families unprepared when health crises arise. Social workers play a central role in ensuring proactive, compassionate planning and facilitating discussions about advance directives, healthcare proxies, supported decision-making arrangements, guardianship, hospice and palliative care, funeral planning, and bereavement support.

Effective **end-of-life planning** requires conversations that begin early, well before a terminal diagnosis makes planning urgent, and are revisited regularly as health, circumstances, and preferences evolve. These discussions must be culturally responsive, recognizing that attitudes toward death and dying are shaped by cultural, spiritual, and familial values that vary across communities. Communication approaches should be adapted to the individual's cognitive and communicative abilities to ensure meaningful participation in decisions about their own care. Where an individual cannot fully communicate their wishes, social workers advocate for decisions

that reflect the person's known values, relationships, and life history rather than defaulting solely to surrogate judgment.

Older adults with I/DD frequently experience disenfranchised grief, and their sorrow over the loss of a parent, sibling, or housemate is often minimized or ignored by those around them. Social workers provide tailored **grief and bereavement counseling** using adapted therapeutic techniques such as memory books, concrete visual aids, and structured rituals to help individuals process loss.

When major transitions such as leaving a lifelong family home or retiring from a day habilitation program produce relocation stress and anxiety, social workers implement behavioral support plans, gradual transition schedules, and **trauma-informed counseling** to support adjustment. End-of-life care for individuals with I/DD reflects the core principles of the Person-Centered Ethics of Care Model: dignity, autonomy, relational support, cultural responsiveness, and compassion, ensuring that individuals are visible within end-of-life systems and supported with the same respect and intentionality that should define every stage of their lives.

Transitioning from Substituted to Supported Decision-Making

As aging caregivers become less able to provide support or pass away, individuals with I/DD often face increased vulnerability in decisions related to healthcare, finances, housing, and long-term services. Historically, families have relied on guardianship or other forms of substituted decision-making, in which another person makes decisions on behalf of the individual. However, there is growing recognition that adults with I/DD should be supported to exercise their legal capacity and participate meaningfully in decisions affecting their lives. **Supported decision-making (SDM)** offers a less restrictive alternative, enabling individuals to make their own choices with assistance from trusted supporters who help them understand information, evaluate options, and communicate decisions. Legal and planning tools, including healthcare proxies, **special needs trusts**, powers of attorney, and SDM agreements, help ensure continuity of support while preserving autonomy, self-determination, and dignity during the transition to later adulthood (AAIDD, 2023). Social workers advocate for the least restrictive frameworks available, helping families establish SDM agreements and limited co-guardianships rather than defaulting to plenary guardianship when a parent passes away.

The Person-Centered Ethics of Care Model and Healthy Aging

The Person-Centered Ethics of Care Model (PC-EoC; Alford & Son, 2025) provides an ethically grounded framework for understanding and supporting healthy aging among individuals with I/DD. Rather than viewing aging through a narrow medical lens focused on diagnosis and decline, the model recognizes that successful aging results from coordinated interactions among multiple systems, including healthcare, behavioral health, housing, family supports, governmental services, financial planning, and community participation (Kattari et al., 2017). The aging experiences of individuals with I/DD are shaped not only by biological processes but by decades of navigating systemic barriers, service fragmentation, and cumulative social disadvantage. The PC-EoC Model positions these realities not as individual deficits but as structural conditions requiring systemic responses grounded in justice, equity, and relational care.

As individuals age, interactions among these systems become increasingly complex and consequential: a change in health status may affect housing stability; the loss of a caregiver may disrupt access to services; financial insecurity may limit community participation. Social workers play a central coordinating role by facilitating communication among providers, disability service agencies, housing programs, and family members; advocating for responsive and accessible services; supporting families through planning and transition; and ensuring that practice remains anchored in the individual's strengths, preferences, and evolving needs rather than system convenience. Guided by the principles of dignity, relational autonomy, cultural responsiveness, and structural awareness, the PC-EoC Model affirms that healthy aging is not achieved through isolated interventions but through coordinated, person-centered systems of care that honor the full humanity of each individual, approaching aging not as a problem to be managed but as a stage of life to be supported with the same commitment to self-determination and inclusion that defines person-centered practice across the entire lifespan.

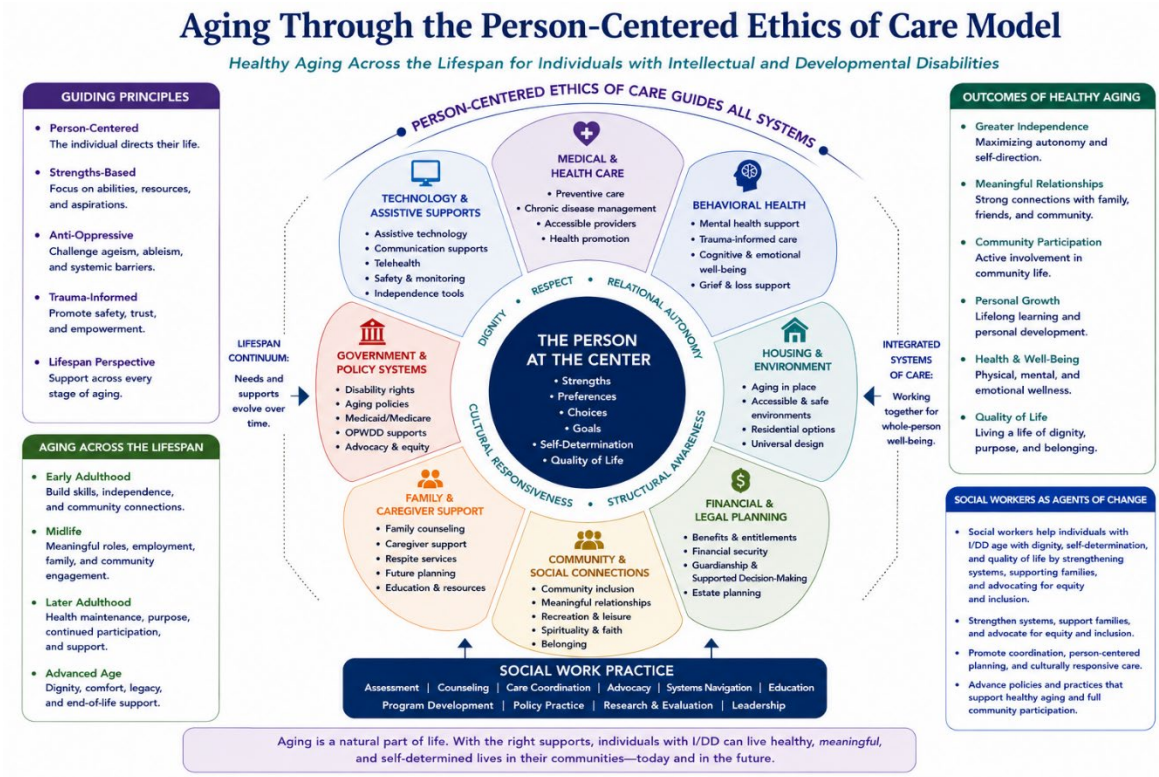


Figure 10.1. *This figure illustrates the **Person-Centered Ethics of Care Model** as a framework for promoting healthy aging among individuals with intellectual and developmental disabilities (I/DD).*

Pause & Reflect

1. Looking at the Person-Centered Ethics of Care Model, which system (e.g., healthcare, housing, family support, financial planning, or community connections) do you believe has the greatest influence on healthy aging for individuals with I/DD? Explain your reasoning.
2. How can social workers use this framework to coordinate services across multiple systems while ensuring that the individual's preferences, strengths, and self-determination remain at the center of decision-making?
3. Reflect on the role of family, community, and policy in supporting healthy aging. What challenges might arise when one or more of these systems are absent or not working collaboratively?
4. As individuals with I/DD age, their needs often become more complex. How does this model demonstrate the importance of interdisciplinary collaboration and person-centered planning throughout the aging process?

Social Work Practice with Older Adults with I/DD

Social workers serve many evolving roles throughout the aging process, including assessor, counselor, advocate, care coordinator, educator, family consultant, crisis intervener, program developer, researcher, and policy advocate (Patil & Ronad, 2025). Because older adults with I/DD navigate two distinct service delivery networks, the aging network and the disability network, social workers serve as essential bridges, clinical anchors, and systemic advocates (Heller et al., 2015). Effective intervention encompasses comprehensive assessment, counseling, family support, interdisciplinary collaboration, healthcare navigation, behavioral consultation, housing planning, benefits counseling, and end-of-life planning. Across all roles and settings, social workers promote self-determination, community inclusion, and equitable access to services, ensuring that the dignity and personhood of each individual remain central to all aspects of practice.

Healthy aging for individuals with I/DD extends far beyond disease prevention, encompassing strong relationships, lifelong learning, recreation, volunteerism, spirituality, physical activity, and active community participation. Social workers promote healthy aging by encouraging adequate nutrition, regular exercise, preventive screenings, social engagement, and the use of assistive technology, and by developing individualized wellness plans that reflect each person's preferences, capacities, and goals. When individuals with I/DD are supported to age well, they remain visible, valued, and engaged members of their communities.

The growing population of older adults with I/DD reveals the urgent need for continued policy development and systemic reform (Pham et al., 2024). Social workers advocate at the macro level for expanded access to Medicaid home and community-based services, equitable healthcare, accessible housing, caregiver supports, transportation, and aging policies that are explicitly inclusive of individuals with I/DD. Effective advocacy in this space requires confronting both ableism and ageism, recognizing that many older adults with I/DD occupy the intersection of multiple forms of discrimination that compound one another and limit opportunities for full community participation. Social workers are uniquely positioned to name these intersecting injustices, challenge the systems that sustain them, and advocate for a future in which aging with

a disability is met with expanded rights, robust supports, and unwavering respect for human dignity.

Conclusion

The transition to later adulthood for individuals with I/DD is profoundly shaped by changes in immediate support networks, including the loss of parents, siblings, and peers, and by the complex intersection of aging and disability service systems. Older adults with I/DD are among the fastest-growing populations served by social workers, and their increasing longevity presents both remarkable opportunities and significant challenges for individuals, families, service systems, and policymakers. Social workers play a pivotal role in this transition, providing **proactive permanency planning** for caregivers, promoting supported decision-making and self-determination for individuals, bridging the gap between disability and aging systems, and offering tailored grief, bereavement, and emotional support as individuals navigate compounding losses. Promoting healthy aging requires coordinated, person-centered systems that recognize each individual's strengths, preferences, and rights while actively addressing lifelong health disparities and supporting meaningful community participation.

Guided by the Person-Centered Ethics of Care Model, social workers are uniquely positioned to lead interdisciplinary collaboration, advocate for equitable policies, and strengthen family and community support, ensuring that individuals with I/DD age with dignity, independence, and the highest possible quality of life. As the field continues to evolve, social work will remain central to building the inclusive, responsive, and just systems that successful aging across the lifespan demands.

Discussion Questions

1. How do the aging experiences of older adults with I/DD differ from those of the general aging population, and what are the implications for social work practice?
2. How can person-centered future planning and advocacy promote self-determination and decision-making for individuals with I/DD as they transition to later adulthood?
3. What role do social workers play in fostering proactive permanency planning for aging parental caregivers and self-advocacy among individuals with I/DD?
4. What are the key challenges in navigating the aging and disability service sectors for individuals with I/DD, and how can social workers support this process?
5. How can social workers reduce health disparities and improve healthcare access for older adults with I/DD?
6. How can social workers support individuals with I/DD socially and emotionally as they face compounding losses, including the death of a parent, sibling, or residential peer?
7. Using the Person-Centered Ethics of Care Model, explain how coordinated systems of care promote healthy aging and quality of life for older adults with I/DD.
8. How can interprofessional collaboration improve the early identification and assessment of cognitive decline or atypical aging among individuals with I/DD, and what specific strategies can social workers implement to facilitate this collaboration?

Case Scenario: "Who Will Care for Marcus?"

Case Scenario: Marcus

Marcus Williams is a 67-year-old African American man with a diagnosis of moderate Intellectual Disability and Autism Spectrum Disorder. He has lived with his older sister, Gloria, for the past 42 years following their mother's passing. Gloria, now 78, became Marcus's legal guardian when he was in his twenties and has devoted her life to his care. Marcus attends a local day habilitation program where he enjoys gardening, puzzles, music activities, and time with a small group of longtime friends.

Over the past two years, Gloria has noticed significant changes in Marcus's health and behavior. He has become slower when walking, fallen twice in six months, and now requires assistance with bathing and dressing , tasks he previously completed independently. He has begun forgetting familiar routines, becoming disoriented in his own neighborhood, and experiencing confusion during conversations. Day habilitation staff report increasing withdrawal, reduced interest in activities he once loved, and greater frustration when routines change.

Marcus has several chronic medical conditions including Type II Diabetes, hypertension, obesity, epilepsy, arthritis, and hearing loss, and currently takes eight prescription medications prescribed by four different providers. Gloria struggles to coordinate appointments among his primary care physician, neurologist, endocrinologist, and psychiatrist. Several providers have dismissed her concerns, attributing Marcus's behavioral changes solely to his disability or aging , without conducting further assessments.

Gloria's own health has also declined. She has congestive heart failure, arthritis, and recently underwent knee replacement surgery. Though deeply committed to Marcus's care, she acknowledges physical exhaustion and worries about the future. No formal care plan has ever been developed. Marcus's nephew Anthony has expressed a willingness to help but lives three hours away and has limited knowledge of disability services. The family has never discussed residential options, Supported Decision-Making, advance care planning, or long-term financial arrangements , conversations Gloria finds emotionally overwhelming.

Marcus was recently hospitalized after falling at home and developing dehydration from an untreated urinary tract infection. Hospital staff recommended placement in a skilled nursing facility. Gloria strongly disagrees, believing Marcus would become depressed if removed from his home and community. Marcus repeatedly states, "I want to stay with Gloria," while also expressing sadness about her declining health.

Key Practice Issues in This Case:

- Balancing Marcus's right to self-determination with health and safety concerns
- Supporting an aging caregiver experiencing significant burden
- Coordinating care across multiple health and disability systems
- Addressing diagnostic overshadowing in medical settings
- Exploring residential and community-based alternatives to institutional placement
- Facilitating difficult family conversations about future planning
- Ensuring Marcus's voice remains central to every decision

Social Work Intervention: PC-EoC in Practice

A hospital social worker is assigned to develop a comprehensive discharge and long-term care plan for Marcus and his family. Using the Person-Centered Ethics of Care Model, the social worker begins facilitating interdisciplinary meetings involving Marcus, Gloria, Anthony, healthcare providers, OPWDD representatives, and community service agencies.

The team must determine how to promote Marcus's health, preserve his independence and quality of life, support Gloria during her own aging process, and develop a person-centered plan that prepares the family for future transitions , all while honoring Marcus's lifelong relationships, preferences, and dignity.

Discussion Questions

1. What biological, psychological, social, and environmental factors contribute to Marcus's current situation, and how do these factors illustrate the complexity of aging with I/DD?
2. Identify the health disparities and examples of diagnostic overshadowing present in this case. How might these barriers affect Marcus's long-term health and quality of life?
3. Using the Person-Centered Ethics of Care Model, identify the major systems that should be involved in Marcus's care. How should these systems collaborate to support both Marcus and Gloria?
4. What interventions should the social worker implement at the micro, mezzo, and macro levels to address Marcus's immediate needs while also promoting long-term healthy aging and family stability?
5. If you were the social worker assigned to Marcus's case, how would you balance his desire to remain at home with concerns about safety, caregiver burden, and future planning while preserving his autonomy, dignity, and self-determination?

Key Terms and Definitions

Term	Definition
Accelerated Aging (Premature Aging)	The earlier onset of age-related physical, cognitive, and functional changes experienced by some individuals with intellectual and developmental disabilities compared to the general population.
Aging in Place	An approach that supports older adults with I/DD in remaining safely and independently in their homes or familiar community settings through individualized supports, environmental modifications, and community-based services.
Caregiver Burden	The physical, emotional, financial, and psychological strain experienced by family members or other caregivers who provide long-term support to individuals with I/DD.
End-of-Life Planning	A person-centered process that prepares individuals and families for future healthcare, legal, and personal decisions through advance directives, supported decision-making, palliative care, hospice, and bereavement planning.
Grief and Bereavement Counseling	Specialized therapeutic support designed to help individuals process the profound emotional, psychological, and behavioral impacts of loss, which is particularly vital for individuals with I/DD experiencing the decline or death of lifelong parental caregivers.
Healthy Aging	The process of maintaining physical health, emotional well-being, independence, meaningful relationships, and community participation throughout later adulthood while maximizing quality of life.
Medicaid Home and Community-Based Services (HCBS) Waivers	State-administered programs that provide funding for long-term services and supports delivered in the community rather than in institutional settings, helping individuals with I/DD maintain independence.
Polypharmacy	The concurrent use of multiple medications, which increases the risk of adverse drug interactions, side effects, and medication-related complications, particularly among older adults with I/DD.
Pre-Admission Screening and Resident Review (PASRR)	A federally mandated screening process designed to ensure that individuals with intellectual disabilities or mental illness are not inappropriately placed in nursing facilities, verifying they receive necessary specialized services.

Proactive Permanency Planning	The strategic process of establishing long-term, stable caregiving and residential arrangements before a crisis occurs, typically anticipating the inevitable transition when aging parental caregivers can no longer provide support.
Residential Planning	The collaborative process of identifying and coordinating housing options and long-term living supports that promote safety, autonomy, accessibility, and quality of life as individuals age.
Special Needs Trusts	A specialized legal and financial mechanism that allows assets to be held for the benefit of an individual with a disability without jeopardizing their eligibility for government public assistance benefits such as SSI and Medicaid.
Successful Aging	A strengths-based approach to aging that emphasizes health, self-determination, participation, purpose, and quality of life rather than focusing solely on disease prevention or the absence of disability.
Trauma-Informed Counseling	A clinical approach to therapy that recognizes, understands, and responds to the pervasive effects of trauma, emphasizing physical and psychological safety, trustworthiness, choice, and empowerment for the client.

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Chapter 11: I/DD: Social Work Practice and Advocacy

Learning Objectives

By the end of this chapter, students will be able to:

- Describe the diverse roles and responsibilities of social workers across the lifespan of individuals with I/DD.
- Apply the Person-Centered Ethics of Care framework to assessment, intervention, advocacy, and systems coordination.
- Analyze the scope of social work practice in clinical, community, organizational, and policy settings.
- Evaluate advocacy strategies that promote self-determination, disability rights, and social justice.
- Examine the role of social workers in interdisciplinary collaboration, program development, and research.
- Integrate ethical, culturally responsive, and anti-oppressive approaches into professional social work practice with individuals with I/DD and their families.

Table 11.1. *Alignment of Chapter Learning Objectives With CSWE EPAS (2022) Competencies*

Learning Objective	CSWE Competency	Alignment Rationale
Describe the roles of social workers in I/DD practice	Competency 1: Demonstrate Ethical and Professional Behavior	Highlights professional identity and ethical responsibilities across practice settings.
Apply the Person-Centered Ethics of Care framework	Competency 7: Assess Individuals, Families, Groups, Organizations, and Communities	Integrates assessment with person-centered, strengths-based practice.
Analyze the scope of practice across systems	Competency 6: Engage with Individuals, Families, Groups, Organizations, and Communities	Emphasizes engagement across diverse systems and populations.
Evaluate advocacy strategies	Competency 3: Advance Human Rights and Social, Racial, Economic, and Environmental Justice	Focuses on disability rights, inclusion, and social justice advocacy.
Examine interdisciplinary collaboration and research	Competency 4: Engage in Practice-Informed Research and Research-Informed Practice	Encourages evidence-informed practice and interdisciplinary collaboration.
Integrate ethical and anti-oppressive practice	Competency 8: Intervene with Individuals, Families, Groups, Organizations, and Communities	Promotes holistic intervention and systems change.

Introduction: The Role of Social Workers in the I/DD Field

Social workers occupy a unique position within the field of intellectual and developmental disabilities (I/DD), serving as clinicians, advocates, care coordinators, educators, researchers, administrators, and policy leaders. Their practice extends well beyond direct service delivery to encompass systems navigation, interdisciplinary collaboration, program development, community engagement, and **disability rights advocacy** supporting individuals with I/DD and their families in overcoming barriers, accessing resources, promoting self-determination, and improving quality of life across the lifespan.

Successful social work practice in the I/DD field requires specialized knowledge and a deep commitment to equity, inclusion, and human rights. Social workers must understand the complex interplay between individual needs, family systems, service delivery structures, and the broader social and political forces that shape access to care. They must also be prepared to challenge ableism, dismantle systemic barriers, and advocate for policies and practices that affirm the dignity and worth of every individual. This demands technical competence, critical self-reflection, cultural humility, and a willingness to center the voices and experiences of individuals with I/DD in all aspects of practice.

This concluding chapter integrates the concepts introduced throughout the textbook, demonstrating how social workers apply person-centered, strengths-based, anti-oppressive, and trauma-informed approaches across micro, mezzo, and macro systems. Guided at every level by the Person-Centered Ethics of Care Model, the chapter illustrates how ethical principles translate into concrete practice, from empowering individuals and supporting families to transforming organizations, shaping policy, and advancing a more just and inclusive society for all individuals with I/DD.

The Expanding Role of Social Workers in the I/DD Field

Social workers assume multiple professional roles that evolve across the lifespan of individuals with I/DD, spanning direct clinical practice, case management, advocacy, policy development, community organizing, education, and leadership, rather than performing a single function. They promote quality of life by conducting comprehensive assessments, facilitating person-centered planning, coordinating services across systems, providing counseling and family support, and developing programs that improve access to community-based care.

The settings in which social workers practice reflect the breadth of need across the lifespan. From Early Intervention and school-based services to OPWDD agencies, residential programs, day habilitation services, vocational rehabilitation, mental health clinics, hospitals, aging services, and hospice care, social workers are present at every stage of an individual's journey serving and the linchpin to services and adapting their roles to meet the demands of each environment while maintaining a consistent commitment to dignity, inclusion, and self-determination.

Assessment and Person-Centered Planning

Comprehensive assessment represents the foundation of effective social work practice with individuals with I/DD. Instead of focusing exclusively on deficits, social workers conduct **strengths-based assessments** that examine individual capacities, family resources, environmental supports, and community assets alongside areas of need. Assessment encompasses psychosocial functioning, adaptive behavior, family systems, cultural influences, environmental barriers, trauma history, risk factors, decision-making capacity, and quality of life; information gathered across these domains guides the development of individualized intervention plans that reflect each person's goals, preferences, and aspirations.

Care Coordination and Systems Navigation

Individuals with I/DD often receive services from multiple providers across educational, medical, behavioral health, governmental, financial, and community systems simultaneously, making integration and **continuity of care** essential. Social workers serve as care coordinators, ensuring services are organized, connected, and responsive to each individual's evolving needs. This includes coordinating OPWDD services, facilitating **Life Plans**, **Person-Centered Goal Plans**, assisting with residential placement, supporting guardianship and Supported Decision-Making arrangements, connecting families with Medicaid and public benefits, and promoting continuity of care throughout major life transitions.

Advocacy Across Micro, Mezzo, and Macro Practice

Advocacy is a defining characteristic of social work practice within the I/DD field, operating across all levels of intervention. At the micro level, social workers empower individuals and families through education, counseling, supported decision-making, and self-advocacy development. At the mezzo level, they work within organizations and communities to improve institutional policies, expand inclusive programming, and strengthen **interdisciplinary collaboration**. At the macro level, they engage in disability rights legislation, advocacy for equitable funding, and systemic reforms that promote community participation and social justice. This multilevel approach is grounded in anti-oppressive practice and seeks to improve access to services and dismantle ableism, challenge structural inequities, and advance the full inclusion of individuals with I/DD across all aspects of society.

Counseling and Therapeutic Practice

Social workers provide therapeutic interventions that address the emotional, behavioral, relational, and psychological needs of individuals with I/DD and their families, including individual counseling, family therapy, group interventions, crisis intervention, grief counseling, and **Behavior Support planning**. Central to clinical practice is recognizing that individuals with I/DD experience the full range of emotional and mental health concerns as the broader population, while often facing additional barriers rooted in stigma, discrimination, trauma, and diagnostic overshadowing, all of which must be addressed within a culturally responsive and trauma-informed framework.

Residential Planning and Life Transitions

Throughout adulthood, social workers assist individuals with I/DD and their families in planning for residential services, employment, aging, end-of-life care, and other significant life transitions, emphasizing informed decision-making, family mediation, crisis resolution, housing stability, and community integration. Central to this work is a commitment to continuity of care across the lifespan, ensuring that transitions are planned thoughtfully, services remain coordinated, and the individual's self-determination and **quality of life** remain at the center of every decision.

Leadership, Program Development, and Research

Beyond direct practice, social workers contribute to developing innovative programs, evaluating service effectiveness, and advancing **evidence-informed practice**. Social work research informs policy decisions, improves service delivery, and strengthens the profession's knowledge base in disability practice (Julkunen et al., 2023). Leadership responsibilities include supervising interdisciplinary teams, developing community partnerships, designing inclusive programs, conducting program evaluations, and translating research findings into actionable practice, positioning social workers as drivers of systemic improvement across the I/DD field.

Specialized Knowledge for I/DD Social Work Practice

Effective practice requires specialized knowledge spanning disability policy, family systems, mental health, trauma, behavioral interventions, healthcare disparities, guardianship, Supported Decision-Making, ethics, aging, end-of-life planning, and sociolegal issues. Social workers integrate this knowledge with cultural humility and interdisciplinary collaboration to address the complex, intersecting needs of individuals with I/DD and their families, recognizing that no single domain of knowledge is sufficient on its own.

Conclusion

Social work has become an essential profession in advancing the rights, inclusion, and well-being of individuals with intellectual and developmental disabilities and their families. Throughout this textbook, readers have explored disability through person-centered, strengths-based, anti-oppressive, and systems-oriented perspectives, frameworks that position social workers not only as service providers but as advocates, leaders, researchers, educators, and agents of social change. The future of disability practice depends on professionals who are committed to advancing equity, promoting self-determination, strengthening communities, and ensuring that individuals with I/DD are recognized as full and valued members of society. Guided by the Person-Centered Ethics of Care framework, social workers are uniquely positioned to transform both individual lives and the systems that shape them.

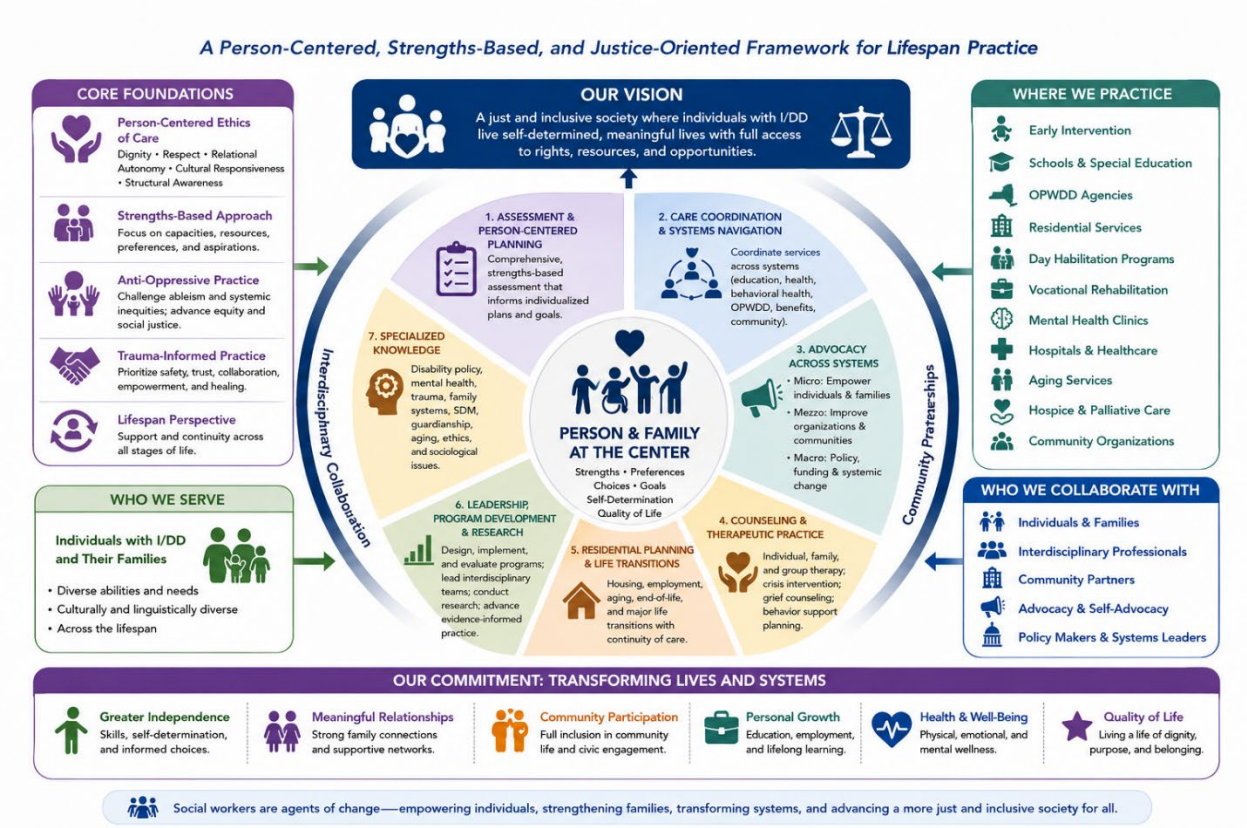


Figure 11. 1. *Illustration of the Person-Centered, Strengths-Based, and Justice-Oriented Framework for Social Work Practice in the I/DD Field.*

Conclusion: The Impact of Social Workers in the Field of I/DD

Social workers have a profound impact on the lives of individuals with intellectual and developmental disabilities, contributing to their well-being through direct practice, **advocacy**, and systemic change. Centering self-determination and applying holistic, person-centered approaches, social workers help ensure that individuals with I/DD and their families are supported in building the lives they desire and deserve.

Discussion Questions:

1. **The Role of Social Workers in the Field of I/DD**
    - How do social workers enhance the quality of life for individuals with I/DD in various settings?
    - What are some examples of how social workers can apply their knowledge and skills to support individuals with I/DD and their families?
    - In what ways can social workers contribute to the development of policies and programs that benefit the I/DD community?

2. **Scope of Practice in the Field of I/DD:**
    - How does a strengths-based approach to assessment benefit individuals with I/DD and their families?
    - What are the key components of capacity building in the I/DD field, and how can social workers effectively engage with communities to promote inclusivity?
    - How can social workers ensure that care coordination is effectively managed, particularly when working with agencies like OPWDD?

3. **Advocacy in the Field of I/DD**
    - What are the most effective strategies for supporting self-advocacy among individuals with I/DD?
    - How can social workers advocate for systemic changes that improve the lives of individuals with I/DD?
    - What challenges might social workers face when advocating for individuals with I/DD, and how can they overcome these challenges?

4. **Counseling and Therapeutic Approaches:**
    - How can social workers tailor therapeutic approaches to meet the unique needs of individuals with I/DD and their families?
    - What are the benefits of incorporating positive behavior support plans into social work practice?
    - How can social workers support individuals with I/DD and their families through significant life transitions, such as adjusting to a disability diagnosis?

5. **Residential Planning and Mediation:**
    - What factors should be considered when helping individuals with I/DD plan for their residential needs?
    - How can social workers effectively mediate conflicts between individuals with I/DD and their families or other parties?
    - What role do social workers play in crisis resolution, particularly for individuals with I/DD who face housing instability?

6. **Policy, Program Design, and Research:**
    - How can social workers contribute to the design and evaluation of programs that serve the I/DD community?
    - What are the key challenges in conducting research in the I/DD field, and how can social workers address these challenges?
    - How can social workers ensure that their research findings are translated into practice and policy changes that benefit individuals with I/DD?

7. **Specialized Expertise in the I/DD Field:**
    - What are some of the specialized knowledge areas that social workers need to effectively support individuals with I/DD?
    - How can social workers identify and address the complex family dynamics that often accompany I/DD?

- What strategies can social workers use to support individuals with I/DD and their families through end-of-life decision-making and other sociolegal issues?

8. **Wrap-Up: The Impact of Social Workers in the I/DD Field:**
    - How does the Ethics of Care model inform social work practice in the I/DD field?
    - In what ways can social workers empower individuals with I/DD to make decisions about their own lives?
    - How can social workers ensure that their contributions to the I/DD field are both impactful and sustainable?

Scenario: Advocating for Inclusive Community Services for Adults with I/DD

Case Scenario: Maria

Maria is a 28-year-old woman with a mild intellectual disability who lives independently in a small rural community with support from a local agency that assists with daily living skills such as budgeting, meal planning, and transportation. She works part-time at a local grocery store and is active in her church. Despite this, Maria feels increasingly isolated due to the lack of social opportunities and accessible community services in her area.

Maria's social worker, Ethan, has noticed her expressing loneliness and frustration. She has shared that few activities exist for adults with disabilities in her community, and she often feels excluded from local events that are not designed to accommodate her needs. She also faces significant barriers to healthcare access, as the nearest clinic is 30 miles away and public transportation is limited.

Ethan recognizes that Maria's situation reflects broader systemic issues common in rural areas, where resources for individuals with I/DD are often scarce and community services may not be inclusive. He decides to work with Maria to advocate for changes that will improve her quality of life and that of others with I/DD in the community.

Advocacy and Social Work Intervention

Ethan begins by meeting with Maria to explore her concerns in depth, listening carefully to her experiences and helping her articulate the specific changes she would like to see. Together, they develop a multi-step advocacy plan:

Community Needs Assessment: Ethan and Maria gather information about the challenges faced by individuals with I/DD in the area, reaching out to other adults with disabilities, their families, and local service providers. They also collect data on existing services, transportation options, and healthcare access.

Building a Coalition: They connect with local advocacy groups, disability organizations, and community members to build a coalition. Maria takes an active role in meetings, sharing her experiences and raising awareness about the barriers individuals with I/DD face in rural settings.

Advocacy Campaign: The coalition meets with local government officials, healthcare providers, and transportation authorities to present their case and advocate for increased funding and services. A petition circulated by Maria calls for inclusive community events, improved transportation, and an accessible local healthcare clinic.

Engaging the Media: Ethan helps Maria prepare a letter to the editor of the local newspaper sharing her story. They also work with a local news station to cover their advocacy efforts, broadening community awareness and support.

Collaboration with Service Providers: Ethan and Maria work with local providers to develop new programs, including a weekly social group for adults with disabilities, organized transportation to healthcare appointments, and advocacy for inclusive practices at community events.

Outcomes

Over time, the advocacy efforts begin to show results. The local government allocates funding for a pilot transportation program providing accessible rides to healthcare facilities and community events. A local church offers space for a weekly social group for adults with disabilities, and several community organizations commit to making their events more inclusive through accommodations such as sign language interpreters and sensory-friendly environments.

Maria feels more connected to her community and empowered by her role in the advocacy process. She continues working with Ethan and the coalition to sustain these changes and advocate for further improvements , emerging as a leader in her community's efforts to create a more inclusive environment for people with I/DD.

Discussion Questions

1. How did Ethan and Maria's advocacy strategy address the systemic issues affecting individuals with I/DD in a rural community? What were the key elements that contributed to the success of their efforts?
2. What specific roles did Ethan play as a social worker in supporting Maria's advocacy efforts, and how did these roles align with the principles of the Person-Centered Ethics of Care Model?
3. How did involving Maria in the advocacy process benefit her personally, and why is it important for individuals with I/DD to be active participants in advocating for their own needs?
4. What challenges did Maria and Ethan face in their advocacy efforts, and how did they work to overcome these challenges?
5. What steps can be taken to ensure that the changes achieved through advocacy are sustained over the long term, and how can social workers continue to support communities in creating inclusive environments for individuals with I/DD?

Key Terms & Definitions

Term	Definition
Advocacy	Actions taken by social workers to promote the rights, self-determination, inclusion, and equitable treatment of individuals with I/DD at the individual, organizational, community, and policy levels.
Anti-Oppressive Practice	A social work approach that recognizes and challenges systemic inequities, ableism, discrimination, and power imbalances while promoting equity, social justice, and inclusion.
Behavior Support Planning	A collaborative, person-centered process used to understand behaviors of concern and develop positive strategies that improve quality of life while respecting the individual's dignity and preferences.
Care Coordination	The process of organizing, integrating, and monitoring services across multiple providers and systems to ensure comprehensive, person-centered, and continuous care.
Community Integration	The meaningful participation of individuals with I/DD in community life through inclusive opportunities for employment, education, recreation, relationships, housing, and civic engagement.
Continuity of Care	The coordination of services across settings and throughout the lifespan to ensure consistent, uninterrupted, and person-centered support during transitions and changing life circumstances.
Disability Rights Advocacy	Efforts to protect and advance the civil, legal, and human rights of individuals with disabilities through education, policy reform, systems change, and community action.
Evidence-Informed Practice	Professional decision-making that integrates the best available research evidence with practitioner expertise, client preferences, and lived experience to guide interventions and service delivery.
Interdisciplinary Collaboration	The coordinated partnership among professionals from multiple disciplines who work together with individuals and families to develop, implement, and evaluate person-centered services.
Life Plan	A comprehensive, person-centered planning document used by OPWDD to identify an individual's goals, strengths,

	preferences, support needs, and desired outcomes while coordinating services across systems.
Person-Centered Goal Plan	An individualized planning tool that identifies meaningful goals, action steps, supports, and measurable outcomes based on the individual's strengths, preferences, choices, and aspirations.
Program Development	The systematic process of designing, implementing, evaluating, and improving services and programs that address the needs of individuals with I/DD and their families.
Quality of Life	An individual's overall well-being as reflected in health, independence, relationships, community participation, personal fulfillment, self-determination, safety, and life satisfaction.
Residential Planning	The collaborative process of identifying and arranging housing and residential supports that promote independence, safety, community inclusion, and person-centered living arrangements.
Self-Advocacy	The ability of individuals with I/DD to understand and communicate their rights, preferences, goals, and support needs while actively participating in decisions affecting their lives.
Social Justice	A core social work principle focused on eliminating inequities, promoting equal opportunity, and ensuring that individuals with I/DD have equitable access to resources, services, and full community participation.
Strengths-Based Assessment	A comprehensive assessment approach that identifies an individual's abilities, resources, resilience, family supports, and community assets in addition to areas requiring support.
Systems Navigation	The process of helping individuals and families understand, access, and coordinate complex educational, healthcare, governmental, financial, and social service systems.
Trauma-Informed Practice	An approach that recognizes the impact of trauma, prioritizes safety, trust, collaboration, empowerment, and cultural responsiveness, and seeks to avoid re-traumatization during service delivery.

References

Julkunen, I., Mauri, D., Ruch, G., & Isokuortti, N. (2023). Connecting social work research with policy and practice: Issues of people, power and politics. *European Social Work Research, 1*(1), 65–83.

APPENDIX A

Student Guide to Midterm and Final Case Presentations

The Midterm and Final Case Presentations are designed to provide students with an opportunity to integrate course concepts into professional social work practice. Students analyze a case involving individuals with intellectual and developmental disabilities (I/DD), apply evidence-based theories and interventions, and demonstrate critical thinking across micro, mezzo, and macro levels of social work practice. These assignments emphasize applying person-centered, strengths-based, anti-oppressive, and trauma-informed approaches while strengthening professional communication, teamwork, and presentation skills.

The purpose of these assignments extends beyond presenting information. Students are expected to assess complex situations, synthesize research, apply theoretical frameworks, develop evidence-informed interventions, and communicate recommendations consistent with professional social work practice.

Selecting and Developing a Case

Each student group will develop a comprehensive case presentation focused on an individual, family, organization, or community affected by intellectual and developmental disabilities. Cases may be based on course material, realistic practice scenarios, current research, or professional experiences with identifying information removed. Topics outside the course may also be selected with instructor approval.

A well-developed case should clearly describe the presenting concerns, strengths, environmental context, family dynamics, cultural considerations, and service systems influencing the individual or family. Rather than simply describing the disability, students should examine the broader ecological factors that contribute to the presenting issues and identify opportunities for intervention.

Conducting a Comprehensive Assessment

The foundation of every case presentation is a comprehensive assessment. Students should examine the biological, psychological, social, cultural, environmental, and systemic factors affecting the individual or family. Assessments should identify both strengths and areas of need while considering protective factors, available supports, barriers to services, and quality-of-life concerns.

Whenever appropriate, students are encouraged to organize their assessment using the Person-Centered Ethics of Care Model, demonstrating how multiple systems, including healthcare, education, family, government, financial supports, housing, and community resources, interact to influence outcomes.

Developing an Intervention Plan

Following the assessment, students should develop a comprehensive intervention plan that reflects evidence-based, person-centered social work practice. Intervention strategies should be presented across the three primary levels of social work practice.

Micro-Level Practice: Interventions focus on direct work with individuals and families and may include counseling, psychoeducation, crisis intervention, case management, supported decision-making, behavior support planning, and person-centered planning.

Mezzo-Level Practice: Interventions target groups, organizations, and service systems. Examples include family interventions, interdisciplinary collaboration, support groups, school consultation, agency coordination, staff training, and program development.

Macro-Level Practice: Interventions focus on broader systems change through policy advocacy, community education, disability rights initiatives, legislative reform, public awareness campaigns, and organizational change. Students should explain why each intervention was selected and how it addresses identified needs while promoting self-determination, inclusion, and quality of life.

Integrating Theory Into Practice

Strong presentations demonstrate the ability to connect theory with practice. Students should incorporate one or more theoretical frameworks discussed throughout the course to explain assessment findings and justify intervention strategies. Theoretical frameworks should guide the intervention rather than simply being described; students should clearly explain how theory informs their recommendations and professional decision-making.

Applicable Theoretical Frameworks:

- Person-Centered Ethics of Care Model
- Social Model of Disability
- Ecological Systems Theory
- Family Systems Theory
- Strengths Perspective
- Trauma-Informed Practice
- Anti-Oppressive Practice
- Neurodiversity Framework

Using Evidence-Based Practice

Professional social work practice requires integrating research into decision-making. Students should support their recommendations using peer-reviewed journal articles, government reports, professional practice guidelines, and course readings. Research should be synthesized to strengthen the rationale for interventions rather than summarized independently of the case.

Organizing the Presentation

Effective presentations are organized, engaging, and professionally delivered. Groups are encouraged to include the following components:

- Introduction to the case
- Comprehensive assessment
- Relevant theoretical framework(s)
- Micro, mezzo, and macro interventions
- Evidence-based rationale
- Expected outcomes
- Conclusion and recommendations

Visual aids should enhance understanding rather than duplicate the presentation. Slides should be clear, concise, and visually organized with limited text, appropriate graphics, and professional formatting.

Preparing the Written Paper

The written paper expands upon the presentation and demonstrates scholarly writing. Papers should include a detailed case description, a comprehensive assessment, the integration of theory, an evidence-based intervention plan, a discussion of expected outcomes, and a conclusion that summarizes the implications for social work practice. Cite all sources according to the current edition of APA guidelines.

Professional Expectations

Students are expected to demonstrate professional behavior throughout the project. Effective collaboration requires equitable participation, respectful communication, accountability, and timely completion of assigned responsibilities. Groups should meet regularly, review each member's work, and practice the presentation before the scheduled presentation date.

Professional presentations should demonstrate organized communication, accurate content, appropriate use of scholarly evidence, confidence, and the ability to respond thoughtfully to questions and feedback.

Strategies for Successful Presentations

Successful presentations typically emphasize the individual rather than the diagnosis, integrate theoretical frameworks throughout the analysis, support recommendations with current research, and demonstrate person-centered, strengths-based thinking. Effective groups clearly distinguish between micro, mezzo, and macro interventions, incorporate ethical and cultural considerations, and explain the rationale for each recommendation. Students should also consider the broader environmental and systemic factors influencing the case while recognizing the individual's strengths, preferences, and goals.

Common Challenges

Students frequently encounter challenges such as focusing exclusively on deficits, summarizing rather than analyzing the case, failing to integrate theory, proposing unrealistic interventions, or overlooking family and environmental influences. Other common issues include overcrowded presentation slides, limited use of scholarly literature, poor time management, and inadequate collaboration among group members. Recognizing these challenges early allows groups to strengthen both their presentation and written assignment while demonstrating higher levels of critical thinking and professional competence.

Evaluation Criteria

Both the presentation and written paper will be evaluated on the quality of case analysis, application of theoretical frameworks, use of evidence-based literature, development of intervention strategies, organization, professionalism, teamwork, critical thinking, communication skills, and adherence to APA formatting standards. Students are encouraged to review the grading rubric carefully before beginning the assignment to ensure they address all expectations.

Reflection

After completing the presentation, students should reflect on both the process and the outcome of the project. Reflection should consider how theoretical frameworks influenced intervention planning, how collaboration strengthened the final product, what was learned from peer and instructor feedback, and how the assignment contributed to the development of professional social work competencies. Students should also consider how the knowledge and skills gained through this project can be applied to future practice with individuals with intellectual and developmental disabilities across diverse settings.

APPENDIX B
Social Work Action Plans for Case Scenarios

This template provides a structured approach for social work students to systematically address the challenges presented in case scenarios. Each step guides students through identifying problems, gathering information, developing person-centered interventions, monitoring progress, and reflecting on practice to ensure responses are thoughtful and organized

Action Plan Template

Step	Action	Details & Instructions	Responsible	Timeline	Goals	Resources Needed
1	**Identify the Problem**	Clearly define the primary issue or challenge faced by the client or community.	Student / Case Team	Day 1	Gain a clear understanding of the core issue.	Case notes, client interviews, background information.
2	**Gather Information**	Collect data and insights from all relevant sources including client, family, service providers, and community.	Student / Case Team	Days 2–3	Comprehensive understanding of the client's situation and needs.	Assessment tools, client history, service provider feedback.
3	**Set Goals**	Define short-term and long-term goals that address the identified problem using SMART criteria.	Student / Client	Day 3	Establish clear, realistic, person-centered goals.	SMART goal framework; client-identified priorities.

4	**Develop a Plan of Action**	Outline specific steps and strategies to achieve the goals. Include interventions, resources, and supports.	Student / Case Team / Client	Days 4–5	Create a detailed, actionable plan tailored to the client's needs.	List potential interventions, resources, and supports required.
5	**Assign Responsibilities**	Determine who will be responsible for each action step to ensure accountability.	Student / Client / Service Providers	Days 5–6	Clear assignment of tasks across all team members.	Communication plan with service providers and stakeholders.
6	**Implement the Plan**	Put the action plan into practice, beginning with the most urgent or impactful steps.	Student / Case Team / Client	Day 7+	Begin taking concrete steps toward goal achievement.	Regular check-ins with client and stakeholders.
7	**Monitor Progress**	Regularly review progress toward goals, identifying barriers and making adjustments as needed.	Student / Client / Service Providers	Ongoing	Ensure the plan is on track; address emerging challenges.	Progress reports, feedback from client and providers.
8	**Evaluate Outcomes**	Assess whether goals were met and determine the effectiveness of the interventions.	Student / Case Team / Client	Weekly / Monthly	Evaluate success and identify areas for improvement.	Outcome data, client feedback, service provider reports.
9	**Adjust the Plan**	Based on evaluation, make	Student / Case Team / Client	As needed	Modify the plan to address new	Ongoing communication with all

		necessary changes to the action plan to better meet the client's needs.			issues or barriers.	involved parties.
10	**Document and Reflect**	Document the entire process including challenges, successes, and lessons learned. Reflect on practice.	Student / Case Team	End of Case	Comprehensive case documentation and professional reflection.	Case notes, reflection journals, supervisor feedback.

Instructions for Use

- **Identify the Problem: Begin** by clarifying the core issue or challenge within the scenario. Work closely with the client or community to understand their perspective and lived experience.

- **Gather Information: Collect** all necessary data, including client history, interviews, assessments, and input from service providers or community members. Ensure information is comprehensive and culturally responsive.

- **Set Goals: Collaboratively** establish achievable, person-centered goals that directly address the identified problem. Goals should be specific, measurable, and developed in partnership with the client.

- **Develop a Plan of Action: Create** a detailed plan outlining the steps needed to achieve each goal, including designated responsibilities and required resources. Interventions should be evidence-informed and realistic.

- **Assign Responsibilities: Ensure** each action item has a designated person responsible for its completion, fostering accountability across the team and with the client.

- **Implement the Plan: Begin** implementing the plan, prioritizing the most critical steps first. Maintain regular communication with the client and all involved stakeholders throughout this process.

- **Monitor Progress: Conduct** regular check-ins to assess progress, identify emerging barriers, and ensure the plan remains aligned with the client's evolving needs and goals.

- **Evaluate Outcomes: After** a designated period, evaluate whether goals were met and assess the impact of each intervention. Use client feedback and outcome data to guide evaluation.

- **Adjust the Plan: Based** on evaluation findings, modify the plan as needed to improve outcomes or address new challenges. Flexibility and responsiveness are essential components of effective practice.

- **Document and Reflect: Maintain** thorough records throughout the process and engage in structured reflection on what was learned, what could be improved, and how the experience informs future practice.

APPENDIX C
Person-Centered Goal Plan

The Person-Centered Goal Plan helps students and clients collaboratively set and achieve goals in a structured, supportive manner. This planning tool ensures that the individual's needs, preferences, and aspirations remain at the forefront of the planning process at every stage of intervention.

Client Information

Client Name:	Date:	Social Worker:	Review Date:

Goal Plan Template

Goal / Objective	Steps / Action Items	Responsible Person(s)	Resources & Support Needed	Timeline	Progress / Notes
Goal 1:					
Objective 1.1:					
Objective 1.2:					
Goal 2:					
Objective 2.1:					
Objective 2.2:					
Goal 3:					

Objective 3.1:					
Objective 3.2:					

Instructions for Use

- **Client Information: Begin** by completing the client name, date, social worker name, and review date at the top of the form.
- **Goal: Identify** a broad, overarching goal the client wants to achieve. Goals should be specific to the client's needs and desires and framed positively.
- **Specific Objective: Break** down each broad goal into smaller, measurable objectives, the milestones that will guide the client toward achieving the larger goal.
- **Steps / Action Items: List** the specific, actionable steps needed to accomplish each objective. Steps should be detailed and clearly describe what needs to be done.
- **Responsible Person(s): Identify** who is responsible for each action item. This may include the client, social worker, family members, or other support persons.
- **Resources & Support Needed: Note** any resources, services, tools, or additional assistance required to complete each step.
- **Timeline: Set** a realistic timeline for when each step or objective should be completed to support progress monitoring and accountability.
- **Progress / Notes: Use** this column to record progress, document challenges, and note any changes to the plan. This section should be updated regularly during scheduled check-ins.

Sample Completed Person-Centered Goal Plan

Client Name:	Date:	Social Worker:	Review Date:
John Mannery	January 10, 2024	Jamie Williams	March 10, 2024

Goal / Objective	Steps / Action Items	Responsible Person(s)	Resources & Support Needed	Timeline	Progress / Notes
Goal 1: Secure a part-time job in the community.					
Objective 1.1: Identify job interests.	Complete a career interest survey.	John Mannery, Jamie Williams	Access to career interest survey.	January 20, 2026	
Objective 1.2: Prepare a resume.	Create a draft resume. Review with job coach.	John Mannery, Job Coach	Computer access, resume template.	January 31, 2026	
Goal 2: Improve independent living skills.					
Objective 2.1: Develop a weekly budget.	List weekly expenses and income; create a budget plan.	John Mannery, Financial Counselor	Budget template, calculator.	February 15, 2026	
Objective 2.2: Cook one meal independently per week.	Choose a recipe, purchase ingredients, and prepare the meal.	John Mannery, Support Worker	Access to simple recipes, grocery funds.	February 10, 2026	

APPENDIX D

Guest Lecture Series: Invited Experts in the I/DD Field

Midterm Assignment Part II

During the guest lecture weeks, students will have the opportunity to hear from invited experts in the field of intellectual and developmental disabilities (I/DD). These lectures cover a range of critical topics designed to enhance students' understanding of how theory translates into professional practice. Guest speakers may include medical professionals, caregivers, policy experts, fellow faculty, and local specialists, each bringing a distinct perspective to the material covered throughout this course.

Students are required to review the guests' biographies prior to each session and come prepared with thoughtful questions related to the speaker's professional experience and expertise. Active participation in the guest lecture series constitutes Part II of the Midterm Assignment.

Proposed Lecture Topics

The guest lecture series covers three critical areas of practice within the I/DD field, offering students direct engagement with professionals working across healthcare, caregiving, and social work systems.

Topic 1: Medical Service Delivery Among Individuals with I/DD

***Focus*:** Medical professionals discuss the challenges and best practices in delivering healthcare services to individuals with I/DD, including access to care, adapting medical practices to meet diverse patient needs, and the role of multidisciplinary teams in providing comprehensive, coordinated care.

Topic 2: The Impact of Caring for a Loved One with I/DD

Focus: Caregivers and professionals explore the unique challenges faced by families and aging adults supporting individuals with I/DD. Discussions address the intersection of aging and disability, long-term care options, caregiver well-being, and strategies for maintaining quality of life across the lifespan.

Topic 3: Social Work Professionals in the I/DD Field
Focus: Social work practitioners share their experiences in direct service delivery, systems navigation, interdisciplinary collaboration, and the role of social workers in influencing policy decisions that affect individuals with I/DD and their families.

Benefits of the Guest Lecture Series

Interactive Learning: Guest lectures make learning more engaging by introducing students to real-world applications of course concepts. The interactive format allows students to ask questions, discuss professional challenges, and gain a deeper understanding of the I/DD field. Hearing from a variety of speakers helps students appreciate the complexity and diversity of the field, as different professionals bring unique insights that enrich course content and provide a more comprehensive view of the issues individuals with I/DD and their families face.

Contributions to Course Objectives: The guest lectures are carefully selected to align with course objectives, ensuring that students can see the practical relevance of what they are learning. This connection between theory and practice reinforces key course concepts while giving students the opportunity to develop critical thinking, professional communication skills, and the ability to apply theoretical knowledge to real-world practice scenarios.

Discussion Questions

The following discussion questions are designed to guide critical reflection following each guest lecture. Students are encouraged to draw on both the speaker's expertise and course material in their responses.

1. Medical Service Delivery Among Individuals with I/DD

- What are some of the unique challenges that medical professionals face when delivering healthcare services to individuals with I/DD?
- How can the integration of multidisciplinary teams improve healthcare outcomes for individuals with I/DD?
- What role can social workers play in bridging the gap between medical professionals and individuals with I/DD to ensure comprehensive care?

2. The Impact of Caring for a Loved One with I/DD

- What are the specific challenges faced by older adults with I/DD, and how do these differ from those faced by younger individuals?

- How can caregivers and social workers collaborate to ensure a high quality of life for aging individuals with I/DD?
- What strategies can be implemented to address the long-term care needs of older adults with I/DD?

3. Social Work Professionals in the I/DD Field

- How do current policies impact the delivery of services to individuals with I/DD, and what changes are needed to improve these services?
- What are the key considerations when advocating for policy changes that affect individuals with I/DD?
- How can social workers effectively influence policy decisions to benefit the I/DD community?

Intellectual/ Developmental Disabilities in Social Work Practice

EMPOWERMENT • INCLUSION • DIGNITY • JUSTICE

Intellectual/Developmental Disabilities in Social Work Practice offers a comprehensive, strengths-based approach to understanding and supporting individuals with intellectual and developmental disabilities (IDD) across the lifespan. Grounded in social justice and the core values of the profession, this text equips social work students and practitioners with the knowledge, skills, and tools needed to promote inclusion, advance rights, and foster person-centered supports.

COVERS THE LIFESPAN

From early intervention to aging and end-of-life, addressing key transitions and support needs.

EVIDENCE-INFORMED & PRACTICE-FOCUSED

Integrates current research with real-world applications, case examples, and critical reflection.

SOCIAL JUSTICE AT THE CORE

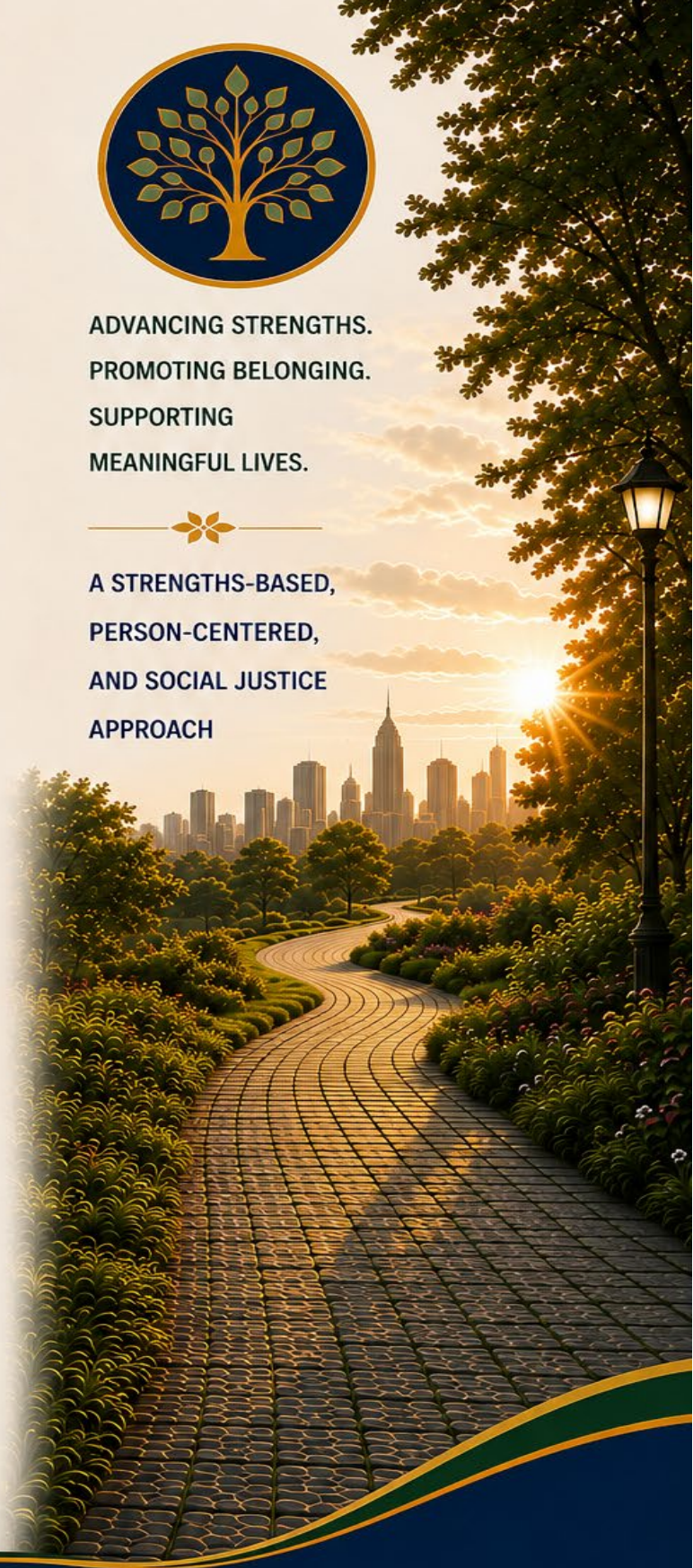
Centers dignity, self-determination, and the voices of individuals with IDD, families, and communities.

TOOLS FOR EFFECTIVE PRACTICE

Includes practical strategies, reflective exercises, and resources for empowering practice.

ADVANCING STRENGTHS.
PROMOTING BELONGING.
SUPPORTING MEANINGFUL LIVES.

A STRENGTHS-BASED, PERSON-CENTERED, AND SOCIAL JUSTICE APPROACH



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